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Allergy and immunology

47 M struggling with hay fever and pollen = Allergic rhinitis

Qs not to miss:

- Types of symptoms
- Red flags: unilateral symptoms, bloody discharge, recurrent epistaxis and pain
- RFs: smoking/ FH/ past Hx of asthma

Cues not to miss:

- Impact on life and work
- SC injection

ICE:

- Ideas: Pollen
- Concern: work
- Expectation: allergy test / SC injection

Management vital points

- Explain allergic rhinitis: allergy affecting nose when exposed to triggers
- ICE – can do allergy test and explain about not 100% accurate
- Allergen avoidance advice (masks N95 / glasses/ avoid high pollen count days)
- Stepwise medication approach (intranasal steroids then add intranasal antihistaminic then consider adding regular oral antihistaminic or montelukast)
- Consider short course of steroids for rapid relief if significant symptoms
- Trial of combination of intranasal steroids and antihistamines (Dymista)/ LTRAs for at least 2 months
- Options for referral for SC or sublingual immunotherapy (exposing person to increasing amounts of an allergen to induce tolerance (weekly doses then 4 weekly for 3 years)

16 F + mum concerned about daughter's asthma = Asthma concern over allergy

Qs not to miss:

- Triggers? Severity of symptoms?
- Red flags (exacerbation): fever, SOB, chest pain, phlegm
- RFs: smoking/ stress due to exams
- Impact: not doing exercise

Cues not to miss:

- Not using spacer

ICE:

- Ideas: specific allergens? Need allergy test
- Concern: asthma related death in the family
- Expectation: Urgent allergy test

Management vital points

- Explain that poor control of asthma due to poor inhaler technique
- Trigger avoidance
- Inappropriate referral to allergy test
- Alternative to spacer due to her embarrassment (DPI or breath actuated inhaler are better options)

15 M + vaccination query by Dad + missed vaccines due to mum refusal = vaccine consent teenager

Qs not to miss:

- Social history
- Gillick competence
- Missed vaccine after age of 10 Y = HPV, DPT booster, and Men ACWY

Cues not to miss:

- Age 15 and need to inform mum

ICE:

- Ideas: Need to know what vaccines was missed and information
- Concern: Risk of infection with petting a cat/ travel abroad
- Expectation: missed vaccine and how to get them

Management vital points

- Potential for safeguarding issues
- Explain vaccination and its importance
- Options to wait few months until he turns 16, so doesn't need any consent. Other option is to proceed if he is Gillick competent (better to liaise with mum so)

13 M + recent admission with angio-oedema + mum wants to discuss further steps = Angiooedema

Qs not to miss:

- Experience and what happened
- Red flags: swelling of lips/face/ hoarseness of voice/ rash/ breathing problems
- Triggers/RFs: insect bites/food/ material/ latex/ cold/ pressure

Cues not to miss:

- Mum anxious and worried

ICE:

- Ideas: Allergic to something
- Concern: more severe reaction can be life threatening
- Expectation: adrenaline autoinjector/ allergy tests/ advice what to do

Management vital points

- Explain allergen as something you've eaten, inhaled or come in contact and the body immune system respond to it as harmful. Angioedema is a deeper form of hives which occurs deep in the skin and more around lips and eyes. Risk of anaphylaxis which involve breathing problems and drop in BP which require immediate care
- If on ACEI – stop immediately
- Allergy diary to record any possible trigger + referral to allergy clinic to do some allergy test if no identifiable trigger + avoidance of any possible triggers
- 6 weeks daily non-sedating antihistaminic such as cetirizine up to 12 months if chronic angioedema
- To discuss adrenaline autoinjectors with specialist via A&G if at risk of anaphylaxis (coexisting asthma/COPD/heart disease/ can't avoid allergy/ happen with trace amount of allergen)
- Safety netting if anaphylaxis to call 999 + give adrenaline autoinjector and lie flat with raised legs then use 2nd autoinjector if needed after 5 minutes if no better.
- Discontinue BBs if adrenaline autoinjector is needed

Women Health

56 F + Hx of IBS recently + recurrent urinary infection not responding to antibiotics + negative MSU = Ovarian cancer

Qs not to miss:

- UTI symptoms? Vaginal symptoms? Sexual active?
- Red flags: persistent bloating, abdominal pain, early satiety, urinary symptoms without infection, new onset IBS >50 years, FH, weight loss

Cues not to miss:

- Urinary symptoms with -ve MSU

ICE:

- Ideas: Infection?
- Concern: Impact on life? Antibiotic not working?
- Expectation: treatment

Management vital points

- Psychosocial aspect (Breaking bad news)
- Investigations – CA125
- Offer examination to assess masses- Urgent 2 WW referral
- Follow up

39 F + cousin with breast cancer risk = breast cancer FH

Qs not to miss:

- Symptoms / lifestyle
- Red flags: breast pain/ lumps/ discharge/ bleeding/ eczematous changes, FH (1st degree with (breast cancer <40/ male breast cancer/ bilateral breast cancer), 1st/ 2nd degree with breast cancer + 1st/ 2nd degree with ovarian cancer, Three 1st or 2nd degree with breast cancer at any age, BRCA1/2 gene mutation
- Contraception

Cues not to miss:

- Anxiety

ICE:

- Ideas: increased risk of breast cancer
- Concern: referral to genetics? Safety of IUD
- Expectation: Referral to genetics

Management vital points

- Anxiety support
- Explain risk given mum ovarian cancer and grandma breast cancer
- Explain breast awareness
- Offer referral for genetic testing
- Discuss IUD is safe and only COC can be risky
- Lifestyle changes

28 F + 10 weeks pregnant + previous miscarriage + exposure to chickenpox

Qs not to miss:

- Hx of chickenpox infection /Vaccination including two doses of varicella
- Symptoms (fever/rash/itchiness)
- Mum's condition
- Red flags: abdominal pain or vaginal bleeding
- Lifestyle in general

Cues not to miss:

- Carer for her mum
- Anxious

ICE:

- Ideas: test or treatment for chickenpox
- Concern: affecting pregnancy/ miscarriage
- Expectation: vaccine/ medication?

Management vital points

- Address anxiety and psychosocial aspect
- Explain risk given unknown immunity
- If high risk exposure and unknown immunity, check varicella zoster IgG Abs
 - If positive, reassure
 - If -ve, urgent referral to specialist for VZIG
- Safety netting if rash
- If no chickenpox, can be vaccinated after pregnancy and avoid pregnancy after for 3 months and can safely breastfeed

17 F + requesting morning after pills = Emergency contraception	Qs not to miss: • Last UPSI in hours • Contraception including condoms /Contraindication for contraception (migraine with aura/ VTE/ stroke/ breast cancer/ ovarian cancer/ PV bleeding • Sexual health screening? STI • Partners/ periods + ask about BP/ BMI Cues not to miss: • Asking for multiple pills ICE: • Ideas: morning pills • Concern: protect against pregnancy • Expectation: multiple pills Management vital points • Explain multiple emergency contraception is not recommended and give Options to quick start contraception today (immediately after LNG-EC and 5 days after UPA-EC) + use condoms till effective (7 days except POP for 2 days) • Emergency contraception with Levonorgestrel within 72 hours + instructions (just delay ovulation does not prevent it/ if vomiting within 2 hrs TCI) • STI prevention and testing • Safety netting: no periods, severe lower abdominal pain, PV bleeding • Pregnancy test after 3 weeks
27 F + 6 weeks pregnant + morning sickness + sleepy with cyclizine + trace ketones = N/V in pregnancy	Qs not to miss: • Drinking and eating?/Non-medical options: ginger • RFs: FH/ medications (iron)/ comorbidities/ infections • Impact on life / psychosocial/ support • Red flags: abdominal pain/ PV bleeding, headache, haematuria, UTI symptoms, dehydration (dark urine/ less amount), weight loss, inability to tolerate fluids Cues not to miss: • Stress at work ICE: • Ideas: pregnancy related sickness • Concern: work safety/ severe symptoms • Expectation: medications? Sick note/ advice about work Management vital points • Explain Dx • Examination to check vitals and exclude red flags / MSU for UTI • Non medication option (small frequent sips/ ginger/ avoid triggers e.g. iron/ odor • Manage reflux with PPI • Alternative fewer sedative meds: Xonvea or Stematil • Follow up in 2-3 days and safety netting if red flags • Address work and give sick note to prevent HG/ discuss legal rights to discuss with HR/ support system
23 F+ 18 weeks pregnant + HTN = HTN in pregnancy	Qs not to miss: • Red flags: headache, blurred vision, PV bleeding, vomiting, oedema, abdominal pain. Seizures • Medications (e.g. ACEI/Aspirin) • Psychosocial: impact/ support Cues not to miss: • Romany travelling community – no address • Inequalities – weak literacy skills ICE: • Ideas: difficult care with travelling • Concern: hospital appts, literacy skills, don't drive so struggle with appts • Expectation: manage BP and hospital appts Management vital points • Explain Dx and risk of preeclampsia • Refer to consultant • Safe prescription: 1st line is labetalol • Safety netting and follow up once a week to check BP and protein in urine • Continuity of care: register GP as her address to stay informed – can have phone calls or voice message – avoid text messages – hospital to send to GP

19 F + wants to change pills + random spotting = STI

Qs not to miss:

- Sexual Hx/ condoms
- Symptoms: discharge, dyspareunia, bleeding
- Red flags: post coital bleeding/ pregnancy/PID (pelvic pain/PV bleeding/ fever)

Cues not to miss:

- Discharge and dyspareunia
- No other partners

ICE:

- Ideas: pills not working
- Concern: pregnant? -ve test
- Expectation: stronger pills? Dianette

Management vital points

- Discuss DDx including STI/pregnancy
- Examination for DDx e.g. polyps and ectropion
- Invx: STI screening (offer online self-sampling tests+ pregnancy test then USS
- Non-judgemental – infection can be asymptomatic – can be from previous partner – refer to GUM clinic if STI
- STI prevention with condoms and partner notification

23 F + Vaginal irritation + -ve STI + light yeast growth = Latex allergy

Qs not to miss:

- Triggers
- Sexual Hx – Hx of oral symptoms
- Contraception/ partners/ periods/ condoms
- Red flags: anaphylaxis

Cues not to miss:

- Worsen after sex then better
- Sex worker
- Non-judgmental

ICE:

- Ideas: recurring thrush?
- Concern: losing client
- Expectation: meds for thrush

Management vital points

- Avoid exposure by latex free male or female condoms/ avoid balloons rubber toys
- Wear Medical alert bracelet
- Explain that it happen more with repeat exposures
- Safety netting for anaphylaxis symptoms

48 F + hot flushes + requesting HRT = menopause

Qs not to miss:

- Periods/ other symptoms
- Rd flags: Post coital bleeding/ IMB/ vaginal discharge/ abdominal pain/ mood
- DDx: pregnancy/ psychological/ endocrine problems
- Contraindications? Breast cancer/ abnormal vaginal bleeding/ DVT/ liver disease

Cues not to miss:

- Impact on work

ICE:

- Ideas: Menopause? Blood test needed
- Concern: HRT and breast cancer
- Expectation: HRT vs alternatives

Management vital points

- Explain perimenopause – clinical, no blood test needed
- Discuss risks: Combined HRT can increase the risk of breast cancer but doesn't affect risk of death from breast cancer. Oestrogen only increased risk of endometrial cancer
- Discuss options oral vs transdermal (less DVT risk)
- HRT is not contraception (options: IUS + oestrogen HRT or combined HRT + POP)
- Discuss non hormonal HRT e.g. SSRI/ clonidine/Gabapentin + lifestyle changes: avoid caffeine or alcohol
- Follow up in 3 months then annually

47 F + requesting HRT + on contraception

Contraception notes with HRT

- Should be used for perimenopausal women or unsure about menopause
- All progesterone contraception options are safe to be used with HRT. (POP/injections/implants with sequential or Mirena with Oestrogen only)
- CHC can be used as alternative to HRT if < 50 Yrs
- If patient with amenorrhoea due to progestogen only method- check FSH: if FSH level >30 IU/L stop after 1 year, if <30 IU/L, repeat again in 12 months
- When to stop contraception:
 - <50 years - 2 years after last period
 - >50 years - 1 year after last period
 - On sequential HRT –contraception required until 55 years

62 F + requesting repeat HRT

Stopping HRT

- In women with POI, HRT should be continued until normal menopause age.
- Risk of increasing breast cancer after 5 years of use and increased risk of stroke
- Options to decrease risk of breast cancer to change combined HRT to oestrogen patches + Micronized progesterone

56 F + bleeding on continuous HRT after swabbing from sequential HRT not responding to increasing progesterone dose = Bleeding on HRT

Qs not to miss:

- Impact of the problem
- Duration > 6 months post changing/starting HRT – need to take action
- Red flags: abdominal pain, bloating, change in bowel habits, urinary symptoms, weight loss

Cues not to miss:

- HRT and work / Helping with mood

ICE:

- Ideas: swapping from sequential to continuous is causing the problem
- Concern: affecting her work
- Expectation: going back to previous HRT

Management vital points

- Offer examination to exclude DDx + swabs for infection +/- STI screen
- Exclude red flags such as cancer by examination and USS
- Steps for unscheduled bleeding within 6 months of starting HRT or > 3months of changing HRT
 - Urgent USS + if normal, change HRT or progesterone dose or type
 - If persistent 6 months after change, then USS within 6 weeks + consider weaning off HRT and try non hormonal HRT
 - If bleeding stop after 4 weeks of stopping HRT – no further action. May consider restarting HRT for 6 months if needed but with urgent USS if bleeding again.
 - If normal invx + wants HRT = OPTIONS (change route/ decrease oestrogen/ change progesterone/ Mirena + oestrogen)
 - Perform immediate USS if heavy bleeding + 2 minor RFs for endometrial cancer
- Urgent referral within 2 weeks if one major or 3 minor RFs for endometrial cancer + stop HRT while assessment
- Offer alternative including SSRI and lifestyle changes

48 F + on HRT + low libido requesting testosterone

Qs not to miss:

- Sexual Hx and Impact on her
- Mood

Cues not to miss:

- Sertraline for low mood
- Relationship

ICE:

- Ideas: testosterone can help
- Concern: relationship with partner
- Expectation: testosterone prescription

Management vital points

- Explain DDx of low libido (medications/ mental health/ hormones (check TSH/HbA1C)/ lifestyle)
- Alternative to sertraline may be an option: Mirtazapine/pregabalin
- Change from oral to transdermal can help
- If still not helpful, check testosterone level and start trial if level <1.4
- Topical testosterone takes 10-12 weeks before benefit is felt.
- Treatment should be tried for a minimum of 3 months
- Monitor after 3 months and give instructions about usage and side effects.

33 F + Menstrual bleeding not responding to TXA = Menorrhagia

Qs not to miss:

- Red flags: IMB, PCB, vaginal discharge, dyspareunia, bloating, bowel symptoms, weight loss, abdominal pain, easy bruising
- Pills/pregnancy/ partner
- RFs: FH of bleeding disorders/medications
- Complications: anaemia (tiredness)/ anxiety
- Plans for family
- Cervical screening
- Precautions to contraception: breast cancer/ DVT/ liver disease/ migraines

Cues not to miss:

- Impact on work and daily activities

ICE:

- Ideas: hysterectomy?
- Concern: no options?
- Expectation: alternative to help the problem

Management vital points

- Examination to exclude DDs and FBC/ swabs
- LNG IUS 1st line if no pathology or small fibroid
- Alternative hormonal: CHC or cyclical oral progestogen
- Non hormonal options: TXA + NSAIDs
- 2ry care option: ablation/ hysterectomy/ uterine artery embolization

31 F + HPV +ve with mild dyskaryosis + patient concern about STI

Qs not to miss:

- Sexual Hx + 4 Ps (partners/ pills/ pregnancy/ periods)
- Rd flags: IMB/post coital bleeding/ FH/ weight loss/ pain
- Vaccination against HPV (before becoming sexually active)

Cues not to miss:

- Partner is cheating/ cancer

ICE:

- Ideas: STI/ cancer
- Concern: partner fidelity/ cancer
- Expectation: cancer treatment/ partner check/ STI can be for years?

Management vital points

- Explain findings (HPV is STI and mild changes in cells = not cancer). No treatment for HPV (body usually clear it)
- Reassure about HPV can be dormant for years
- Next steps
 - If -ve HPV = recall after 3 years
 - If HPV +ve and -ve cytology = recall after 12 months
 - If -ve at 12 months = routine recall /If +ve = repeat after 12 months
 - If -ve at 24 months = routine recall/ if +ve, refer for colposcopy
 - If +ve HPV and +ve cytology = refer for colposcopy
 - If inadequate results, repeat in 3 months, if still same then refer
- Colposcopy is microscope: visualize +/- biopsies = can cause spotting/ cramps = avoid tamponades/ sexual intercourse/ douching for short period

27 F + pregnant + concern about antenatal screening

Qs not to miss:

- Pregnancy and previous use of contraception/ partner
- FH of genetic diseases

Cues not to miss:

- Member of latter-day church of Jesus Christ (Mormon)
- Not keen on abortion

ICE:

- Ideas: which screening to skip
- Concern: don't want to know about conditions that doesn't result in abortion
- Expectation: antenatal screening which to do and which not

Management vital points

- Explain 1 dating scan at 11-14 weeks (will be asked for combined test for Down's/Edward and Patau's syndrome if wants) + 3 screening: infectious diseases (HIV/syphilis and HBV) – important for both mum and baby to prevent infection in baby/ sick cell and thalassemia before 10 weeks (can be skipped) and 20-week anomaly scan (4 of 11 incompatible with life (her choice)

52 F + wants to delay 2 WW referral for suspected breast cancer	Qs not to miss: - Red flags: Changes in the mass/ bleeding/discharge from breast/ pain Cues not to miss: - Holiday / Previous GP made this risk sound low ICE: - Ideas: prioritize holiday + why hidden last consult? - Concern: cancer/ holiday which has been planned for 6 months - Expectation: rearrange 2 WW referral Management vital points - Address the difficulty of cancelling the flight vs health priority - Explain high risk of cancer and spread which can affect survival – time is critical - Offer support for documentation for the insurance company - Hidden consult to avoid stress while under invx
61 F + PV bleeding + not on HRT + menopause = PMB	Qs not to miss: - Periods Hx? HRT? Sexual Hx - DDx/ red flags: post coital bleeding/ IMB/ discharge/ weight loss - RFs: early menarche/ late menopause/estrogen only HRT/ infertility/ PCOS Cues not to miss: - Friend scares her about cancer ICE: - Ideas: No idea? Period? - Concern: expect more bleed? - Expectation: reassurance that this is normal Management vital points - Explain PMB and steps needed - Explain next steps about USS and biopsy - Offer follow up
14 F + Hx of self-harm + mum concerned about premenstrual dysmorphic disorder (PMDD)	Qs not to miss: - Explore about anxiety and mood/ ask about last self-harm - Assess safety and suicidal risk/ support - Exclude other DDx: drugs/ alcohol/ depression - Criteria to Dx (5 or more symptoms (liable/ irritable/ hopeless/ anxious + depressed/ lack of focus/ tired/ sleep problems/ out of control/ physical symptoms) in the majority of cycles improve after menses significantly affecting function after exclusion of other causes) Cues not to miss: - CAMHS log waiting list ICE: - Ideas: PMDD and need referral - Concern: suicide/ affect life - Expectation: Referral to O&G Management vital points - PMDD is severe form of PMS - Referral for 1st line SSRI – CBT/ Dialectical behavioural therapy - Other options CHC (Yasmin) in primary care + lifestyle changes - No benefit from gynaecology referral as per guide
79 F + urinary incontinence = stress and urge mixed incontinence	Qs not to miss: - Red flags: UTI/ weight loss/ PV bleeding/ IBS symptoms - Triggers/ RFs/ DDx stress vs urge/ UTI/ DM - Menopause/ HRT Cues not to miss: - Avoiding going out ICE: - Ideas: no idea - Concern: affecting relationship with friends - Expectation: tablets to help Management vital points - Explain Dx/ offer examination/ Urine and blood test - Bladder training for 6 weeks and pelvic floor exercise for 3 months + lifestyle changes. If failed – medications such as mirabegron/ antimuscarinic for urge and duloxetine for stress incontinence, then refer is no better

Sexual health

14 F + was seen with mum for acne + alone today requesting pills for heavy periods = contraception

Qs not to miss:

- Red flags: IMB/ abdominal pain/ vaginal discharge/ safeguarding
- DDx PCOS (hirsutism/ weight issues/ irregular periods)
- Partner and safety
- Risk with contraception (migraine/ cancers/ DVT/smoking)
- Assess competence

Cues not to miss:

- Planned sexual activity – no plan to inform mum (confidentiality)

ICE:

- Ideas: Pills will help with both acne and contraception
- Concern: doctor will tell mum about plans
- Expectation: Pills and reassurance about not informing mum

Management vital points

- If competent, give reassurance about confidentiality and ensure safety
- Explain contraception options + STI prevention by using condoms
- CHC is good option for heavy periods/acne and contraception – explain it
- Give advice about missed pills (leaflet)/ possible need for advice
- Follow up after 3 months to check compliance and BP

48 F requesting contraception

Contraception in women >40 years

- More risks
- HRT is not contraception – need contraception 1 or 2 years based on age
- If >50, avoid CHC and progesterone injections
- Other methods can be safely used till the age of 55 year
- Discuss smoking cessation (will exclude CHC)

41 M + asking for the snip and worried about complications

Vasectomy:

- Explain the procedure – high risk of regret – alternative options for females
- Risks: reversible in only 50% (not funded in the NHS)
- Can be done under local anaesthesia
- Risk of long-term testicular pain/ very low failure rate
- Still need STI protection with condoms
- Contraception still needed for 2-3months after to ensure azoospermia by 2 sample 4 weeks apart

36 M prefers to be called as a female wants bridging endocrine treatment = Gender identity

Qs not to miss:

- Mental health assessment – exclude red flags

Cues not to miss:

- Guidelines brought by the patient

ICE:

- Ideas: wants to become a woman
- Concern: long waiting list – options
- Expectation: GP to refer to gender clinic and start meds

Management vital points

- Explain limited expertise which can affect safety with the needed monitoring
- Offer options – other GP or doctors + refer to gender clinic
- Bridging therapy can be prescribed if already self-prescribing hormones/ to decrease risk of self-harm/ after specialist advice
- Offer support groups/ and ensure safety netting
- GP can only accept private trust provider from Right to Choose with NHS contract (if unable to afford follow up appt, everything will stop leaves GP in charge fully about everything)

Dad with letter asking for hormone therapy for his daughter 12 F (wants to be male)

61 M requesting prostate check for LUTS = PSA	Qs not to miss: • Red flags: UTI/ ED/ weight loss/ back pain/ blood in urine Cues not to miss: • Friend with prostate cancer ICE: • Ideas/concerns: prostate cancer? • Expectation: blood test Management vital points • Explain prostate gland and DDx – need for blood test and PR • Discuss PSA – instruction and results analysis (1 in 4 with high level has cancer)
39 F struggling to get pregnant for the past 9 months = Infertility	Qs not to miss: • Sexual Hx (Frequency of intercourse and quality) and menstrual HX • Hx of STI (post coital bleeding/ IMB/ Vaginal discharge/ dyspareunia/pelvic pain) • POI symptoms (dry vagina/ hot flushes/ period irregularity) • Partner (previous relationship/ children/ problems/smoking/drugs) • FH of early menopause /PMHx of hypothyroidism/ pelvic surgeries/ BMI/PCOS Cues not to miss: • Hx of depression/ impact on her work as pharmacist ICE: • Ideas: age/stress/physical issue affecting fertility • Concern: private treatment for this problem/ job and relationship security • Expectation: referral for fertility clinic with partner/ advice about NHS funded treatment Management vital points • Offer examination to exclude DDx (PCOS/BMI/swabs) and for partner • Explain guidance to refer after 12 months trial for conceiving with regular sex 2-3 times weekly despite normal results (earlier if ≥ 36 year (this case) or known cause for infertility or on treatment can affect fertility) • Tests (hormones (FSH/LH/TSH/Prolactin) + ovulation monitoring (progesterone level 7 days before expected period + semen analysis) • 8 in 10 will conceive in 1st year and half of who didn't will conceive in 2nd year • Mental health counselling + lifestyle changes + folic acid
28 M + requesting anti -depressants to feel better = Premature ejaculation	Qs not to miss: • Symptoms of depression? Sexual Hx / erection issues • Red flags: pain/ penile discharge/ testicular lumps • DDx: stress/ medications/ prostate issues/ ED Cues not to miss: • Antidepressant to feel better/ no depression symptoms/ Work relationship ICE: • Ideas: lifelong problem needs fixing • Concern: relationship/ workplace problem • Expectation: antidepressants to help Management vital points • Non-medical: squeeze technique and stop go technique – masturbation 1 hour before – breaks during sex – thick condoms with local anaesthetic • Dapoxetine more specific than other SSRI, PRN 1-3 hours before sex but not more than once daily used if all the following: poor control over ejaculation, a history of premature ejaculation over the past 6 months, marked distress or interpersonal difficulty as a consequence of premature ejaculation, and an intravaginal ejaculatory latency time of less than two minutes • Sildenafil and other SSRI (daily dosage) can be used • Psychosexual therapy can be an option if persistent

**17 F asking for
abortion despite
being on pills =
Termination of
pregnancy**

Qs not to miss:

- 4 Ps (pills/periods/partner/ pregnancy) – Lifestyle (smoking and alcohol)
- Red flags: PV bleeding/ abdominal pain/ dizziness
- Safeguarding (e.g. Trafficking/ coercion (not consensual))
- Partner agreement (legally can't refuse it)

Cues not to miss:

- Parents unaware of sexual activity

ICE:

- Ideas: want abortion - Concern: scared/ anxious
- Expectation: to get it done today

Management vital points

- Options: keeping baby or putting baby up for adoption!!
- Explain abortion and its complications (Medical abortion: 2 pills: one called mifepristone to disturb pregnancy and 2nd one is misoprostol after few days to help uterus to contract) – surgical suction from 7-15 weeks and surgical D&C after 15 weeks
- Patient can self-refer – doctor can refer to another doctor if can't advise for TOP
- If <16 = need 2 doctors to sign or if doesn't want = parents to sign for her best interest – or need to seek advice

Infectious diseases

**17 F +
embarrassed +
painful spots down
below = Genital
herpes**

Qs not to miss:

- Menstrual and sexual Hx + partner
- Contraception and condom – 4 Ps
- Describe the spots

Cues not to miss:

- Embarrassed/ mum unaware/ exclude safeguarding
- Cheating on me?

ICE:

- Ideas: Acne
- Concern: embarrassed/ pain/ spread of acne
- Expectation: to get some cream

Management vital points

- Confirm confidentiality
- Explain Herpes/ recurrence 4-5 times in first 2 years – long term condition with no cure but controllable – STI – can be latent for years (doesn't mean infidelity)
- Need examination to confirm Dx with chaperone
- Refer to STI for contract tracing and screen for others STI + partner testing
- Educate about contraception and STI prevention
- Can offer antiviral with acyclovir for 5 day 5 times a day, repeat if new lesions + analgesia + barrier cream
- If not keen to go to GUM = follow up after 5 days + avoid sex if lesions are present + test for other STI by vaginal swab or first void urine sample
- Can be infective while asymptomatic = use condoms to decrease risk
- Discuss long term contraception

**31 M + new
registered pt want
to discuss HIV and
shared care
urgently with GP =
HIV confidentiality**

Qs not to miss:

- Shared agreement details
- Hx of HIV infection/ last viral load/ symptoms/ compliance
-

Cues not to miss:

- Wife doesn't know/ wife's cousin receptionist in this surgery

ICE:

- Ideas: add the shared care meds to his records/ locking records to be seen by one GP – no permission for text message or online access for others
- Concern: shared care meds arrangements/ wife and HIV status
- Expectation: as ideas

Management vital points

- Explain HIV and AIDs
- No legal obligations to inform partners as long as taking precaution to prevent transmitting it, but it strongly recommended to inform them. If not taking precautions, this is criminal offence. Doctors has the rights to disclose information to patient at risk if patient with HIV putting others at risk after telling patient with HIV as the last resort
- Shared care plan – no issues as per practice policy
- Restricting medical records – no issues after discussing risks of it

ANP debrief + prescribed mebendazole for pregnant and 5 months child = threadworms	Qs not to miss: • Debrief: Hx/ pregnancy/ social Hx/PMH/ allergies Cues not to miss: • Just started 3 months ago • Struggling with time ICE: • Ideas: don't know about risks of mebendazole in this groups • Concern: GP to find fault in the consultation • Expectation GP go over things quickly Management vital points • Explain proper management in this group by trying hygienic c changes for 6 months then try meds if safe (not advised in pregnancy and < 6months) • Address time constraint/ offer extended appt • Correct mistake and ask for significant event analysis/ discuss with ES
22 M + unwell with fever and headache behind ears since back for SA + negative malaria test = Dengue fever	Qs not to miss: • Red flags: unusual bleeding or bruising/ worsening abdominal pain/ persistent vomiting/ liver enlargement/leukopenia/ shock/ restlessness • Vaccination before travel Cues not to miss: • Travel to Sout Asia • Looking after 4-year nephew ICE: • Ideas: malaria despite negative test and given insect bites • Concern: false -ve results – inability to complete final project – looking after his nephew • Expectation: confirm Dx – advice about stress – advice about contact w nephew Management vital points • Explain possible viral Dengue fever (DDx malaria and typhoid) • Avoid mosquito bite to avoid infecting others –risk of reinfection with other subtypes • Offer examination to check vital and do abdominal examination • Invx to confirm Dx to check NS1 antigen and serology/FBC/LFTs • Conservative usually + Avoid NSAIDs + ensure hydration • Offer support for stress from the university • Safety netting about red flags
1 Year child + FU after discharge with febrile seizure + mum concerned = Febrile seizure	Qs not to miss: • FH of fits/ epilepsy • Current situation: new symptoms/ red flags: unwell/ recurrent fit within 24 hrs/ high fever/ vomiting/ skin rash/ seizure > 15 min, focal signs, incomplete recovery after 1 hr • Antenatal/ neonatal/ developmental Hx Cues not to miss: • Feeling guilty about extra blankets ICE: • Ideas: increase risk of epilepsy • Concern: recurrence? Intellectual delay? Epilepsy risk • Expectation: advice if recurrence/ future epilepsy risk Management vital points • Address anxiety and reassure almost all children make complete recovery • Safety netting if red flags to contact 999 • If recurrent (1in 3 usually in the 1st year 70%): left lateral position + protect from injury + may be sleepy for 1 hr after the fit + call ambulance if lasts> 5 mins • Risk of epilepsy is low (double risk of general population =4%) • No lasting effect on intellectual function

Angry patient for delaying antibiotic prescription for son for AOM = angry patient

Qs not to miss:

- Social Hx
- Explore recent infection – acute otitis media
- Exclude ref flags: unwell child/ photophobia/ persistent fever/ rash

Cues not to miss:

- Daughter had meningitis and passed away

ICE:

- Ideas: Not giving antibiotics due to cost
- Concern: without antibiotics, will deteriorate as her sister did few years ago
- Expectation: written note to have antibiotics when he has infection in the future

Management vital points

- Address health anxiety
- Explain about viral infection can take up to 1 week – better within 12 hours of taking antibiotics is likely not due to antibiotic – 8 in 10 will be better without antibiotics for AOM in comparison to 9 in 10 if given antibiotics – antibiotics help with pain/ reduce perforation/ collateral otitis media
- Antibiotics are needed if < 2 years with bilateral infection/ AOM with otorrhea
- Alternative: topical ear drops for pain relief/ delayed antibiotics if worsening

14 F + positive chlamydia + last consultation with mum denies sexually active = STI

Qs not to miss:

- Red flags: SAFEGUARDING (ask about partner)/ PID (fever/ abdominal pain/ dyspareunia) \IMB/ postcoital bleeding
- Menstrual/ sexual hx – condoms
- Competent or not

Cues not to miss:

- Age/ denied sexual activity/ safeguarding? Call without mum

ICE:

- Ideas: STI?
- Concern: break confidentiality?
- Expectation: Dx and treatment

Management vital points

- Assess competence before ensure confidentiality
- Explain laws about sex/ensure confidentiality and safety – any sex <13 years should be reported to child protection service as criminal offence. Any sex before 16 is illegal and should consider Gillick competence and ensure safety
- Offer referral to GUM for contact tracing and partner treatment
- Avoid sex until she and her partner are treated as well
- 1st line 7 days doxycycline 100mg BD or (2nd line: Azithromycin 1 g once then 500mg OD for 2 days if pregnancy or breastfeeding)
- Discuss STI prevention and contraception
- Offer retesting to all people infected with chlamydia 3-6 months after completion of treatment to check reinfection
- Test cure only if pregnant/ poor compliance suspected/ symptoms persist

17 F + seen by PA + tonsillitis with exudate + lymph node enlarged in neck +? enlarged spleen for antibiotics? = Glandular fever	Qs not to miss: - Red flags: sudden severe abdominal pain/ vomiting/ breathing problems - Ask about exercise and sports Cues not to miss: - Antibiotics/ Swelling in stomach/ rash initially all over body/ Playing rugby ICE: - Ideas: Tonsillitis - Concern: swelling in stomach? Want to be better for upcoming Rugby matches - Expectation: Antibiotics? Management vital points - No need for antibiotics – likely viral (EPV) – can cause rash - Arrange blood test in 2nd week with FBC and monospot test - Conservative management: symptoms last for 2-4 weeks - Avoid heavy lifting/ contact sports for a month to avoid splenic rupture - Safety netting – if stridor/ problems with swallowing water and dehydration/ splenic rupture – for 999 - No need to avoid school
24 M + looking yellow, tired and itchy since coming for holiday = Hepatitis A	Qs not to miss: - Travel Hx/ Urine and stool colour/ sexual activity/ 'Vaccination before travel - Red flags: unwell/ persistent vomiting Cues not to miss: - Scared/ ideas bout hepatitis (related to sex worker/ needles) ICE: - Ideas: Gilbert's syndrome? - Concern: something serious/ liver problem/ drinking related? Infectious? - Expectation: reassurance/ what it is? Any risk fo transmission? Management vital points - Explain hepatitis + prognosis (recover fully within few months) + symptomatic treatment (paracetamol and antihistaminic). - Confirm Dx with serology (HAV IgG and IgM) - Infection control: avoid close contacts/unprotected sex/ avoid sharing items/ wash hand - Avoid alcohol until full recovery - Notify health protection unit - Avoid school/ nursery/ work until not long infectious (7 days post jaundice or symptom onset) - Regular follow up every 2 weeks + repeat LFT until normal
38 M + new patient moved from Bolivia with no medical Hx + health check showing HCV +ve = HCV	Qs not to miss: - Symptoms: tiredness/ weight loss/ jaundice/ itchiness - Alcohol/ blood transfusion/ operations / sexual history/ drug history/ tattoos/ needlestick injury - Red flags: HCC (RUQ pain/ weight loss/ jaundice)/ acute liver failure Cues not to miss: - Antibiotics needed? Pt unaware of HCV and its seriousness ICE: - Ideas: which blood test is abnormal - Concern: 2 different bloods test. Is it serious? - Expectation: treatment? tests Management vital points - Explain Dx/ prognosis (90% cure rate with meds) - Abstain from alcohol - Infection control (avoid sharing items such as needles and razors/ safe sex practice/ inform sexual partners for screening and treatment) - Refer to hepatology specialist + invx in the meantime (HIV/ HBV/ HAV/ HbA1C/ Ferritin/ repeated LFTs) - Offer HBV and HAV vaccination

8 F + dad requesting antibiotics as classmate has suspected meningitis	Qs not to miss: • Red flags: signs of meningitis (fever/ unwell/ rash) • Vaccinations • Contact with the case (household type setting (sleeping and living together) during the last 7 days before the onset of symptoms or direct exposed to large particles droplets around time of admission to hospital Cues not to miss: • Wants to know classmate dx and updates (confidentiality) • Dad is lawyer and want to use his position if feels his child is at risk! ICE: • Ideas: need to have rifampicin • Concern: risk of meningitis without having it • Expectation: antibiotics Management vital points • Explain steps in managing suspected meningitis (NOTIFIABLE disease managed by public health team) • Even it is confirmed, it is not routinely to offer antibiotics to contacts at school/ nursery/ friends/ work/ food or drink sharing/ travelling in the same transport • If needed, antibiotics 1st choice is ciprofloxacin 250 -500mg stat • Reassure about low risk of transmission at 1ry school (1/18000) • Ensure confidentiality of other child – advise that public health will inform parents if needed/ one in charge of monitoring at school and reporting • Safety netting if any red flags
45 M + worried about needle stick injury while cleaning new house = Needle stick injury	Qs not to miss: • Vaccination including HBV and tetanus • Hx of drugs/ alcohol/ smoking/ operations/ transfusion • 1st aid measures (washing/ encourage to bleed/ don't suck or scrub) • Red flags: signs of wound infection • Idea about previous tenants to ask them about their infection status Cues not to miss: • GUM clinic refusal as wife work near – can have wrong idea ICE: • Ideas: HBV/HIV • Concern: Worried about impact on life and family • Expectation: antibiotics Management vital points • Explain needle stick injury risk of having HIV/HBV/HCV – risk is low • If presented within 72 hrs – refer immediately to A&E for PEP • Discuss referral to GUM clinic – no antibiotics as it is viral • Offer tetanus booster if last one > 10 years ago • Monitoring: HIV/Hepatis/ FBC at baseline, at 6 weeks/ 3 and 6 months • Offer psychological support and clear follow up plan
Ongoing fatigue after COVID with normal invx = post COVID	Qs not to miss: • Impact of life/ mental health/ support/ smoking/ alcohol • DDx of fatigue • Red flags: weight loss/ bowel habits changes/ PR bleeding Cues not to miss: • Not able to go work – financial issues • Normal blood results • Long COVID is made up condition ICE: • Ideas: fixable cause? Thyroids? Long COVID? • Concern: TO go back for his Job and his wife go to her work • Expectation: Good news Management vital points • Give Dx of long COVID and explain possible chronic nature • Dx of exclusion after having relevant blood tests • Consider referral for long COVID clinic if significant, persistent symptoms with impact on daily life usually after 12 weeks (4-12weeks post COVID) • Signpost to social prescriber and citizen advice for financial advice • Signpost patient to online resources

6 months + mum requesting antibiotics just in case while abroad after episode of bronchiolitis	Qs not to miss: - Immunizations and developmental hx - Red flags: fever/ rash/ cough/ unwell Cues not to miss: - Website saying antibiotics prevent serious bronchiolitis ICE: - Ideas: without antibiotics can get worse again and need hospitalization - Concern: while abroad might get worse/ Worried about access to healthcare - Expectation: Prescription of antibiotics even private prescription to pay for it. Management vital points - Explain bronchiolitis as viral infection – no need for antibiotics as not helpful to prevent hospital admission - Recommend contacting the hotel and insurance to find out about local providers - Explaining that just in case prescription can lead to harm more than benefit (resistance/ delayed presentation) except in some exception like COPD patients - Infection control (good hygiene)
6 F + Pictures of rash on face and widespread rash on trunk + mum concerned about rash = Parvovirus B19/ Slapped cheek syndrome 28 F + pregnant + daughter got parvovirus	Qs not to miss: - Anyone in the household is pregnant including yourself? - Red flags: non blanching rash/ unwell/ neck stiffness/ photophobia/ vomiting - Vaccination and developmental Hx / allergy? Cues not to miss: - Partner is pregnant ICE: - Ideas: something minor or common/ allergy? - Concern: school attendance/ unborn baby for partner - Expectation: advice about ideas and concerns Management vital points - Explain possible Dx of slapped cheek syndrome with no needed for further invx - Advise supportive care - Confirm the child is non-infectious once the rash appears and can return to school at this stage. Pregnant lady: - Ask about current symptoms of possible infection/ previous infection - Obtain consent to contact the pregnant partner. Arrange serology testing to determine Parvovirus B19 and rubella immunity or recent exposure. If non-immune or confirmed infection, liaise with obstetrics and gynaecology. Risks to the foetus: Parvovirus B19 can lead to foetal anaemia or rarely, hydrops fetalis. - Avoid contact with other pregnant ladies (>15 mins) until results of immunity available. Can be infectious for 10 days before the rash appear - Safety netting if symptoms: fever/rash/joint pain
2 F + mum concerned about recurrent infection (5th episode in 7 months)	Qs not to miss: - Dx of these infections/ need of antibiotics/ general condition of child - Immunization/ developmental history - RFs: Nurseries/ other siblings/ atopic/ 2nd hand smoking - Rd flags: >4 ear infection in a year/ > 2 serious sinus infection a year/ Hx of IV antibiotics for infection/ serious infection as pneumonia/ complications such as FTT and poor growth Cues not to miss: - No one listening to me/ upset ICE: - Ideas: allergy? - Concern: immunity problem? Leukemia - Expectation: check immunity and test allergies Management vital points - Reassure about it is normal to have up to 8 URTI every year more common if RFs due to their developing immune system - No need to check for immunity or allergy

55 M + persistent chest infection despite 3 different antibiotics = TB

48 M + Hx of persistent cough and night sweating + new back pain and weak legs = TB: Potts disease

Qs not to miss:

- Red flags: weight loss/ haemoptysis/ night sweating/ bruising/ lumps? If back pain, ask about cauda equina syndrome symptoms
- RFs: smoking/ occupation/ travel/ immunosuppression/ steroids/ chemo/ drugs
- Impact on life and work
- Vaccinations including BCG

Cues not to miss:

- Left upper lobe crepitation / RA on methotrexate/ Travel to South Asia

ICE:

- Ideas: antibiotics not strong enough? Pseudomonas?
- Concern: possibility of serious infection/ impact on work and life
- Expectation: stronger antibiotics works on pseudomonas

Management vital points

- Explain possible TB infection – treatable infection with specific antibiotics over 6-12 months
- Urgent referral for hospital admission if high symptomatic/ unwell/ complex social needs as homeless. Otherwise, referral to local TB team
- Transport by ambulance is the best way (avoid public transport)
- Invx: CXR + 3 sputum sample from deep cough (one early morning and test for acid fast bacilli and mycobacteria culture)
- Further antibiotics might mask the symptoms and affect test accuracy
- Long term follow-up
- Notify Public Health team (notifiable disease)

Mental Health

48 M + feeling low + a lot going on and can't cope = depression

58 F + no response to different anti-depressants + = depression

Qs not to miss:

- PHQ – (little interest in doing things/ feeling depressed or hopeless)
- Red flags: suicidal thoughts/ psychosis/ risk to others/ safeguarding risks
- RFs: alcohol/ smoking/ drugs/ alcohol dependence/ isolation/ lack of support
- Impact on work/life – what are you doing for living?
- Assess risk: self-harm thoughts/ plans/ protective factors/ your future?

Cues not to miss:

- Not able to focus and had some errors in his work
- Drink alcohol to help with sleeping

ICE:

- Ideas: depressed
- Concern: how long will it take? Work security?
- Expectation: help/ medication is the best option

Management vital points

- Explain depression and go for non-medical and medical options
- Lifestyle changes: less alcohol (as it decrease the effect of SSRI)/ sleep hygiene/ stress management/ avoid isolation/ more exercise (effective as SSRI)/ healthy diet/ consider probiotic supplement/ positive thinking and meditation
- Recommend time off work to ensure safety and offer sick note and follow up
- Safety netting about suicidal thought/ safeguarding/ risk to others

35 F + NVD 4 weeks ago +} TV AND radio speaking about me = postpartum psychosis

Qs not to miss:

- Red flags: sounds talking about your better off dead or harm yourself or others/ thoughts of self-harm/ baby safety/ mania symptoms / Visual hallucinations/ thoughts of being controlled/someone placing ideas into mind/ fever
- RFs: sleep problem/ stresses/ Hx of psychology/ FH
- Baby care/ Support
- RFs: alcohol/ smoking/ drugs

Cues not to miss:

- To admit her with her baby

ICE:

- Ideas: this illogical but you are hearing these sounds
- Concern: what's going on/ things getting worse
- Expectation: tests/ help to stop these symptoms

Management vital points

- Explain Postpartum psychosis can happen after giving birth – treatable and can recover fully with antipsychotics and regular follow up + prophylactic treatment in future pregnancies
- Mother and baby unit admission with baby in the psychiatric unit
- Safety measures/plan in the meantime + crisis management with emergency contact number

16 F + BMI 15.8 + Mum worried about weight and daughter not keen to speak = anorexia nervosa	Qs not to miss: - Eating habits/ eat alone or not/ social relationship/ mental health/ exercise/ menstruation problems/ dental problems/ sleep problems - Previous weight/ fashion/ ideal weight/ comments about weight - Red flags: induced vomiting after eating/ suicidal thoughts/ dizzy spells/ recurrent infections/ losing >1kg/ week - Medications e.g. laxatives / drugs/ diet pills/ diuretics - Try to speak to daughter first then continue with mum - Support at home Cues not to miss: - Daughter will not speak and avoid eye contact - Thinks she is too fat ICE: - Ideas: just stage, she will grow out of it - Concern: worried about something more – browsing history of weight loss - Expectation: reassurance/ test/ specialist Management vital points - Explain anorexia nervosa: eating disorder when you are trying hard to lose weight and often thoughts about your weight dominate your life. Very serious but treatable with full recovery - Offer examination: BMI/ vital (red flags: pulse <40/min/ tachycardia on standing/ postural hypotension) and muscle mass - Arrange urgent admission in psychiatry unit + explain that recovery can take few months and can involve therapy/ counselling/CBT/ medications - Blood tests: FBC/ U&Es/calcium/ mg/ ck/ phosphate / glucose and ECG
24 F + T1DM + recurrent admission for ketoacidosis + non-compliant with insulin = diabulimia	Qs not to miss: - Hx of DM/ control previously/ triggers for non-compliance/ side effects of insulin - Weight/ self-image/exercise/ same Qs as above - Red flags as above + DKA (ketones in urine/ stomach pain/ vomiting) Cues not to miss: - BMI 18 – HAS to ask about weight / Insulin causes havoc with my weight ICE: - Ideas: Weight related - Concern: weight – not aware of how dangerous skipping insulin - Expectation: advice about DM and stopping fixation on weight Management vital points - Explain diabulimia occurs when pt skips or reduces insulin to lose weight - Explain serious risks on kidney/ eyes/ nerves or life - Refer to MDT (eating disorder clinic) + CBT/family therapy/ psycho- education and consider SSRI in the meantime + patient leaflet and follow up - Address weight challenge and advice about balanced diet and exercise
69 F + requesting inhalers as thinks she got asthma = anxiety with panic attacks	Qs not to miss: - Mood and social Hx/ lifestyle/ support/ drugs/ sleep - Red flags: chest pain/ palpitations/haemoptysis/ suicidal thoughts Cues not to miss: - Symptoms for few minutes at any time - Loneliness ICE: - Ideas: it is asthma - Concern: asthma attack can be fatal/ being alone and if become unwell - Expectation: advice / inhaler Management vital points - Explain dx of panic attacks and why happen - Offer non-medical advice about exercise/ CBT - Medications options: BBs for panic attacks – SSRI for anxiety

49 M + ongoing left upper arm affecting left side of chest= stress 61 F + carer for mum + exhausted and headache = carer breakdown	Qs not to miss: • Social/ mood/ lifestyle/ impact/ sleep/ stress • Rd flags: thoughts of self-harm/ cardiac chest pain/ SOB/ palpitations Cues not to miss: • Stressful work/ break down in the morning ICE: • Ideas: chest pain = heart problem • Concern: heart attack • Expectation: reassurance Management vital points • Address concern and explain stress = body in fight or flight response for longer period cause body exhaustion and physical symptoms • Non medication: sleep hygiene/ mindfulness/ breathing techniques/ exercise/ avoid alcohol/ smoking/ drugs/ caffeine • Possible referral and signposting to counselling session/ online resources Carer case • Cue is “extreme measures” – giving mum more sleeping pills • Clarify risks of increasing pills for her mum + offer alternatives for her agitation by offering home visit/ review with specialist/ excluding infection + offer caregiver support services • Exclude DDx for fatigue and red flags by examination and invx • Safeguarding concern: ensure safety of mum – refer to safeguarding team
55 M + requesting more sleeping pills = insomnia	Qs not to miss: • Sleeping problem/ sleep hygiene/ lifestyle/ mood/ phone usage/ diet • Social/ work/ drugs/ physical problem like pain Cues not to miss: • Pushing for pills/ downplay potential addiction/ Hx of OSA ICE: • Ideas: work stress causing sleeping problems • Concern: lack of sleep can affect work • Expectation: sleeping pills – aware of addictive nature Management vital points • Zopiclone: Short-term use only 3-7 days ; caution with sleep apnoea and risk of dependency specially with stress. • Focus: Address root causes like stress – stress management as above case • Sleep Hygiene: Give leaflet: Avoid screens before bed at least for 1 hour before sleeping, set a consistent routine, and try relaxing activities. • Lifestyle: Limit afternoon caffeine, maintain a regular sleep schedule, and include physical activity. • Manage comorbidities: OSA/ pain • Safety: driving/ time off work • If no improvement or symptoms > 3 months: consider CBT/ referral to sleep clinic/ melatonin for pt> 55 years + regular follow ups

77 M + requesting more pain killer (tramadol) for his OA = carer support	Qs not to miss: • Social Hx/ previous medications/ previous plans • Triggers for the flare up/ support/ impact • Caring for partner impact (emotional/ physical/ financial/ self-neglect/ isolation) • Red flags: suicidal thoughts – medication side effects Cues not to miss: • Not have chance to rest it/ Struggle to come to surgery ICE: • Ideas: stronger pain killer – can't do operation because caring for his wife • Concern: GP not prescribing meds/ leaving partner if need to have operation • Expectation: prescribe tramadol Management vital points • Address personal needed: taking care of yourself is important for you and your partner to be able to be more effective in your caregiving role. Ask: what happen if you become unwell? To challenge perception of not needing carer. • Refer to social prescriber for advice about carer/ financial issues/ support • For partner while having his operation: day care or short-term respite/home care • Offer his medication in the meantime
54 M + struggling with PTSD requesting higher dose of meds = PTSD	Qs not to miss: • Impact on life and work • Red flags: suicidal thoughts/ psychosis Cues not to miss: • Financial difficulties ICE: • Ideas: worse because venlafaxine dose reduction • Concern: worsening symptoms • Expectation: higher dose and referral to therapy Management vital points • Offer referral to 2ry care for Trauma-Focused Cognitive Behavioural Therapy (CBT)/ Eye Movement Desensitization and Reprocessing (EMDR) up to 12 sessions • Financial and Practical Support: refer to social prescriber and care coordinator • Signpost to resources e.g. Combat Stress/ Veterans UK • Increase medication dose and review in 2 weeks
19 M + compulsion to organize workplace = OCD	Qs not to miss: • Triggers/ check of insight • Do you have persistent, unwanted thoughts or images that you can't shake off? • Are you compelled to perform certain actions repeatedly to feel better? • Do these thoughts or actions consume significant time or disrupt your life? • Red flags: suicidal thoughts/ psychosis/ severe depression/ anorexia nervosa/ self-neglect Cues not to miss: • Patient has VW disease – CI with SSRI! ICE: • Ideas: OCD • Concern: impact on life/ stigma of mental health • Expectation: rapid help with meds better than counselling/ letter to university Management vital points • Explain OCD as when a person has certain thoughts repeatedly (obsessions) and feels the need to do certain actions (compulsions) over and over. • 1st line: CBT with exposure and response prevention • 2nd line SSRI e.g. escitalopram – if contraindicated, then clomipramine • Follow up every 2-4 weeks + support letter to university + signposting to support groups + lifestyle changes and stress management • Referral if red flags or < 18 years or no response to above plan

78 M + visual hallucination of seeing his wife who recently passed away = Bereavement

Qs not to miss:

- Insight
- Impact on life/ appetite/ sleep/ mood
- Loneliness/ support/bereavement/ isolation/guilt
- Red flags: suicidal thoughts/ prolonged >6months/ complicated with depression

Cues not to miss:

- Ask about his wife

ICE:

- Ideas: Worried about going mad
- Concern: For how long will be like this?
- Expectation: help from the doctor

Management vital points

- Validating patient feeling as normal part of grief/ bereavement which is the experience we go through after losing someone important to us.
- Signpost to bereavement services e.g. Cruse Bereavement Care
- Advise about self-care with sleep hygiene/ lifestyle changes
- Follow up and safety netting if red flags
- Social prescriber referral for loneliness for support groups
- If symptoms > 6months, refer to local older adult mental health team for possible CBT and SSRI

34 M + requesting more meds + rejected and asked to speak to GP = diazepam addiction

Qs not to miss:

- Hx of anxiety and depression + impact on life
- Mood/sleep/energy/appetite
- Hx of diazepam/ other meds/ non medication options
- RFs: lifestyle/ stresses/work

Cues not to miss:

- Offer alternatives early (pt will be angry and demanding)/ anxiety getting worse

ICE:

- Ideas: Diazepam is the only meds working for my anxiety
- Concern: GP wants to stop diazepam
- Expectation: New prescription (very defensive)

Management vital points

- Address challenges of managing anxiety and discuss side effects of diazepam, e.g. addiction and dependence (withdrawal symptoms if wants to stop)/ tolerance (higher doses needed with time) / affect memory/ concentration and coordination. It can also cause depression and emotional blunting. Risk of falls/ accidents/ respiratory depression
- Offer alternatives like CBT (explain)/ increase dose of SSRI
- Plan to gradually stop by 1mg per month (6 months for this patient)

19 F with visible bruises + feeling low and anxious = Forced marriage

Qs not to miss:

- Red flags: safeguarding/ abuse/ suicidal thoughts
- Abuse: emotional/ financial/ sexual
- Social Hx and support/ lifestyle/ impact on life/ sleep/ appetite/ relationship
- Assess safety at home and coping mechanism

Cues not to miss:

- Bruises on arms/ looks distressed

ICE:

- Ideas: something wrong as can't stop worrying
- Concern: afraid of the consequences of speaking about bruises/ forced marriage/ impact on study and life
- Expectation: support with anxiety/ help to handle this forced marriage

Management vital points

- Explain forced marriage is not = arranged marriage where both parties consent to the match. It is serious human rights violation, and it is illegal.
- Immediate referral to safeguarding services to ensure safety
- Safety/crisis planning safe place to go and how to contact emergency services
- Mental health support with therapy +/- antidepressants
- Legal support: forced marriage unit for forced marriage protection order and support services
- Police if in emergency
- Follow up to ensure safety

9 M + mum concerned about behaviour changes with normal blood results difficult behaviour	Qs not to miss: • Red flags: suicidal thoughts • DDx: ADHD (inattention/ hyperactivity)/ autism (repetitive pattern)/physical problems (hearing/ vision/ DM/ speech) • Social Hx and environment at home • RFs: parental stress/past response (e.g. expecting rewards for all compliance)/ attention seeking/ life changes Cues not to miss: • Normal behaviour at school ICE: • Ideas: blood sugar related problem • Concern: blood sugar • Expectation: Help or medications to help Management vital points • Reassure given normal bloods and assessment at school, unlikely to be physical related problem • Consider help from a paediatrics mental health team (CAHMS) for therapy • In the meantime, given leaflet about some tips: tailored approach, calmness. Good communication with example, focus on positive, avoid smacking – be consistent – be model for them – be supportive
45 F + raising concerns about colleague doctor	Qs not to miss: • Ensure safety of patient and other patients • Type of concern: neglect/abuse • Past experience with the doctor both professionally and socially • Red flags: safety/ abuse Cues not to miss: • Confidentiality ICE: • Ideas: To get your worries off your chest • Concern: concern about colleague fitness to practice/ consequences about confidentiality/ about documentation • Expectation: practice to investigate this concern Management vital points • Ensure confidentiality (no need to document within pt medical records = just write in a piece of paper and escalate • Arrange confidential meetings with senior staff • Ensure safety of patient by monitoring colleague documentation and arrange feedback from other patients • Offer support to colleague vis occupational health to address stress • Safety netting ti raise concerns if further safety issues
25 F + staff complaining about PM = staff as a patient	Qs not to miss: • Impact on life and work • Red flags: suicidal thoughts • Alternative work options (in a nice way) Cues not to miss: • Confidentiality/ conflict of interest ICE: • Ideas: stress due to bullying at work • Concern: financial issues if been fired • Expectation: sick note/ speak on her behalf and pass all concern to partners Management vital points • Ensure confidentiality and good documentation • Avoid being judgemental and maintain objectivity and be neutral • Decline speaking on her behalf and advise formal complaint • Signpost to occupational health/HR and legal advisors • Medically, signpost to support groups/ CBT/ discuss medications • Sick Leave: Issue a fit note for medical leave or amended duties without delving into workplace specifics. • Follow-up Care: Arrange follow-ups to monitor health and consider discussing a potential practice transfer if it helps the staff member feel more comfortable. • Support work environment

28 M + gambling addiction = gambling disorder	Qs not to miss: - Red flags: financial harms/ suicidal thoughts/neglect/abuse/safeguarding - Triggers/RF: FH, environmental factors, drug abuse, mental health problems including bipolar, financial problems - Problem Gambling Severity Index (PGSI) Qs (Do u bet more than you can afford to lose? Do you needed to gamble with larger amounts of money to get the same feeling of excitement? Have you borrowed money or sold anything to get money to gamble) Cues not to miss: - Good insight and good support ICE: - Ideas: gambling disorder? ADHD related - Concern: worsening financial situation - Expectation: medication (Naltrexone)/ letter to speed ADHD assessment Management vital points - Explain gambling disorder as chronic addictive condition make it difficult for individuals to control gambling behaviour with its financial impacts - Refer for CBT and then for Naltrexone if not helpful - Signpost to StepChange/ Gambler's anonymous for 12-srep program - Expedite ADHD referral or explore right to choose pathway as this helps
53 Y + fall after tripping due hoarding behaviour = Hoarding disorder	Qs not to miss: - Behaviour / like to keep things/ insight - Impact on life and relationship - Screen for depression/ anxiety/ ADHD/ OCDS/ physical health/ cognitive issues Cues not to miss: - Convinced by sister to come ICE: - Ideas: no idea about why he is doing such behaviour - Concern: losing independence/ get rid of possessions - Expectation: how to manage/ organizations to keep things safer Management vital points - Explain hoarding as difficulty to throw away possessions regardless of actual value and it is a treatable mental health problem - Refer for CBT/ Motivational interview - Consider meds if coexisting depression/anxiety/ OCDS - Regular checkup with social services involved - Signpost to Hoarding UK/ Clouds End CIC to help decluttering and Age UK to ensure safety
29 M + chronic tiredness + normal blood tests= chronic fatigue syndrome	Qs not to miss: - To Dx 3 of the followings for at least 3 months (debilitating fatigue/ disproportional post exercise fatigue/ unrefreshing sleep or sleeping disturbance / cognitive difficulties or brain fog) - DDx: anaemia/sleep apnoea/ thyroid/ DM/ depression/ post COVID - Red flags: weight loss/neurological symptoms/ no lumps or bumps/ Addison's disease (dehydration/ pigmentation) - Impact on work Cues not to miss: - Might be out for days after doing strenuous exercises/ very unrefreshed after sleep/ fatigue not relieved by test/ caused by normal effort ICE: - Ideas: thyroid problem? - Concern: no quick fix? Financial problems - Expectation: quick solution/ iron or thyroid tablets even normal bloods Management vital points - Explain CFS as Dx of exclusion/ manageable disease with fluctuating nature - Invx to exclude DDx - Refer to CFS clinic for tailored advice and interventions (physio/psych/occupational therapy) - Lifestyle: sleep hygiene/ balanced diet/gentle physical exercise/ avoid alcohol and smoking/ hydration) - Psychological support and energy management (be realistic about expectations)

MSK

31 Y + Knee pain = meniscal injury

Qs not to miss:

- Causes: Trauma/ infection/inflammation/ Mechanism of injury
- Red flags: locked knee/ fever/ redness/ unable to bear weight/ rupture cyst

Cues not to miss:

- Professional sportsman
- Twisting injury/ swelling returned /mild swelling /give away/ +ve McMurray's test

ICE:

- Ideas: sprain
- Concern: stopping badminton is not an option
- Expectation: stronger painkiller/ x-ray

Management vital points

- Conservative 1st: RICE – rest/ ice/compression/ elevation for the 1st 48 hours + NSAIDs + physiotherapy to strengthen muscles
- If no improvement after 6 weeks of physio= refer to orthopaedics routinely for MRI - Urgent referral if locking in MRI
- Follow up and safety netting

28 F + foot pain in the insertion of the planter fascia requesting steroids injection = Planter fasciitis

Qs not to miss:

- Impact on life and work
- RFs: weight/ footwear/ foot problems/ more weight bearing

Cues not to miss:

- Marathon in 4 weeks

ICE:

- Ideas: overtraining
- Concern: next marathon
- Expectation: rapid recovery with steroid injection

Management vital points

- Explain planter fasciitis (tissue that support arch of your foot and absorb shock while walking) and it can up to 18 months for full recovery
- 1st line is conservative with: resting the foot + supportive footwear to support arch of foot + insoles to correct foot pronation + losing weight + pain relief with simple analgesia + NSAIDs + cold therapy for 15-20min few times a day + stretching exercises (leaflet)
- 2nd line steroid injection (USS guided) : discuss risks and possible waiting time – temporary relief and high recurrence of symptoms. Risks > benefits
- Offer private clinic for injection if essential
- Refer for podiatry and physio if no better in few months then ortho team referral for shockwave therapy and possible surgical options

**38 F + foot pain +
stiff ankle =
Achillis
tendinopathy**

Qs not to miss:

- DDx planter fasciitis/gout
- Red flags: non weight bearing/ trauma/ fever /sudden onset severe pain (rupture)
- Impact on work
- RFs: quinolones abx/ hypercholesteremia/ DM

Cues not to miss:

- Ankle stiff to dorsiflex/ lump on the heel/ work as a police officer

ICE:

- Ideas: broken bone in feet?
- Concern: job security/ time off work/ coming holiday
- Expectation: x-ray and specialist referral

Management vital points

- Explain Dx (inflammation and stiffness of the strap connecting your heel bone to your calf muscles which usually go within 3-6 months)
- Cold packs 15 min several times a day + analgesia
- Rest and avoid activities that worsen the pain then reintroduce gradually + weight bearing exercise
- Refer to physio if no better in 7 days. Early physio is crucial for specific exercise like eccentric loading exercise (+ provide leaflet about it)
- Avoid injecting steroids as increased risk of tendon rupture
- Footwear modification and orthotics to alleviate pressure on the tendon
- Next step is to refer to orthopaedics if no better for shock therapy or surgery
- Workplace adjustment including occupational health

**51 F + spinal
fracture after fall at
home + letter from
ortho requesting
DEXA scan =
fragility fracture**

Qs not to miss:

- Hx of fractures/ mechanism of fracture/ trauma
- RFs: thyroid problems/ smoking/ medications including steroids/ menopause/ obesity
- Impact on work and life/ work circumstances/ sick note discussion
- Red flags: cauda equina syndrome/ weight loss/ bone pain

Cues not to miss:

- Reluctant to give sick note to work/ want just 2 weeks sick note
- Fall down a flight of stairs

ICE:

- Ideas: DEXA scan / go back to work asap
- Concern: financial problem with the 6 weeks sick note/ fracture again
- Expectation: DEXA scan + fit to work note before the 6 weeks

Management vital points

- Explain osteoporosis/ DEXA scan / fragility fracture
- Discuss fit note (option of amended duties or phased return to work or go private for fit to work fit note)
- Preventing fracture with lifestyle changes (weight bearing exercise/ weight loss/ stop smoking)/ HRT/ Vitamin D and calcium
- Start bisphosphonate even before DEXA scan + arrange DEXA scan
- Refer to physiotherapy to speed up the recovery
- Safety netting: fracture/ cauda equina syndrome/ weight loss

33 F + learning disability + hip fracture = iatrogenic fracture	Qs not to miss: • Impact on life and work • Social life and sexual Hx • Ask in a simple way: x-ray = pictures of the bones • Red flags: fever/ severe pain/ unable to bear weight/ trauma/ safeguarding Cues not to miss: • Patient has LD • Sudden onset/ no external rotation/ not able to bear weight/ on Na valproate and depo contraception injection/ crutches/ limping ICE: • Ideas: sprained hip? • Concern: tear in hip? How long to go back to dancing? Scan? Painkillers with epilepsy medications • Expectation: scan/ pain killers Management vital points • Explain possibility of fragility fracture/osteoporosis in simple terms • Refer to A&E for x-ray – reassure • Discuss impact on holiday – may need to cancel with supporting letter • Discuss Na valproate side effects on bone and write to specialist about alternative and risk of pregnancy on this teratogenic effect. • Follow up to discuss contraception • Offer fit note
54 F + pain in arm and hand going to hand = CTS	Qs not to miss: • Impact on activities and work • DDx hypothyroidism (weight gain/ fatigue/ dry hair and skin) • RFs: working with vibrating hand tools/ repetitive hand activities/ FH • Red flags: thenar atrophy/ severe pain/ grip weakness = refer Cues not to miss: • +ve Phalen's test + altered sensation in thumb and index + obese ICE: • Ideas: no idea • Concern: knitting course/how long? • Expectation: physio/ x-ray Management vital points • Explain Dx as compression of a nerve that runs under a thick tissue in the wrist • Lifestyle changes: weight loss/ avoid activities that make it worse • Offer splint to support the position + physio + offer steroid injection • Possible nerve conduction studies + expected recovery in 3-6 months
46 F + pain in wrist and hand + normal x-ray = wrist tendinitis	Qs not to miss: • DDx: swelling/ redness/ colour changes/ stiffness • Impact on work • Red flags: redness/ weakness/ swelling/ severe pain/ neck pain • RFs: using hand a lot in work Cues not to miss: • Pain more at the end of the day and in the morning with no stiffness or swelling • Pressure around the wrist make it better / work as massage therapist ICE: • Ideas: wear and tear? / HRT related? • Concern: financial problems/ rheumatism as mum • Expectation: painkillers/ referral? Management vital points • Explain x-ray not showing osteoarthritis/ it is not RA • Explain tendonitis as inflammation of the tendons connecting muscle to bone • Analgesia/ ice therapy/ physiotherapy/ not putting much strain on wrist • Support wrist with strap/ splint • Steroid injection for severe pain but short-lived action • Acupuncture and other alternative option can be helpful but no much evidence

48 F + Painful swollen hands + positive RF and high CRP = Rheumatoid arthritis	Qs not to miss: • Impact on life and hobbies/ work • DDx psoriasis – ask about rashes • Extraarticular/systemic symptoms: eyes/vasculitis or fever/ fatigue/ weight loss/ rheumatoid nodules • Red flags: red/swollen/ warm joint/ fever/ eye symptoms • Suspect RA if persistent synovitis for weeks (not days) + small joints + stiffness + swelling + symmetrical Cues not to miss: • Stiff hands for about 50 minutes • Breaking bad news ICE: • Ideas: allergy? • Concern: serious problem? RA? Will affect her not to play piano • Expectation: some tablets/ reassurance Management vital points • Explain RA: as inflammation in joints as the body attacks itself +reassure that there are some treatments to control it and avoid complications to the joints • Refer urgently within 3 weeks to rheumatology if suspecting even before bloods • Refer within 3 days if (small joints of hand or feet are affected/ > 1 joint/ delay of seeking advice > 3 months • Offer NSAIDs + PPI cover but avoid steroids not to mask symptoms • Consider x-ray and anti-CCP
26 F + bilateral wrist pain + recent delivery = De - Quervain's tenosynovitis	Qs not to miss: • Impact on daily activities/ Screen for mental health issues • Red flags: redness/ weakness/ swelling/ severe pain/ weight loss/ fever Cues not to miss: • First time mum/ worse since partner went back to work • Pain in the area behind the thumb on the wrist itself • +ve Finkelstein test ICE: • Ideas: lifting baby? RA? • Concern: carrying baby/ RA • Expectation: treatment/ exercise Management vital points • Explain Dx as inflammation of the tendon around the base of the thumb which is common in new mum due to caring for the new baby • Recommend thumb splint + avoid activities exacerbating the pain • Refer to physio for specific exercises (graded exercise therapy) • NSAIDs + steroids injection if severe then refer if no response • Refer to new mum groups and social prescriber for more support
31 F+ discoloration and pain in the fingers of the hands = Raynaud's	Qs not to miss: • Triggers: cold/ smoking/ caffeine/ stress • Red flags: joint involvement/ severe pain/ Hx of thrombosis/ ulcers/ black color Cues not to miss: • Affecting his painting • Fingers turn pale, then blue then red on exposure to cold ICE: • Ideas: related to cold • Concern: ability to paint • Expectation: Dx and help Management vital points • Explain Dx as a condition of the small blood vessels in the finger and toes that overreact to the cold or stress causing narrowing and limit blood flow • Arrange bloods to exclude 2ry causes: FBC/ANA/ ESR • Avoid triggers as above + keep warm with gloves/ hand warmers / avoid sudden temp changes + exercise regularly • Trial of CCBs if needed with nifedipine + explain about side effects and can be used as short term when cold weather • Refer if suspected 2ry causes or red flags

**38 F + elbow pain
worse on
movement = tennis
elbow**

Qs not to miss:

- Red flags: fever/ swelling/ stiffness/ multiple joints + abdominal pain/ vomiting blood or black stool

Cues not to miss:

- Tender lateral epicondyle
- Work in Morrisons with repetitive scanning in the till
- Taking much ibuprofen with hx of gastric ulcers

ICE:

- Ideas: pulled muscle in the arm
- Concern: impact on work
- Expectation: stronger pain killer (Naproxen)

Management vital points

- Explain Dx as inflammation of the outside of the elbow especially the strap connecting muscle to the bone due to overuse
- Avoid movement that make it worse/ amended duties sick note
- Avoid oral NSAIDs due to gastric ulcer Hx + offer topical NSAIDs or codeine
- Forearm support/band + Offer physiotherapy then steroid injections if not helpful
- Referral if no better after 6 months or worsening symptoms or uncertain Dx

**45 F + shoulder
pain + moving to
new house =
rotator cuff
disorders**

Qs not to miss:

- Red flags: neck pain/ weakness/ sudden severe pain/ fever/ swelling/ mass/ multiple joints/ suspected dislocation
- Impact on daily life/work

Cues not to miss:

- Limited arm abduction >60 degree + positive Hawkin's test
- Moving to new house and caring for partner

ICE:

- Ideas: pulled muscle in shoulder
- Concern: to get better within 5 days before back to work
- Expectation: steroid injection

Management vital points

- Explain Dx as inflammation of the strap that attached from shoulder to arm
- Avoid movement that make it worse/ amended duties sick note
- Analgesia + PPI cover
- Physio + exercise (ARC shoulder exercise)
- Consider steroids if severe symptoms or after 6 weeks if no improvement – improvement can take up to 1 week – mask pain not treating the problem – it works as anti-inflammatory so it will help to engage with physio – risk of recurrence after 12 months
- Rest/ amended duties sick note
- Refer if severe pain/ sudden pain/ not improving after 6 weeks/ suspected tear

68 M + A&E letter about sudden muscle pain and stiffness + high CRP/ESR = Polymyalgia Rheumatica

Qs not to miss:

- Red flags: weight loss/ joint inflammation/ vision problem/ headaches/ scalp tenderness/ jaw claudication (GCA)
- Impact on daily life
- Suspect if (bilateral aching and stiffness > 45 min for > 2 weeks in > 50 years)
- DDx: weight loss/peripheral joint pain and swelling/ fibromyalgia

Cues not to miss:

- Stiff shoulder/neck and hips more in the morning + stiffness > 1 hour
- Struggle getting up from setting / tearful due to impact of the problem

ICE:

- Ideas: worsening osteoarthritis/ RA?
- Concern: losing independence
- Expectation: medication to return to normal life

Management vital points

- Explain Dx as inflammatory condition affecting muscles not caused by anything you've done, and it can be effectively treated with steroids
- Consider invx to exclude DDx (RA/hypothyroidism/ myeloma)
- Start daily prednisolone 15mg + gastric protection + bone protection
- Follow up then tritrate dose based on response every 4-6 weeks
- Regular bloods tests FBC/LFTs/ U&Es/ ESR
- Refer to rheumatology + social prescriber referral for mental health
- Lifestyle changes: stop smoking/ weight bearing exercise/ balanced diet

76 M + hip pain = Osteo-arthritis

Qs not to miss:

- Red flags: severe back pain/ weakness/ joint swelling or stiffness > 1 hour/ fever/ weight loss/ saddle paraesthesia/ bowel or bladder changes/ unable to bear wt
- Impact on life/ Hx of cancer

Cues not to miss:

- Morning stiffness 20 min/pain more in the end of the day/ better with rest/ grating feeling when walking

ICE:

- Ideas: wear and tear/ need hip replacement
- Concern: impact on mobility
- Expectation: referral to ortho team

Management vital points

- Explain Dx as wear and tear of the joint with age and use of the joint
- Pain management with paracetamol or codeine (not NSAIDs as pt on warfarin)- no benefit from topical NSAIDs (can be use with knee and thumb OA)
- Physio for exercise for 6 weeks and use of insoles as 1st line then refer to ortho
- Consider social prescribe and occupational health referral (carer?)
- If suspecting fracture – arrange x-ray

44 M + big toe pain + tophi in ears = acute gout

Qs not to miss:

- Red flags: fever/ colour changes/ feeling unwell/ multiple joints
- DDx: septic arthritis/ OA/ RA
- RFs: diet (high seafood/ red meat)/ medications/ excess alcohol/ FH
- Impact on patient and work

Cues not to miss:

- Patient on thiazides / aspirin+ pain improved with ibuprofen

ICE:

- Ideas: no idea
- Concern: what to do
- Expectation: medications

Management vital points

- Explain Dx as joint inflammation due to uric acid deposit in the joints
- Discuss RFs and how to avoid: less alcohol/ change diet/ change medications
- Bloods: uric acid/ U&Es, HbA1C and lipid profile after 4-6 weeks
- Rest + elevate legs + ice packs + weight loss + avoid dehydration + stop smoking + regular exercise + limit sugary + don't stop preventive medications
- Flare up: Medications (NSAIDs or colchicine or steroids)
- Offer long term preventive therapy if tophi/ > 1 attack in 12 months/ joint damage/ renal impairment/ long term diuretics/ uric acid stones with allopurinol or febuxostat after 1-2 weeks of flare up aiming uric acid <300 (do every 6 weeks)

52 M + ankle pain + high uric acid = gout

15 F + back pain after trampoline injury = mechanical back pain

Qs not to miss:

- Red flags: weakness in legs/ radiating pain/ problems in urinating or evacuation of bowels/ saddle paraesthesia/severe back pain/weight loss/fever/ recent travel
- RFs: heavy lifting/ work/ position/ injury

Cues not to miss:

- Normal examination – stiffness < 15 mins

ICE:

- Ideas: Scheuermann's disease?
- Concern: impact on coming Rugby match/ serious problem?
- Expectation: imaging with x-ray or MRI/ blood tests

Management vital points

- Explain mechanical back pain as pain comes from the way your muscle, joints and spine move and work due to strain of the muscles due to the back injury
- Light mobilization/ no heavy lifting/ continue with usual daily activities
- Analgesics with paracetamol/ NSAIDs
- Address possible difficulty in playing next match due to possible worsening of the injury + need for physio for enhanced recovery
- Explain Scheuermann's disease as a common disease of the thoracic vertebrae lead to increased kyphosis, poor posture and back pain

44 M + recurrent back pain + positive straight leg test = sciatica

Qs not to miss:

- Red flags: weakness in legs/ radiating pain/ problems in urinating or evacuation of bowels/ loss of perianal sensation /severe back pain/weight loss/fever/ recent travel/ night sweats/ hx of cancer
- Impact on work
- RFs: heavy lifting/ work/ smoking (delay healing)

Cues not to miss:

- Obtained tramadol from friend/ not believe in physio

ICE:

- Ideas: torn muscles?
- Concern: painkillers not strong enough
- Expectation: tramadol/ sick note for few weeks

Management vital points

- Explain Dx as irritation of the nerve that runs from the spine to your legs can be caused by swollen muscle/ ligament/ slipped disc
- If any red flags – urgent referral
- If no red flags, explain recovery can take few weeks to months + manage conservatively (stay active with simple exercise e.g. walking and swimming + using heat/ cold pads + avoid prolonged rest)
- Analgesia with simple options or codeine
- Work: Self-certifying up to week then amended duties if needed
- Refer to physio if persistent pain for 2-3 weeks, then for MSK clinic if no better for possible MRI (delay exercise for few weeks)
- Discuss tramadol side effects + offer alternatives (codeine)

45 M + wants to complaint + MRI report showing disc prolapse = managing angry patient with sciatica

Qs not to miss:

- Red flags: weakness in legs/ radiating pain/ problems in urinating or evacuation of bowels/ loss of perianal sensation /severe back pain/weight loss/fever/ recent travel/ night sweats/ hx of cancer
- RFs: heavy lifting/ work/ smoking (delay healing)

Cues not to miss:

- Reception told patient just wear and tear/ GP filled as nerve impingement due to wear and tear/ MRI report as degenerative changes with disc bulging
- Want to make complaint (Angry patient)

ICE:

- Ideas: shouldn't receive wrong dx by receptionist/ delay in treatment
- Concern: incorrect Dx received lead to doing exercises that make pain worse
- Expectation: to be referred to specialist for fixing back

Management vital points

- Explain slipped disc: the inside part of the disc which is soft bulges out through the harder outside part and can press down the nerve that runs past it which give symptoms called sciatica and can be treated with painkillers and physio and rarely needed surgery. Discuss red flags and when to seek urgent care
- Discuss management options as above case
- Dealing with complaint with apology and explain complaint procedure: 1st to do it by writing or by phone to PM and then will be acknowledged withing 48 hours with a written response within 10 days
- Dealing with angry patient tips: acknowledge/ avoid argument/ apology for the experience/ don't criticize others/ offer solutions or explain complaint steps.

32 F + back pain after lifting elderly resident = safeguarding

Qs not to miss:

- Red flags: weakness in legs/ radiating pain/ problems in urinating or evacuation of bowels/ loss of perianal sensation /severe back pain/weight loss/fever/ recent travel/ night sweats/ hx of cancer
- RFs: heavy lifting/ work/ position/ injury
- Safety of residents
- Impact on work and life

Cues not to miss:

- Lifted elderly resident off the floor at work/ manager pushing for additional work
- Elderly left on the floor for extended hours

ICE:

- Ideas: injured back as forced to do tasks not trained to do
- Concern: back gets worse? Affording off work/ quality of care for residents
- Expectation: pain killers/ fit note with amended duties

Management vital points

- Explain Dx of mechanical back pain + manage as case 94 + fit note with amended duties + refer to occupational health services to recommend workplace necessary adjustments
- Discuss workplace safety concerns then discuss with safeguarding team for advice about further steps
- Educate patient about whistleblowing rights and encourage speaking up with the manager about her concerns and then to the CQC if still ongoing issues + reassure about her protection under the Public Interest Disclosure Act
- Ensure confidentiality and support

27 F + Hx of fracture radius + Hx of lacerations + Hx of bruises in different areas + requesting pain killer for arm = Domestic abuse

Qs not to miss:

- Red flags: bleeding from ears/ urine/ mucus/ stool/ suicidal thoughts/ children safeguarding concerns
- RFs: smoking/ alcohol/ drugs/ financial hardships
- Impact on life /mental health/ children
- Assess patient/ daughter safety

Cues not to miss:

- Patient sitting with head down + given minimal answers

ICE:

- Ideas: painkillers
- Concern: doesn't want any of this getting back to partner
- Expectation: painkillers and left alone

Management vital points

- Explain Dx: what you mentioned is called domestic abuse and it is not the normal. it sounds like you may not be safe, and I am concerned about your safety and your daughter. You should be treated with respect and dignity.
- Explain support available for the patient: Fortalice/ Women's Aid
- Reassure about confidentiality even they don't want to take actions except if there are children involved when we should ensure children safety
- Pain control with low dose codeine as other simple options not working
- Discuss safety plan: safe place to escape to/ emergency number to contact
- Refer to social prescriber team and mental health team if needed
- Refer to social services to ensure child safety + reassure patient that this is not to remove children from you but just to provide extra support

38 F + letter from rheumatology to discuss Ehler Danlos S = Ehler Danlos S

Qs not to miss:

- Impact of this diagnosis on her/ questions to ask?
- Red flags: mental health crisis/ septic arthritis symptoms/ dislocations

Cues not to miss:

- Impact on work as teacher

ICE:

- Ideas: flare up of EDS caused by increase work pressure
- Concern: worsening pain might cause her to take time off work
- Expectation: help for pain relief and letter to school

Management vital points

- Explain EDS as group of disorders affecting connective tissue in body that support joint, blood vessels and sin causing overly flexible joint/ fragile skin causing joint pain/ dislocations/ bruises/ muscle fatigue/ Mitral valve prolapse
- Explain no cure and management with low impact exercise (swimming/ walking) and avoid high impact exercise (running) or high stress exercise (weightlifting) or contact sports + rest your jaw (avoid chewing gums or hard foods) + wear supportive shoes + avoid overstretching + energy conservative techniques to avoid fatigue + mindfulness and stress reduction + ensure hydration and balanced diet
- Refer to physiotherapy to guide her exercise program
- Fit note or amended duties fit note or letter to support her
- Signposting to support group e.g. Ehler Danlos Society for support

57 F with multiple joint arthritis + on different opioids + asking for higher doses= Chronic pain management

Qs not to miss:

- Impact of pain
- Red flags: suicidal thoughts/ cauda equina syndrome/ weight loss/ weakness
- Social history/ mood

Cues not to miss:

- Note that pain clinic wants to stop meds
- They don't understand how much pain you're in

ICE:

- Ideas: want to get rid of pain + able to mobilize + worry about physio make thing worse
- Concern: pain worse as times goes/ addicted and tolerant to painkillers/ more sleepy with painkillers
- Expectation: increase dose of painkillers or arrange pain-killing injections

Management vital points

- Acknowledge that patient is suffering and how distressing is chronic pain
- Explain Dx of chronic pain: when we have pain for longer time, our pain system works in different way than when we have acute pain when our nervous become very sensitive even without further damage and resting doesn't help the pain.
- Recommend referral to physio for supervised exercise program
- Discuss medications: starting by explaining that current medications not working and side effects can be lethal and affect QOL. Then discuss stopping opioids and gabapentin gradually
- NSAIDs/ paracetamol/opioids/ steroids injection don't work with chronic pain
- Consider off licensed antidepressant e.g. duloxetine/ citalopram/ fluoxetine if not already taking them even not depressed to improve QOL e.g. sleeping
- Refer to support groups/ social prescribers for more support
- Refer to pain MDT clinic / CBT/ consider acupuncture

32 F + pregnant + restless leg = Restless leg in pregnancy

Qs not to miss:

- Impact on life and sleep
- RFs: caffeine/ medications/ alcohol/ dehydration/ less exercise
- Red flags: muscle weakness/ sensory problems/ signs of DVT/ back pain with CES/ lack of iron/ sugary food/ stress

Cues not to miss:

- Co-codamol for pelvic pain

ICE:

- Ideas: restless leg due to pregnancy – prefer to cut off her legs
- Concern: impact of sleep/ concerned about taking medications with pregnancy/ anxious about ability to work till 39 weeks to maximize maternal leave
- Expectation: safe medications to help with the problem and sleep

Management vital points

- Explain Dx as a common condition during pregnancy causes uncomfortable sensation in legs causing urge to move more at night.
- Discuss RFs and managing it to help: reduce caffeine intake (no more than 2 cups/ sleep hygiene/ moderate exercise/ adequate fluid intake/ diet rich in diet/ stress management
- Invx to check iron/ magnesium and treat if anaemia with supplements
- Refer if severe symptoms to specialist as medications for RLS are contraindicated
- Address safe prescription in pregnancy so need to stop codeine
- Discuss maternity leave: options are adjusting work schedule/ starting maternity leave earlier/ or write a letter to employer

**45 M + chest pain
during weightlifting
= Costo-
chondritis**

Qs not to miss:

- Red flags: central crushing pain/ radiating to jaw or neck/ associated with SOB or palpitations or dizziness/ calf muscle pain
- Explore chest pain (nature/ duration/ site/ triggers/ associated symptoms)
- RFs: smoking/ FH/ unhealthy diet/ lack of exercise/ stress/ comorbidities
- DDx: ACS/ PE/ pneumothorax/ trauma/ reflux

Cues not to miss:

- FH of heart attack at 62

ICE:

- Ideas: heart disease or impending heart attack
- Concern: having heart condition that can affect his work/ ability to do exercises
- Expectation: investigations/ ECG/ referral

Management vital points

- Explain Dx as inflammation of the cartilage connecting your ribs to your breastbone which is worse with certain movement such as weightlifting exercise
- Invite for examination
- Analgesia with OTC NSAIDs
- Refer to physio if no improvement
- Lifestyle changes to decrease risk of heart attacks
- Good safety netting advice

Gastro

61 F + long hx of reflux requesting camera test = iatrogenic dyspepsia

Qs not to miss:

- Red flags: weight loss/ melena/ odynophagia or dysphagia/ hematemesis/ change of voice/ food stuck in oesophagus
- Impact on life
- RFs: smoking/ diet/ alcohol/ medications/ NSAIDs/ Stress

Cues not to miss:

- Back pain taking aspirin from friend and amitriptyline
- Brother died from oesophageal cancer

ICE:

- Ideas: worsening reflux
- Concern: oesophageal cancer
- Expectation: camera test

Management vital points

- Explain Dx as range of symptoms (discomfort/ bloating/burning/ fullness) in the upper part of the digestive system due to triggers or causes
- Discontinue aspirin + reduce dose of amitriptyline + offer alternative
- Testing for H. Pylori after stopping PPI for 2 weeks
- Lifestyle advice about stopping smoking/alcohol/ caffeine/ spicy or fatty food
- Follow up if symptoms persist and negative H. Pylori then refer to camera test
- Consider psychological support given health anxiety

Osteoporosis case:

- Cue: compliance with the medication's instructions + low osteoporotic FRAX score 9% but patient just wanted to take it as preventive measures
- Recommend stopping alendronate + advise conservative management as 81
- Plan: PPI + camera test if no better in 4 weeks

65 F + Hx of osteoporosis and on alendronate + heart burn = iatrogenic dyspepsia

22 M + requesting test as dad got colon cancer = familial adenomatous polyposis

Qs not to miss:

- Red flags: weight loss/ melena/ abdominal mass/ change in bowel habits
- Impact on life and work

Cues not to miss:

- Anxious and worried
- AFP is autosomal dominant

ICE:

- Ideas: high risk of getting cancer/ operation to stop it
- Concern: high possibility of cancer
- Expectation: urgent test to rule out this condition

Management vital points

- Explain Dx as condition runs in families when some lumps/growth grow in the lining in the bowel and if not treated, one of these will certainly turn into cancer usually by age of 40. The chance of getting this condition from dad is 1:2 (hope)
- If red flags – then 2 WW referral to colorectal cancer
- If asymptomatic then refer to genetics for blood test and colonoscopy screening
- If screening picked up any polyps, will be removed and if it large number then surgical removal of the colon can be offered

12 F with Hx of CP + 5 days constipation = teenager constipation	Qs not to miss: • Red flags: no flatus/ abdominal distension/ severe abdominal pain/ bleeding/ persistent vomiting • RFs: diet/ fluids/ medications/ PEG feeding/ environmental factors Cues not to miss: • Normal examination by PA/ smears in his nappies/ Child on baclofen ICE: • Ideas: constipation • Concern: she is in pain? Getting worse • Expectation: laxatives Management vital points • Explain RFs constipation: less mobility/ baclofen/ less fluids/ PEG feeding • Avoid triggers/ RFs + lifestyle modifications + encourage mobility • Consider reviewing meds including baclofen and discuss with specialist • Offer macrogol laxative as 1st line then senna if no better • Follow up after a week – if no better or faecal impaction then add senna • Refer if any red flags or child distress (avoid PR or supp in 1ry care)
26 months F + Hx of constipation + getting worse since stopping macrogol = toddler constipation	Qs not to miss: • Red flags: weight loss/ blood in stool/ vomiting/ ribbon like or pale stool/ severe abdominal pain/ safeguarding issues • RFs: potty training/ less fluids/ lack of high fibre diet/ stress Cues not to miss: • Starting potty training ICE: • Ideas: there is an underlying cause for this constipation – will be dependant of laxatives • Concern: something serious • Expectation: laxatives again? Tests? How to prevent it? Management vital points • Explain RFs for constipation: potty training which lead to hard stool then painful when try to go + no serious underlying issues due to lack of red flags • Offer laxatives with 1 sachet daily then increase if needed + advice to continue until finishing potty training and even symptoms stop to avoid recurrence • Ensure high fibre diet/ good hydration and regular toilet • Potty training advice: don't push + give rewards + temporary breaks
69 M + discuss letter about diverticulitis = diverticulosis	Qs not to miss: • Response to antibiotics and experience in hospital • Red flags: weight loss/ changing bowel habits/ severe diarrhoea or abdominal pain/ persistent vomiting/ fever/ severe PR bleeding • RFs: constipation/ diet/ fluids/ medications Cues not to miss: • Requesting rescue antibiotics when in holiday • Patient had constipation and on codeine • Not been informed about previous Dx of diverticulosis ICE: • Ideas: diverticulitis? What it is? Why flare up? • Concern: holiday in 4 weeks? Need rescue antibiotics? • Expectation: explain + given abx Management vital points • Explain Dx as inflammation of small pouches in the bowel which is common with age and can be managed with some lifestyle changes and avoiding triggers • Explain complications with safety netting advice • Address RFs as constipation – need to change diet to high fibre diet and increase fluids + stop codeine and replace with alternatives like topical NSAIDs • Don't give abx due to risk of resistance, masking underlying serious problems and advice to seek urgent care if any red flags • Address patient no aware of diagnosis and offer complain process

27 F + known Crohn's disease + flare up = CD flare up	Qs not to miss: - Red flags: severe diarrhoea >6 times/ fever/ dehydration/ tachycardia/ hypotension/ severe abdominal pain/ persistent vomiting - DDx: recent travel/ gastroenteritis (N/V/D)/ DKA (dehydration/ DM) - Compliance with medications - RFs: smoking/ alcohol/ toilet routine/ food/ courses of antibiotics/ infections - Impact on work Cues not to miss: - Triggers– stopped methotrexate due to COVID + NSAIDs + stress due to exams ICE: - Ideas: CD flare up need medication adjustment - Concern: not able to do her cabin crew exams - Expectation: increase dose of methotrexate/ why flare up? How to prevent it? Management vital points - Explain flare up as activation of the inflammation of the bowel after being quiet. Explain triggers: certain food/ stress/ stopping medications/ NSAIDs - Avoid greasy/fried food – eat thoroughly fruits and vegetables – leaflet - Offer reducing course of steroids (if on only one immunosuppressant) or call on call gastroenterologist + arrange follow up + safety netting - Offer sick note
6 months + mum concern about worsening eczema and unsettled after feeds = CMPA	Qs not to miss: - Red flags: blood in stool/ persistent vomiting/ dehydration/ hives/ lip or face swelling/ breathing problems/ weight loss/ baby safety issues (NAI) + exclude any mum red flags as postnatal psychosis - Ask about feeding/ allergy/ growth/ FH - Impact on mum/ support/ mood - DDx lactose intolerance/ GORD/ colic/ nappy rash/ constipation Cues not to miss: - Financial issues/ mum stress - Mum vegan (so no symptoms while on breastfeeding) - Worsening eczema after shifting to normal formula ICE: - Ideas: teething/ reaction to new formula? - Concern: baby in pain/ impact on growth? Mum coping? Cost of formula? - Expectation: eczema management/change formula/ prescription for formula Management vital points - Explain CMPA as baby's body attack bowel when exposed to cow milk - If severe symptoms with faltered growth, refer to paediatrics. If not, then refer to dietician to monitor growth and advice about milk free products - Trial of eliminating cow milk for 2-4 weeks + prescribe eHF if formula fed + monitor response then controlled home reintroduction usually by dietician - Explain no need for allergy test + give leaflet + arrange follow up + support mum
27 F + concern about her 8 weeks baby crying a lot = infantile colic	Qs not to miss: - As 110 Cues not to miss: - Normal 6 weeks baby check + - Mum not coping (Mum words (she won't shut and driving me insane)) ICE: - Ideas: underlying serious problem - Concern: mum not coping and feeling isolated - Expectation: reassurance + support at home + something to stop baby crying Management vital points - Reassure + Explain colic as common problem with repeated bouts of excessive crying in healthy baby usually > 3 hours a day. It is not your fault, and it usually improve by 3-4 months - Explain some tips to comfort baby: background noise/ hold gently/ breastfeeding - Parental wellbeing: rest when baby is sleeping + support groups as CRY- SIS - No much evidence about colic medications – can try if wants to

18 weeks F + suspected reflux on gaviscon + still unhappy after feeds = reflux

Qs not to miss:

- As 110

Cues not to miss:

- Recent visit to A&E for suspect chest infection – reassured as reflux
- Mum anxiety about her baby

ICE:

- Ideas: reflux but not improving – A&E missed something
- Concern: underlying problem in her chest or throat?
- Expectation: referral to specialist/ trial of another medication as Gaviscon not working

Management vital points

- Explain Dx of reflux as regurgitation of the milk because the muscle at the lower end of the gullet (oesophagus) is too relaxed. It's normal for this to not be fully developed in babies but eventually matures as in adults. As the contents of the stomach are acidic this can irritate the lining of the oesophagus causing your baby symptoms like cough, unsettles and slow weight gain
- Recommend small frequent feeds for 2 weeks then thickened formula for 2 weeks if not helping then 2 weeks Gaviscon then 4 weeks PPI if not helpful (as in this case) then refer if no response
- Avoid overfeeding or head elevation

49 M + requesting fit note for the court = fit note

Qs not to miss:

- Red flags: diarrhoea (bleeding/ weight loss) / mood (suicidal thoughts)
- Explore social Hx and impact of the court hearing on him

Cues not to miss:

- Fit note for missed court hearing (Explore court situation) – backdated and not presented with any problem at this time
- Alcohol abuse

ICE:

- Ideas: sick note
- Concern: potentially face jail for not turning up without this sick note
- Expectation: sick note

Management vital points

- Explain the legal issues about fit note as only can be issued if patient has been seen when he was ill or soon after. No need for sick note if he has been off for less than 7 calendar days. Can be issued the same day or soon after and may be based on report from other professional. Don't issue fit note for this patient
- Offer alternative: write a letter summarizing his records and his own words
- Brief advice about alcohol and available support groups
- Address anxiety regarding the court hearing and offer support groups

62 F + excessive hunger + Hx of prediabetes = polyphagia

Qs not to miss:

- Red flags: binge eating disorder/ bulimia nervosa (self-image issues/ induced vomiting/diuretics /laxatives/ excessive exercise as compensation) / Cushing's syndrome (stria/ moon face/ steroids)/ hypoglycaemic episodes (shaking/ sweating/hunger/palpitations/pass out)
- Impact on mental health
- DDx: DM/ hyperthyroidism (feels warm) / menopause (hot flushes) / Cushing's/ steroids/ depression
- RFs: stress/ feeling bored/ steroids/ lack of sleep

Cues not to miss:

- Hunger in the evening + skipping breakfast and lunch + putting on weight
- Bored at times when binge eating

ICE:

- Ideas: prediabetes turning into full diabetes?
- Concern: related to hypoglycaemia?
- Expectation: want something to stop this excessive eating

Management vital points

- Explain Dx of polyphagia which is extreme hunger which usually caused by various reasons such as DM/ hypoglycaemia/binge eating disorder
- Investigations to exclude DDx (HbA1C/ TSH)
- Refer to health coach given this situational pattern of hunger
- Lifestyle modifications: not to skip meals and regular meals

29 F + obese + requesting injections to lose weight = obesity

Qs not to miss:

- DDx: hypothyroidism/ PCOS/ medications
- Lifestyle: food/ exercise/ alcohol

Cues not to miss:

- BMI: 30.2/ Normal bloods (THS/PCOS screen/ HbA1C)

ICE:

- Ideas: weight loss injection will help
- Concern: wants to look nice for upcoming holiday in few months
- Expectation: weight loss injection

Management vital points

- Explain Dx of obesity with increased risk of complications + acknowledge difficulties in losing weight
- Recommend dietary changes + supervised physical activity (moderate exercise 150 min/week) + behavioural therapy with CBT
- Offer Orlistat (BMI >30 or > 27 if other risk factors as HTN/DM/dyslipidaemia) + explain mechanism, side effects and expected weight loss and follow up)
- Explain eligibility for these injections (BMI >35 + non -diabetic hyperglycaemia + high QRISK) and should be prescribed by 2ry care
- Mechanism of GLP-1: increases insulin sensitivity and secretion, suppresses glucagon secretion, and slows gastric emptying. Discuss side effects.

68 F + palliative patient + district nurse requesting antiemetics	Qs not to miss: - Red flags: bowel obstruction (abdominal pain/ vomiting/ distended abdomen/ not passing flatus)/ sepsis (fever/ UTI/ chest infection)/ unstable vitals - Impact on quality of life/ social history/ advanced care planning - RFs: constipation (use of laxatives/ morphine) Cues not to miss: - Metastatic pancreatic cancer + on morphine + not opened bowels for 3 days - Prefer oral medications not making her more sleepy/ Fear of end of life ICE: - Ideas: nausea due to cancer progression – hopes it is temporary - Concern: lose power/ approach end of life - Expectation: antiemetic to maintain quality of life Management vital points - Explain that nausea is common symptoms in palliative patient due to disease progression/ medications/ hydration status - Antiemetics options: metoclopramide is the best choice in this case given it is prokinetic effects with no sedative effect (cyclizine/ levomepromazine/ haloperidol/lorazepam) and not cause constipation (ondansetron) - Step up constipation options: add stimulant laxative senna to the laxido + ensure hydration (add lactulose in renal failure) - Address fear of end of life (fear of suffering? losing control? Isolation?) - Home visit if needed to exclude red flags and to discuss advanced care planning
55 M + abnormal LFTs + USS showing fatty liver = NAFLD	Qs not to miss: - Red flags: abdominal pain/ haematemesis/ jaundice/ weight loss/ vomiting - RFs: weight/ diet/ alcohol/ sedentary life Cues not to miss: - BMI 34/ alcohol 15 units/ weeks + Patient wants to know results first as worried ICE: - Ideas: USS showing cancer - Concern: cancer/ then fatty liver means liver failure? - Expectation: results of the scan then to stop liver from getting worse Management vital points - Explain fatty liver as excess fat builds up in the liver that can affect the liver function and causing symptoms such as fatigue. It can be alcohol related, or lifestyle related. Reassure it is not cancer - Discuss next steps to arrange blood test to calculate FIB4 score and if high > 2.67 (advanced fibrosis) to refer to specialist for fibro scan - Explain that lifestyle changes is the best management (healthy diet/ more exercise/ less alcohol/ lose weight) - Screen for other metabolic problems (DM/HTN/dyslipidaemia) + assess QRISK
26 F + abdominal pain + bloating + normal bloods and stool sample = IBS	Qs not to miss: - Red flags: weight loss/ change in bowel habits/ black stool/ PR bleeding/ persistent vomiting/ dysphagia/ abdominal masses/ FH of bowel cancer/ anaemia/ positive FIT test or faecal calprotectin or high CA125 - Criteria of IBS: abdominal pain > 6 months +relieved by defecation or ass with altered bowel frequency or stool form + 2 of (bloating/ worse by eating/ passage of mucus/ altered stool passage) + Dx of exclusion - RFs/triggers: stress/ food/alcohol/ smoking/FH + impact on life Cues not to miss: - Started new course which is intense - Symptoms worse when something important happening ICE: - Ideas: related to stress/ more serious causes? - Concern: ability to complete the course due to - Expectation: Dx and treatment Management vital points - Explain IBS as a chronic disorder consist of number of symptoms e.g. pain/ diarrhoea/ constipation/ bloating in presence of RFs - Lifestyle: low FODMAP diet/ more soluble fibres/ more fluids + food diary - Stress management with relaxation therapy and more physical exercise - Symptomatic management with loperamide for diarrhoea, peppermint oil for bloating, Buscopan for pain and laxatives for constipation (Avoid lactulose)

28 M + PR bleeding + Hx of piles + normal FIT test = haemorrhoids	Qs not to miss: - Red flags: change in bowel habits/ black stool/ weight loss/ masses in abdomen/ severe perianal/ anal pain - RFs: straining during bowel movement/ sitting for longer periods/ constipation/ chronic cough/ heavy lifting/ low fibre diet/ less fluids/ obesity/ occupations with repeated bending/ squatting (plumbers or electrician) Cues not to miss: - Don't understand the relative of FIT test as he has PR bleeding - Patient is truck driver (prolonged sitting) ICE: - Ideas: cancer due to FH and it is painless - Concern: serious things like cancer - Expectation: reassurance Management vital points - Explain Dx as enlarged veins in the lower rectum and around anus due to RFs - Offer examination (DRE) - Avoid RFs or triggers - Symptomatic relief with topical creams as short term - Refer if severe symptoms or advanced grades or thrombosed piles - Conservative: regular breaks to relieve pressure on rectal veins + improve sitting position (use donut cushions) + ensure hydration + high fibre diet + regular toilet - Explain that FIT test identifies the degraded Hb not the fresh blood
7 F + seen today with suspected mesenteric adenitis + now has high fever and severe abdominal pain = appendicitis	Qs not to miss: - Red flags: Severe abdominal pain/ severe vomiting/ unwell/ PR bleeding - DDx: constipation/ obstruction/ trauma - Impact on mum (new to the country) Cues not to miss: - Child too unwell to travel by taxi – patient new to the country - Pain was in the middle in her abdomen then moved to the right side ICE: - Ideas: need antibiotics to get better - Concern: something serious - Expectation: antibiotics over the phone for immediate relief Management vital points - Explain Dx is when small part of the intestine called appendix become inflamed and swollen. It needs quick treatment to prevent complications - Emergency hospital admission – make sure to arrange ambulance as patient unaware of the system
41 F + recent bloody diarrhoea after holiday + negative stool sample = UC new Dx	Qs not to miss: - Red flags: Severe abdominal pain/ severe vomiting/ PR bleeding/ weight loss/ night sweating/ unwell/ severe diarrhoea > 6-8 times a day. - DDx IBS/ Coeliac disease - Other symptoms: mouth ulcers/ skin lesions/ joint pain/ eye lesions/ fatigue - RFs: FH/ non-smoker/stress/ other autoimmune problems Cues not to miss: - Previous attack of gastroenteritis and red eye few months before - FH of psoriasis ICE: - Ideas: traveller's diarrhoea - Concern: no concerns - Expectation: reassurance Management vital points - Explain Dx as a long-term condition of the colon or large intestine which becomes inflamed with small ulcers or sores in its surface with some flare ups - Invx: FBC/ ESR/ Coeliac serology/ TSH/ LFT/ U&Es + faecal calprotectin - Urgent specialist referral if high faecal calprotectin for cameral test and biopsy - Avoid antidiarrheal – can cause megacolon

7 F + reduced appetite + more fussy + normal growth = Picky eating

Qs not to miss:

- Red flags: weight loss/ abdominal pain/ vomiting/ diarrhoea/ blood in stool/ night sweating/ difficulty swallowing
- DDx: constipation/ UTI/ ADHD/ autism/ food allergies/ GERD/ medications
- RFs: stress/ overly strict mealtime rules/ eating habits of peers

Cues not to miss:

- Impact on mum

ICE:

- Ideas: she not getting proper nutrition/ malnourished
- Concern: impact on growth
- Expectation: advice for better eating/ blood test to check

Management vital points

- Explain Dx as children are often fussy with food at this age as they have growing sense of independence and worry about new things including food. As long they are healthy and growing well, there's no need to worry
- Parental support by reassurance + some tips for fussy eaters (give same food as other family/ eat with your kids/ small portions and praise if they are eating/ don't force them to eat/ avoid too many snacks/ no food as a reward + involve them in food preparation + positive reinforcement + gradual exposure to food)
- Signposting to local parental support + Refer if suspected nutritional deficiencies or flattered growth/ suspected medical conditions
- Consider bloods for more reassurance (FBC/ Iron studies and vitamins)

68 M + palliative metastatic pancreatic cancer + wife wants to discuss hospital letter = Palliative care – Islam

Qs not to miss:

- Explore about religious beliefs/ hospice/ wishes/ preferred place of death
- Ask about eating/ drinking/ passing urine and stool
- Ask about pain and other symptoms
- Ask about DNACPR and SOI

Cues not to miss:

- Specific religious rituals
- Not keen to take morphine medications to able to perform prayers

ICE:

- Ideas: adherence to religious practices are essential for his peace in the afterlife
- Concern: interventions can interfere with religious obligations
- Expectation: guide that aligns with his faith and ease suffering

Management vital points

- Start with acknowledge difficult time and reassure about working together to keep patient comfortable in his final days
- Urgent home visit to assess pain and other symptoms – can try alternatives to morphine due to intolerance (such as fentanyl or oxycodone)
- Discuss need to stop unnecessary medications
- MDT team (involve palliative team/ district nurses/ liaison form Muslim community or chaplaincy services)
- Discuss advanced care planning
- Support to family (bereavement support)

**72 M + elevated
ALP Page's disease
of bone**

Qs not to miss:

- Red flags: weight loss/ night sweats/ jaundice/ falls/ cauda equina syndrome
- Ask about bone symptoms as pain/fractures/ neurological symptoms
- Impact of symptoms on work

Cues not to miss:

- Often asymptomatic – accidental with x-ray showing blade of grass lesion in long bones or cotton wool appearance in the skull
- Bone warm to touch + pain in bone or joint worse with weight bearing more at tibia/femur and forearm+ numbness + deformities as enlarged skull

ICE:

- Ideas: pain due to getting older and having arthritis
- Concern: symptoms going to force you to sell the family farm
- Expectation: painkillers to help the symptoms

Management vital points

- Explain Dx as disease of the bone with increase bone turnover in one or multiple areas which lead to weak bone causing pain and prone to fractures
- Arrange X-ray to exclude bone mets or osteosarcoma
- Arrange bloods to check bone profile
- Refer to rheumatology for confirming Dx + starting medications (bisphosphonates) +
- Refer physiotherapy + arrange follow up

**45 M + negative
mouth swab +
persistent bad
breath = Halitosis
due to GORD**

Qs not to miss:

- Red flags: dysphagia/ hoarseness of voice/ weight loss/ vomiting/ hematemesis/ recurrent tonsillitis/ oral ulcers > 3 weeks/ oral lump or abnormal gum hypertrophy/ suspected sinus disease)
- RFs: bad oral hygiene/ smoking/ GORD (RFs: coffee/ spicy food/ alcohol/ NSAIDs/SSRI)

Cues not to miss:

- Affecting his work
- Gaviscon for heartburn

ICE:

- Ideas: no idea about what is causing it
- Concern: impact on work and his marriage
- Expectation: effective treatment

Management vital points

- Explain Dx as acid reflux which happens when stomach acid travels up into your food pipe or throat causing symptoms like bad breath
- Recommend lifestyle changes (Avoid RFs as above) + ensure dental hygiene and tongue cleaning with tongue scraper + suggest antibacterial mouthwash like chlorhexidine.
- Offer PPI trial for 8 weeks then arrange test for H. Pylori if not fully resolved
- Refer urgently to ENT or oral specialist for persistent halitosis if + oral ulcer > 3 weeks or lump or abnormal gum enlargement or recurrent tonsillitis or suspected sinus disease.
- Refer to psychiatry if significant distress over halitosis and normal assessment

49 F + ongoing lump in the throat + normal examination = Globus due to GORD

Qs not to miss:

- Red flags: weight loss/ night sweats/ hoarseness of voice/ swallowing problems affecting eating and drinking/ odynophagia/ otalgia/ fever
- RFs: coffee/ spicy food/ alcohol/ stress/ smoking/ NSAIDs/ SSRI

Cues not to miss:

- Constantly need to clear the throat to feel better
- Swallowing problems with dried food only

ICE:

- Ideas: throat cancer given Hx of endometrial cancer
- Concern: possibility of cancer
- Expectation: reassurance

Management vital points

- As above case + explain globus as feeling of having a lump in the throat when in fact there is nothing there
- • Recommend lifestyle changes (Avoid RFs as above) +
- Explain that symptoms likely related to SSRI and NSAIDs + need to stop NSAIDs + add PPI cover for 4 weeks
- Refer to ENT if no improvement after 4 weeks and refer urgently to H&N if red flags

Neurology

**72 M + wife
worried about
partner mental
difficulties =
dementia new Dx**

**Dementia
potential Dx**

Qs not to miss:

- Red flags: stroke (weakness/ severe headache/ swallowing problems/ speech problems) / trauma/ falls/ delirium (chest infection/ UTI/constipation)/ safeguarding issues (agitation)
- DDx: different types of dementia (Hx of stroke/ MI/ PD/ behaviour changes)/ delirium (exclude infections)
- Ensure safety (ask about the patient and driving)
- Ask about impact on partner (caring/ mood/ daily life)

Cues not to miss:

- 6 CIT 18 + normal examination
- Patient thinks that it is just a transient phase due to virus infection

ICE:

- Ideas: Patient had dementia as her mum, and she responded to medications
- Concern: support for dementia?
- Expectation: diagnosis of dementia to start meds asap today

Management vital points

- Explain Dx as syndrome or group of symptoms associated with ongoing gradual decline of brain function which include memory, personality, communication and ability to look after themselves
- Recommend referral to memory clinic after doing set of bloods
- Offer support for the partner: signpost to social prescriber to explore support groups/ financial support/ carer support/ day centres
- Explain medications only prescribed by 2ry care after confirming Dx with imaging
- Ensure patient safety and advice about driving

**45 F + wants to
make complaint as
not given steroids
early for her Bell's
palsy = Bell's palsy**

Qs not to miss:

- Red flags: headache/ weakness/ red eye/ neck stiffness/ fever/
- DDx Ramsay Hunt Syndrome (painful rash and herpetic vesicles)

Cues not to miss:

- Conflicting information about Dx/ steroids
- Early steroids help improvement faster -now she is 2 weeks since first symptoms

ICE:

- Ideas: is it virus or Bell's palsy or even a stroke
- Concern: steroids would have cured it if it been given earlier
- Expectation: good explanation of Dx/ whether she should be given steroids earlier/ why face not back to normal yet? Should she file a complaint? Need to test for a virus?

Management vital points

- Explain Bell's palsy as a condition that causes temporary weakness of one side of the face due to weakness of the nerve supply the face with unknown pathology (possibly virus) and full recovery for most patient (84%) that can take up to few months.
- Explain that guide recommend **considering** steroids to be given if presented within 72 hours for 10 days and it is effective more in mild cases
- Advice about eye care with lubricating eye drops/ tapping over the eye at night/ wear sunglasses when outside and refer if persistent eye closure problem
- Follow up after 3 weeks from the onset – to refer if no improvement and another follow up at 3 months – to refer if incomplete resolution to neurology
- Direct patient to PALS if wants to do complaint without criticizing others

87 F + Hx of dementia acute confusion + speak to paramedic = Delirium

Qs not to miss:

- DDX: infection (UTI/ chest infection/ gastroenteritis)/ brain haemorrhage (fall)/ constipation/ dehydration/ pain
- Red flags: weakness/ headache/DKA/ bleeding/ bruises/ safeguarding issues/ neglect/ stroke
- Social Hx (carer/ support/ house)
- Advanced decisions (DNACPR/ SOL/ preferred place of care)

Cues not to miss:

- Observations done by the paramedic = pneumonia – improved
- Daughter will stay with her for few days to look after her

ICE:

- Ideas: initially felt she need hospital admission but if doctor see she can be managed at home, then it is fine. Likely chest infection
- Concern: what will happen when daughter leave after 4 days
- Expectation: GP to manage in the best possible way

Management vital points

- Explain Dx as chest infection causing her confusion which is common with dementia and should be improving with antibiotics + proper hydration
- Offer referral to admission avoidance team or Urgent community team for better monitoring
- Arrange follow up in 2 days to ensure improvement
- Give clear specific safety netting advice if worsening confusion/ breathlessness/ inability to eat and drink/ fall) to ring 999
- Apply sick day rule (stop most medications in this case)
- After recovery arrange home visit to discuss carer and advanced care planning

39 F + blurry vision + ipsilateral afferent relative pupillary light defect = optic neuritis/ multiple sclerosis

Qs not to miss:

- Red flags: vision loss/double vision/ loss of colour vision/ trauma/ acute red painful eye/ severe headache/ weakness/ sensory loss/ gait problems
- DDX: acute eye infections/ glaucoma/ temporal arteritis/ retinal artery or vein occlusion/ increased intracranial pressure
- RFs: FH
- Explore social Hx

Cues not to miss:

- Eye pain on movement + reduced vision + reduced colour vision
- Swollen optic disc + ipsilateral afferent relative pupillary light defect
- Episodic pattern followed by recovery periods

ICE:

- Ideas: stress related symptoms
- Concern: hope nothing is serious
- Expectation: blood test and vision check

Management vital points

- Explain MS as a chronic relapsing condition where your immune system attack your brain and spinal cord causing inflammation and damage of these nerves
- Explain episodic nature usually last > 24 hours after stable period of at least 1 month in absence of other causes
- Same day referral to ophthalmology if eye symptoms for IV steroids
- If no eye symptoms and suspected MS= refer urgently to neurology team
- Explain support groups and arrange FU appointment

78 F + CT scan showing glioblastoma + son wants information about scan

Qs not to miss:

- Ask about both mum and son (carer)
- Ask about LPOA to take decisions on her behalf
- Red flags (mum): confusion/ safety/ speech/ weakness
- Impact on carer

Cues not to miss:

- Letter with CT showing glioblastoma and mum asked not to inform her family + mum informing family the result is Ok + no LOPA
- Be empathetic

ICE:

- Ideas: mum lack capacity to make decisions
- Concern: mum not making sensible decisions
- Expectation: to know the results of the scan

Management vital points

- Explain confidentiality mean we only share her medical details after her consent to protect her privacy and autonomy, but we are here to support both of you
- Offer alternatives like joint consultation or arranging home visit to assess capacity + discuss with patient whether she needs her son to be present or not
- Don't disclose information in this case + be empathetic

61 F + requesting antibiotics for UTI + breast cancer Hx = Cauda equina syndrome

Qs not to miss:

- Red flags: bilateral sciatica/ bilateral progressive neurological deficits/ difficulty initiating micturition or impaired sensation of urinary flow/ loss of sensation of rectal fullness/ perianal sensory loss/ lax anal sphincter/ ED
- RFs: Hx of back pain/ Hx of cancer/ symptoms of undiagnosed cancer (weight loss/ fatigue/ bruises)

Cues not to miss:

- Don't notice when passing urine with constant accidents + numb while wiping + faecal accident in bed while unaware + breast cancer Hx

ICE:

- Ideas: Urine infection
- Concern: without antibiotics will struggle to collect her grandson
- Expectation: some antibiotics

Management vital points

- Explain Dx as a serious condition when the nerves at the very bottom of your spine are compressed and if untreated can cause permanent damage
- Immediate referral to A&E with ambulance for urgent MRI
- Explain that they need to arrange one to collect her grandson from school
- Follow up after discharge and to assess other DDx if normal A&E discharge

12 M + mum concerned about headache with normal examination = paediatric migraine	Qs not to miss: • Red flags: trauma/ fever/ neck stiffness/ weakness/ double vision/ speech problems / fits/ biting tongue incontinence/ fainting/ sudden headache with maximum intensity within 5 minutes/change in behaviour/ headache triggered by cough or exercise • RFs/triggers: FH/ dehydration/ stress/cheese/ chocolate/ smoking • DDx: tension headache (band like) and cluster headaches (lacrimation/ comes in waves for few minutes) • Aura symptoms: visual loss (fortification spectra) / speech problems/ sensory symptoms not > 1 hour and followed by headache within 1 hour Cues not to miss: • Bilateral throbbing headaches for 1-48 hours + lead to avoiding his football ICE: • Ideas: form of seizures? • Concern: has to stop football? Seizures? • Expectation: tablets to treat and prevent it as he taking tablets every day Management vital points • Explain Dx is a condition that causes attacks (episodes) of headaches. Between migraine attacks, the symptoms go away completely. • Discuss offering nasal triptan as 1st line + anti sickness if needed • Follow up after 4 weeks and consider referral for preventive treatment given impact on football
31 F + ongoing headache + normal examination = Acute migraine	Qs not to miss: • Same as above case Cues not to miss: • FH of migraine • Patient on combined contraception pills ICE: • Ideas: migraine? • Concern: need any stronger painkiller • Expectation: Dx as migraine Management vital points • Explain Dx as above • Recommend stopping COCP due to higher risk of DVT + discuss alternatives • Recommend headache diary to identify triggers • Lifestyle advice about good sleep/ avoid stress and smoking • Offer oral sumatriptan as 1st line + antiemetics as metoclopramide • Consider chronic therapy if > 2 episodes per month or acute treatment is non effective or poorly tolerated. Propranolol 80mg OD as 1st line then Topiramate or Candesartan or pizotifen or person at risk of medication overuse headache dir to frequent use of acute drug s Rimegepant or amitriptyline • Refer to neurology if migraine is more frequent or ass with neurological changes
39 F + Hx of migraine + preventive medicine not working with her + wants to try CGRP therapy = Chronic migraine	Qs not to miss: • As above case + impact on life Cues not to miss: • Debilitating migraine • Patient doesn't want any more children ICE: • Ideas: need the CGRP • Concern: long wait to see the consultant • Expectation: GP prescribe it or to go to private clinics Management vital points • As above + • Explain you can't prescribe It without specialist advice + unbale to recommended specific neurologist and it is difficult to know who can privately prescribe it. Even it is prescribed privately, we can't convert into NHS prescription due to need for ongoing specialist expertise and cost effectiveness • Give options to try topiramate as patient doesn't want any more children

51 F + headache + worried about mum discharge from hospital = Migraine -social stress

Qs not to miss:

- As above
- Social History and ask about mum baseline and current support

Cues not to miss:

- Can't afford to have headache
- Carer for mum – worried about premature discharge

ICE:

- Ideas: no idea about the cause of headache
- Concern: something serious? Concerned about mum discharge? Worried about caring responsibilities
- Expectation: treatment and assistance in addressing discharge plan

Management vital points

- As 135 +
- Explain the discharge process which involve MDT and if patient still concerned can speak to PALS to raise her concerns or liaise with hospital discharge team to raise daughter concern + manage unsafe discharge by MDT involving social prescriber and urgent community response team if unsafe discharge
- Offer carer support by signposting to counselling or social prescriber and organizations like Carers UK or recommend respite care services
- Follow up after discharge to assess the situation (possible home visit)
- Signpost to resources such as age UK for practical support to both of them

21 M + seizure + normal blood tests from A&E = First seizure

Qs not to miss:

- Red flags: fever/ neck stiffness/ weakness/ weight loss/ Hx of cancer/ behaviour changes/ trauma
- RFs: FH/ Hx of seizures/ medications or drugs/ alcohol abuse/ Hx of cancer
- DDx: hyperventilation/ weakness/ headache/ fever
- Impact on life (ask about work and driving)
- Ask about the seizure before/ during and after

Cues not to miss:

- Patient will be panic if started speaking about driving – be ready
- Address work and family responsibilities

ICE:

- Ideas: A&E over acting, I feel it is just a faint as I feel fine now
- Concern: lose my licence and I can't afford to be without it
- Expectation: GP will disagree with A&E and carry on as usual

Management vital points

- Explain seizure as abnormal movement that happen when there is a surge in electrical activity in your brain
- Refer for fist fit clinic within 2 weeks for EEG and imaging
- Driving – explain that patient shouldn't drive (for 6 months for group 1 holders and 5 years for group 2 holders and obliged to inform DVLA, and not doing that can has impact on his driving licence and insurance
- Ensure safety: patient to inform friends and family and avoid dangerous exercise (swimming alone/ cycling/ rock climbing)
- Fite note to switch to a new site where u can get public transport

74 F + episode of right arm weakness last night = TIA	Qs not to miss: - Red flags: ongoing symptoms or not (weakness/ speech problems/ balance problems/ sensory problems/ headaches/ confusion/ swallowing problems) - RFs: smoking/ diet/ alcohol/ obesity/ AF (palpitations)/ previous episodes/ CVD - Social Hx Cues not to miss: - RFs: PAD/ HTN + Patient on antiplatelet - Sudden weakness and loss of grip for 30 minutes ICE: - Ideas: trapped nerve - Concern: wasting GP time - Expectation: reassurance from GP Management vital points - Explain TIA/ ministroke which happen when blood flow to part of the brain is partly blocked which make you at risk of having full stroke - Patient should be referred for urgent assessment within 24 hours (as presented within 7 days) – - If symptoms more than 7 days, then refer within 7 days + Don't offer aspirin - Offer aspirin 300mg if not on antiplatelet / Don't offer if already on antiplatelet and ask to continue same dose/ Don't offer if on anticoagulation or bleeding problems and refer for urgent CT scan - Driving: cannot drive until seeing TIA clinic (usually at least for a month or 1 year for group 2 drivers) similar to stroke amaurosis fugax
67 M + speak to paramedic + patient with left arm weakness/ nerve entrapment = stroke	Qs not to miss: - Same as 132 Cues not to miss: - Some residual numbness and weakness + others symptoms even resolved (dysphasia and off balance) + subtle left side facial asymmetry - Confusion point: Hx of radicular pain in the same arm ICE: - Ideas: patient has nerve entrapment - Concern: less concerned after resolving of this weakness - Expectation: routine referral for assessment scan to assess her nerves Management vital points - Explain Dx of stroke when blood vessel in the brain blocked or burst + why it not radicular pain (sudden onset, weakness, balance and speech difficulties) - Refer to A&E and don't give aspirin (need to exclude haemorrhage first) - Arrange follow up to discuss RFs and compliance with medications - Advice about driving as 132 + address social needs
78 M + letter from A&E with recurrent falls + pill-rolling tremors + rigidity = Parkinson's disease	Qs not to miss: - Red flags: severe headache/ severe pain in back or hip/ confusion/ chest pain/ weakness/ weight loss/ speech problems or double vision - DDx: orthostatic hypotension/ vertigo/ vision problems - Symptoms of PD (rigidity/ slow motion/ resting tremors/ postural instability) + (writing changes/ voice changes/ sleep problem) - Social Hx and driving - RFs: age/ dementia/ FH/ medications such as antipsychotics Cues not to miss: - Always trip over ICE: - Ideas: dementia and age related - Concern: not concerned, it is just a trip - Expectation: reassurance – don't want to waste others' time Management vital points - Explain Dx as incurable but controllable disease affecting part of the brain which control movement and balance that can affect quality of life - Refer to neurologist then MDT including physio and occupational therapy to ensure safety at home - Must inform DVLA and insurance to ensure his safety - Signpost to support groups and refer to social prescriber if need more support

65 M + declined A&E – patient with partner concerned about sudden forgetfulness = transient global amnesia	Qs not to miss: • Red flags: headaches/ weakness/ speech problems/ falls/ ongoing symptoms/ trauma or seizures/ duration > 24 hours • Dx: TIA/ seizures/ migraine- associated amnesia/ transient epileptic amnesia • RFs: sex (in this case) / exercise/ stress/ Valsalva like manoeuvre Cues not to miss: • Ask about the episode (before/ during and after) • Only transient loss of short-term memory (not long term) and looks normal during the episode with no other symptoms ICE: • Ideas: no idea but paramedics were concerned about stroke or ministroke • Concern: ministroke? Safe to drive for his daughter weddings • Expectation: to have something to stop it from recurring Management vital points • Explain Dx as a poorly understood condition when you lose your short-term memory for up to 24 hours with no risk of ministroke or stroke. No further steps usually needed given the history and normal examination today • No impact on driving unless recurrent in group 2 • Safety netting if red flags as above to call 999 • Consider referral to neurologist if unsure about Dx
41 M + MND requesting letter for assisted dying = assisted dying	Qs not to miss: • Ask about pain/ mood/ breathing problems/ support • Explore all possible reasons behind this decision (mental/physical/ financial) • Ask about who is involved in this decision Cues not to miss: • Wants a letter to Dignitas (to end his life) • Aware that assisting suicide is illegal – need simply report of his condition • Pt say under data protection act of 2018 that you have right to have your records ICE: • Ideas: wants to end life in Dignitas • Concern: you will end your life suffocating to death/ getting proper documents prior to losing use of hands as process take 2-3month s • Expectation: GP will quickly complete this report Management vital points • Explain that the current law doesn't allow assisted dying which means that we are not allowed to provide any paperwork related to this process as it is considered as criminal act. Be sensitive and acknowledge his feelings • Explain available support to manage end of life symptoms and making him comfortable + explain available emotional and psychological support • Possible involve a colleague or other sources of support (palliative team)
78 F + concerned about partner driving behaviour = Dementia and driving	Qs not to miss: • Red flags: accidents or near misses / safety for him at home/ safety of her/ danger to others when driving/ aggression or delirium/ signs of infection • Impact on life/ how she is coping/ support at home Cues not to miss: • She is not able to drive/ he went to the shop before it opens ICE: • Ideas: dementia causing driving problems • Concern: accidents/ losing independence/ how to convince him to stop driving/ worried he might be angry with her • Expectation: advice/ assessment for partner/ handle the situation ethically Management vital points • Acknowledge worries and concerns • Ideally, discuss this with the partner and find a way to speak to him: try to have joint phone consultation or arrange home visit to assess capacity and to discuss driving concerns or involve family to convince him • Ask about further plan if partner stop driving – refer to social prescriber to arrange help with public transport • Patient should inform DVLA if he has dementia and if refusing, we might need to inform DVLA given safety concerns Emergency plan: if feel immediately unsafe contact emergency

84 F + daughter concerned about mum visual and auditory hallucination = Dementia – Lewy body

Qs not to miss:

- Red flags: severe headaches/ weakness/ double vision/ speech problems/ safety concerns to patient or others
- Ask about support/ mood/ memory and impact on life/ carer
- DDx: alcohol? Infection? Constipation? Dehydration

Cues not to miss:

- Patient has insight + No Psychosis history with good mood
- Criteria (fluctuating cognition/ recurrent visual hallucination/ sleep disorder/ one or more symptoms of parkinsonism)

ICE:

- Ideas: something to stop these hallucination
- Concern: hallucinations are distressing and worried about scan and bloods results
- Expectation: something to stop hallucinations

Management vital points

- Explain LBD as incurable but controllable brain disorder that can lead to problems with thinking, movement, behaviour, mood, and hallucinations.
- Refer to old age psychiatry of memory clinic to start this patient on medications like donepezil or antipsychotic (haloperidol or risperidone)
- Daughter can try positive distraction if mum stresses about this hallucination or using validating therapy and to ask about what they are experiencing
- If worried about safety, involve social prescriber or care coordinator to arrange regular carer
- If any safety concerns about others involve social services or police

82 F with AD + partner concerned about her behaviour = Dementia – aggression

Qs not to miss:

- Red flags: Safety concerns/ delirium (signs of infection/ constipation/ dehydration/ medications)/ falls
- RFs for aggression: pain/ delirium/ side effects of medications/ overstimulation

Cues not to miss:

- Injuries to his arm and burn

ICE:

- Ideas: no idea what's causing her behavioural change
- Concern: escalating behaviour further / worried about her risk of falls
- Expectation: GP to fix this issue or help

Management vital points

- Explain that aggression/ agitation or psychosis can be a symptom of dementia.
- Refer to specialist to prescribe risperidone or haloperidol as only antipsychotics for non-cognitive symptoms of dementia
- In case of challenging behaviour: address underlying causes like thirst or need to go to toilet or pain + if no better to move her to a safe low stimulation environment and use of de-escalation techniques such as active listening, verbal responding or pictures + don't get upset + take a break if possible and safe + try soothing her with music or distraction
- Support carer by signposting them to dementia UK or Alzheimer's society
- Arrange follow up after 6 weeks

55 F + sudden severe facial pain = Tri-geminal neuralgia

Qs not to miss:

- Red flags: weight loss/ headaches/ weakness/ sensory changes in face/ hx of optic neuritis/ FH of MS/ age < 40 years/ hearing loss/ rashes/ depression
- RFs/triggers: talking/ chewing/ brushing teeth/ touch or breeze on face/ cold wind

Cues not to miss:

- Pain is affecting quality of life – like singing
- Sudden facial pain like electric shock several times a day lasting few seconds

ICE:

- Ideas: dental problems even dentist reassure me?
- Concern: something serious like nerve pain or tumour
- Expectation: cause for it/ treatment?

Management vital points

- Explain Dx as long-term irritation of a nerve in the face called trigeminal nerve.
- Pain diary + try to avoid triggers
- 1st line is Carbamazepine – titrate gradually then reduce if pain free for 1 month
- Refer if no better or atypical presentations or red flags or significant symptoms
- Signpost to counselling if affecting QOL and causing depression

Dermatology

**29 F + Hx of hives
fever and asthma +
itchy red bumps =
acute urticaria**

Qs not to miss:

- Red flags: swelling of face/lips/ tongue/ breathing difficulties/ fever
- DDx: chronic > 6 weeks/ infection
- Triggers/ RFs: food/ medications/ insect bites/ infections/ products/ stress/ exercise/ cold/ heat/ sun/ vibrating tools/ contact

Cues not to miss:

- Rash: come and go/ raised/ itchy/ appear in different spot
- Not have any smart phone to send pictures

ICE:

- Ideas: stress related? Or allergic reaction
- Concern: something serious/ lack of sleep and work
- Expectation: medications

Management vital points

- Explain Dx as a condition happen when itchy skin lesion appear in your skin triggered by things like stress or allergies with no clear reason
- Diary to identify triggers + avoid potential triggers
- Non-sedating antihistaminic for up to 6 weeks to 6 months + follow up at 6 weeks + add LRA or increase dose then refer

**55 M + photo
showing red purple
rashes on arms =
Lichenoid drug
reaction**

Qs not to miss:

- Red flags: lip or tongue swelling/breathing problems/ widespread rash
- DDx: lichen planus (very itchy/ better with UV)/ allergic reaction
- RFs: work as welder/ sun exposure/ medications (BBs/ ACEIs/ Diuretics/ NSAIDs)

Cues not to miss:

- Recently diagnosed with AF and started new meds (including BBs) 3 months even the rash started 3 weeks ago
- Works as welder
- Criteria (symmetrical/ not involve nails and mm/ sun exposed areas more)

ICE:

- Ideas: reaction to the welder at work
- Concern: rash getting worse
- Expectation: may be bigger steroids tube can help as long as small one help a bit

Management vital points

- Explain Dx as skin rash that happen as a reaction to certain medications causing this red purple patches which is usually worse with UV rays which explain why it is more in the sun exposed areas. Can start few weeks to months after starting the medications. Stopping meds helps the rash to gradually clear up
- Stop or reduce dose the medications (BBs) and shift to alternative (CCBs) – can take long time to clear up
- Stronger topical steroids or oral steroids if severe
- Avoid triggers like UV

31 M + photo of legs showing psoriasis patches + been given antifungal with no improvement = Psoriasis

Qs not to miss:

- Red flags: guttate psoriasis/ systemically unwell/ fever/ mental crisis
- Ask about nail or joint problems
- RFs: Stress/ FH/ smoking/ alcohol/ medication like BBs
- Impact on life and work

Cues not to miss:

- PMHx of hypothyroidism

ICE:

- Ideas: psoriasis? Need stronger cream
- Concern: impact of the rash on upcoming holiday for engagement
- Expectation: effective treatment

Management vital points

- Explain Dx as long-term non curable but manageable condition affecting the skin and sometimes nails and joints as our defensive system is affecting our skin
- Explain that psoriasis can recur with RFs or triggers. Avoid triggers
- Offer emollients + potent topical steroids like Enstilar foam OD for 2 weeks then arrange follow up
- If no response after 2 weeks, then arrange referral to dermatology for phototherapy, immunotherapy. Refer to rheumatology if affecting joints
- Signposting to British association of dermatologists

32 F + photo with patchy hair loss = alopecia areata

Qs not to miss:

- DDx: trichotillomania/ telogen effluvium (sudden profound shedding of hair)/ tinea capitis/ thyroid problems
- Impact on her daily life

Cues not to miss:

- Stress at work and wedding in 3 months

ICE:

- Ideas: hair loss due to stress
- Concern: lose more hair which can ruin your wedding day
- Expectation: what is the cause and treatment

Management vital points

- Explain Dx as small round patches of hair loss as your body attacking your hair follicles but the hair is often growing back on its own after some time
- Investigations: FBC/ TSH/ Ferritin to assess deficiencies
- Offer potent topical steroids for 3 months or urgent referral for intralesional steroids but growth might take few weeks even after the wedding
- Give cosmetic solutions options like hair styling or hairpieces or wigs
- Address psychological factors and offer referral for counselling services + signposting to support groups such as Alopecia UK

38 M + hair loss + diagram showing androgenic alopecia = Andro- genic alopecia

Qs not to miss:

- DDx as above
- RFs: FH/ stress/ medications/ poor diet

Cues not to miss:

- Impact on his work and new relationship

ICE:

- Ideas: male pattern balding/ could it be alopecia?
- Concern: impact on current job and new relationship
- Expectation: treatment or referral for hair transplant operation

Management vital points

- Explain Dx as male pattern baldness or androgenic alopecia which develop in half of men over 50 years. Most treatment are to slow the process not reversing it
- Explain that most people will choose not to do anything given poor response to treatment and need for long term treatment which is costly with rebound shedding after stopping.
- Can have trial of Minoxidil 5% scalp foam as an option in the NHS or other non NHS options like surgical hair transplant
- Option to have cosmetic options like hair styling or hair camouflage or wigs (can be available in the NHS if eligible)
- Caffeine shampoos might be good choice via the chemist

14 F + Hx of acne + struggling with acne = Acne - self care

Qs not to miss:

- Red flags: suicidal thoughts
- RFs: PCOS/ oily care products/ emollients/ dairy products/ FH
- Impact on her life and mood

Cues not to miss:

- Mum idea about antibiotics and incorrect avoidance of all makeup

ICE:

- Ideas: This acne and need stronger treatment as OTC is not helpful. Whether stopping make up is correct or not
- Concern: Impact on her mood, confidence and daily activities
- Expectation: antibiotics to help within few days

Management vital points

- Explain acne as a common skin condition due to changes in the sebum glands and hair follicles in the skin. IT NEED AT LEAST FEW WEEKS TO IMPROVE
- Discuss skin care: keep skin clean by washing gently twice with mild non alkaline cleanser + avoid scrubbing too hard + not to prick or squeeze the spots + CAN USE NON-COMEDONGENIC or OIL FREE MAKEUP but clean your makeup brushes regularly to avoid infection and not share makeup with others
- Lifestyle changes: avoid sugary food or fizzy drinks + reduce dairy products
- Offer speaking to support groups or counselling if affecting mood OR CAMHS
- Can use acne products but at least for a month before expecting improvement + stop for few days if causing irritation then to resume gradually + FU in 4 weeks

16 F + photo showing acne in the back = Acne and PCOS

Qs not to miss:

- As above

Cues not to miss:

- BMI 28 – struggling to lose weight + irregular periods + dark hair around face/arms
- FH of PCOS / Not able to use cream on back

ICE:

- Ideas: acne need some medications
- Concern: acne worse
- Expectation: something stronger than a cream

Management vital points

- Explain acne then explain PCOS as a benign long-term condition in the ovary which cause hormonal imbalance
- Explain acne management as above but offer 3 months oral antibiotics with topical BPO to reduce resistance then review for possible further 12 weeks of abx or maintenance with topical treatment as Acnecide wash
- For PCOS – arrange bloods to check hormonal levels + no need for USS
- Offer COCP as good option for irregular periods/ PCOS/ and acne (Use Yasmin and try to avoid Dianette as 1st line due to increased risk of DVT)
- Advice weight loss to reduce complications like T2DM or CVD
- Reassess the young woman at or before full reproductive maturity (that is, 8 years post-menarche). This will require withdrawing the COC for 3 months or longer to determine the persistence of symptoms + contraception counselling

11 M + Hx of Eczema + mum concerned about him picking the spots all the time = Skin picking disorder

Qs not to miss:

- Red flags: signs of infection/ bullying at school/ suicidal thoughts
- DDx: eczema flare up/ scabies (severe itchiness/ other affected people)
- RFs/triggers: stress/ itchiness/ mental health problems (OCD/depression)

Cues not to miss:

- Photos of red spots on the forearm/ lesions are not itchy
- Moving to 2ry school – feels anxious

ICE:

- Ideas: scabies or other infection
- Concern: lesions will scar over and causing bullying at school
- Expectation: treatment for infection

Management vital points

- Explain Dx as a mental health condition where you can't stop picking at your skin which often happens in response to stress or anxiety
- Referral to social prescriber to help liaise with social services for school allocation
- Referral to CAMHS for CBT and possible medications
- Advise to keep his nails short/ wearing gloves at night/ putting tape on fingernails + distraction techniques + skin care with emollients or covering the lesions to heal

75 F + itchy rash not settling with OTC treatment = Scabies

Qs not to miss:

- Red flags: breathing problems/ swelling of lips and mouth/ signs of infection
- RFs: contact with infected person/ bad hygiene/ recent travel

Cues not to miss:

- Rash worse at night + affecting between the fingers + red bumpy rash
- Mobility issues due to stroke (Social Hx)
- Not able to take photo or come to surgery

ICE:

- Ideas: dry skin or allergy to something
- Concern: getting worse – not coping with the itchiness and affecting sleep
- Expectation: stronger medication

Management vital points

- Explain Dx as a skin condition caused by tiny mites creating itchy rash. It is contagious to people by close contact. Inform care agency
- Offer permethrin 5% lotion to use over the whole body then will be repeated after 1 week – address difficulties in applying it – can get help from carer
- Any household contact should be treated as well even asymptomatic
- Need to wash all clothing and bedding – can get help from carer or can put them in a plastic bag for at least 72 hours
- Itchiness can persist for up to 4 weeks after treatment.
- For itchiness: sedative antihistamine + calamine lotion or topical hydrocortisone

82 F+ Hx of BCC+ photo showing actinic keratosis = Actinic / solar keratosis

Qs not to miss:

- Red flags: increase in size rapidly/ painful/ bleeding/ colour changes/ weight loss
- DDx: Eczema/ BCC/ SCC
- RFs: Age/ sun exposure/ Hx of BCC

Cues not to miss:

- Few years patch + flaky + Hx of BCC
- Photo of scalp showing flat patch with warty yellowish rough surface

ICE:

- Ideas: eczema or reluctantly say skin cancer
- Concern: cancer
- Expectation: reassurance + cream to sort it out

Management vital points

- Explain Dx as skin damage due to cumulative sun exposure. It a common treatable condition but if not treated can turn into cancer (SCC)
- Explain need to protect skin with sunscreen and wearing protective clothing
- 1st line is topical 5% fluorouracil BD for 2-8 weeks or diclofenac gel 3% for 3 months. Side effects are skin irritation
- Refer is no improvement to dermatology for possible cryotherapy
- Follow up after 4 weeks to assess response

58 F + Hx of melanoma + with photo showing new red lesion with telangiectasia= BCC

Qs not to miss:

- Red flags: rapid increase in size/ change in colour or shape/ weight loss/ lymphadenopathy/ night sweating
- DDx: Eczema/ SCC/ smoking/ Hx of sunburn/ fair skinned individuals/ tanning
- RFs: sun exposure/ previous skin cancer/ FH

Cues not to miss:

- Patient worried and wants urgent referral due to delayed previous referral
- Health anxiety due to delay in melanoma Dx

ICE:

- Ideas: another skin cancer that spread like melanoma
- Concern: life threatening cancer and impact on caring for her dad and work
- Expectation: 2 ww referral

Management vital points

- Explain Dx as localized common type of skin cancer which never spreads to other parts of body and only affect the superficial layer of the skin and grow very slowly.
- Routine referral to dermatology except if in the face to be referred as urgent for removal to confirm Dx and sometimes topical therapies if superficial
- Avoid recurrence by using sun creams F50/ wear protective clothing and avoid tanning beds with regular skin checks
- Address health anxiety and refer to mental health services for more support

5 F + photo with cold sores around the mouth = Cold sores

Qs not to miss:

- Red flags: fever/ inability to swallow/ dehydration/ widespread rash/ bruising/ behaviour changes/ recurrent infection/ poor weight
- Ask about contact with other infected persons
- DDx impetigo (yellow crusting/ fever)
- RFs: fatigue/ stress/ exposure to sunlight, heat, cold/ injuries to skin/ poor diet

Cues not to miss:

- Mum worried about possibility of someone kissing her daughter
- Pain and tingling then rash with blisters

ICE:

- Ideas: skin infection need antibiotics
- Concern: if cold sores mentioned then concern about someone kissing her?
- Expectation: treatment and reassurance about possibility of someone kissing her

Management vital points

- Explain Dx as viral skin infection causing small painful blister around the mouth. and verbalize it is unlikely to be related to kissing as no other signs of abuse and it is not the only way for spread (can spread by sharing utensils/ lip balm/ direct contact with infected saliva or herpes lesions)
- Conservative: pain relief + stay hydrated + avoid spreading the virus + sun protection + can go to school if feeling well

73 F + photo showing rash with vesicles in the forehead = Shingles

Qs not to miss:

- Red flags: eye affection/ nose affection/ vision problems/ headache/ severe pain
- RFs: Previous hx of shingles/ not vaccinated/ stress/ infection/ on steroids

Cues not to miss:

- Looking after 3 grandchildren and visit husband in nursing home
- Allergy to plaster

ICE:

- Ideas: sweat rash?
- Concern: something might prevent me from visiting my husband
- Expectation pain control

Management vital points

- Explain Dx as a rash caused by the same virus as chicken pox but it stays dormant inside your nerves for many years and take up to few days to crust over and heal
- It is contagious so it better to avoid contact with pregnant and immune-suppressed. It can cause chicken pox by direct contact. If dried or covered may be able to decrease the spread
- Pain killer + Antiviral if presented within 3 days of rash onset + immunocompromised/ non truncal involvement/ moderate to severe pain or rash or >50 years to reduced postherpetic neuralgia +
- Refer if anew vesicles are forming after 7 days of antiviral treatment or delayed healing + consider neuropathic pain if postherpetic neuralgia
- Give alternative options for visiting her husband like video call for few days

6 months F + photo showing rash in the abdomen = Eczema in children

Qs not to miss:

- Red flags: breathing problem/ unwell/ infection (oozing or fever or crusting)/ FTT/
- DDx: CMPA (blood in stool/ vomiting/ unsettled after feeding/ constipation or diarrhoea) / Urticaria/ skin infection
- RFs: FH/ exposure to irritants

Cues not to miss:

- Itchy + spreading + started after changing to formula mil
- Impact on mum and sleep

ICE:

- Ideas: allergy to formula milk?
- Concern: rash causing discomfort for her
- Expectation: cream and blood test for milk allergy

Management vital points

- Explain Dx as very common skin condition causing dry skin and rash due to genetic and environmental factors and it usually better over time. It is not CMPA due to absence of other symptoms and FH of eczema
- Generous amount of emollients + mild to moderate steroids for 7 days
- 1 fingertip unit = 2 adults' hand
- Avoid triggers or irritants (use soap or bath substitutes/ avoid perfumes)
- If no better with this approach – can try switching to hypoallergenic formula
- Ensure support to mum

41 M + photo showing cyst on back worried about cancer = Epidermoid cyst

Qs not to miss:

- Red flags: ulceration/ bleeding/ weight loss/ night sweats/ malaise/ signs of infection (redness/ discharge/ fever)/ rapid growth/ irregular borders
- RFs: sunbeds/ FH

Cues not to miss:

- Emotional burden on him given his health anxiety

ICE:

- Ideas: something abnormal/ don't want it to be on body at all
- Concern: cancer? Concerned about its look and your mental well being
- Expectation: referral for removal in the NHS

Management vital points

- Explain Dx as common benign lump that forms under the skin due to skin growing inward instead of shedding a way. It is not cancerous
- Refer for removal under NHS if (repeated infection > 2 per year / regular bleeding or pain or pressure symptoms or large size > 5cm)
- Offer options for private referral if patient wishes an excision for cosmetic reasons
- Consider anxiety and depression review and referral to psychological support

22 M + excessive hand sweating for few months = Hyperhidrosis

Qs not to miss:

- Red flags: night sweating/ weight loss/fever/ sensory changes in hands
- RFs: FH/ stress/ medications
- Criteria for primary (focal, excessive + > 6 months + at least 2 of the following (bilateral/ interfere with daily activities/ at least once a week/ onset < 25 years/ FH/ stops during sleep)

Cues not to miss:

- Impact on life and work and relationship with partner
- Worsening anxiety and using more propranolol
- Stopped aluminium chloride due to side effects

ICE:

- Ideas: excessive sweating getting worse
- Concern: costing you everything: work, relationship, and can't cope
- Expectation: referral for iontophoresis and Botox injections

Management vital points

- Explain Dx as excessive sweating due to overacting sweat glands that can affect the whole body or certain areas
- Adjust using aluminium chloride (1st line) : apply at night to dry skin of affected areas then wash off in the morning + use every 1-2 days until improvement + avoid side effects by using barrier cream or mild steroids
- Consider stopping propranolol as causing worsening symptoms and try alternative anxiety measures
- If suspected 2^{ry} causes – then do bloods (FBC/ ESR/ U&Es/ LFTs/ TB screen or HIV screen) +/- CXR
- Follow up in 4-6 weeks then refer to dermatology for other oral agents/ iontophoresis/ botulinum A toxin injection or surgery if no improvement
- Option of Individual funding request for NHS funding of intervention that is not routinely commissioned

74 F + Hx of chronic urticaria was give Incorrect prescription of lorazepam instead loratadine – = duty of Candour

Qs not to miss:

- Red flags: falls/ driving accidents or near misses/ fever and neck stiffness/ swelling of lips and tongue or breathing problems
- Ask about current urticaria
- Ask about medications: compliance/ overdose?
- DDx alcohol/ any medications from relatives/ illicit drugs

Cues not to miss:

- On the information given – Lorazepam is prescribed instead of loratadine
- Patient is confused/ drowsy – had an accident

ICE:

- Ideas: higher dose of loratadine causing this
- Concern: drowsiness means complications or serious disease?
- Expectation: treatment for rash/ explain why this drowsiness happen

Management vital points

- Explain this error as I'm sorry it seems there has been a mix-up with your medication that happened at last consultation, instead of receiving the antihistamine, you were given a tablet that can make you very sleepy
- Discuss potential consequences of this error
- Explain steps to prevent such error such as incident reporting
- 1st step is to stop the wrong medication and start new correct one for the rash
- Offer a letter to share with the insurance company about this error if helpful
- Offer steps for the complaint process

32 F + blotchy rash on abdomen = PUPP (pruritic urticarial papules and plaques of pregnancy)	Qs not to miss: - Red flags: breathing problems/ swelling of tongue or lips/ fever/ headaches/ obstetric cholestasis (yellowish skin/ pruritic in palms and soles) - DDx: urticaria/ eczema/ infection/ obstetric cholestasis Cues not to miss: - Patient is pregnant (cue is want safe prescription or had to stop drinking) - Pink itchy blotchy started in the stretch marks on abdomen and upper thigh sparing palms of hand and soles of feet ICE: - Ideas: rash due to recent house renovations due to contact? - Concern: serious rash during pregnancy - Expectation: explain Dx and reassure about baby safety Management vital points - Explain Dx as a common skin disorder that usually presents in pregnant women in their 3rd trimester and more common with first pregnancy with unknown cause with spontaneous recovery within 4-6 weeks after delivery. There is no cure except delivery. No impact of this condition on the baby - Conservatives with moisturisers and mild steroids/ antihistamines - Blood tests with LFTs and serum bile acid level to exclude obstetric cholestasis
28 F + Hx of eczema + suspected eczema of feet worsening with to steroids + photos showing rash in feet = Tinea pedis	Qs not to miss: - Red flags: recurrent infections/ immunodeficiency - DDx: eczema/ infected eczema - RFs: sweaty feet/ closed tight shoes/ long term steroids/ DM - Ask about pregnancy Cues not to miss: - Peeling of the skin + discoloration of the toenails + bad odour ICE: - Ideas: fungal infection? Wrong dose of steroids - Concern: impact on work - Expectation: quick fix Management vital points - Explain Dx as fungal infection of the foot = athlete's foot caused by fungi that flourish in warm environment like between toes and in sweaty shoes - Self-care advice: well, fitted breathable shoes + shoes need to dry fully before wearing again + keep feet cool and dry + regular change of cotton socks + avoid scratching + don't share towels + wash socks and towel at 60 degrees - Offer antifungal starting with topical for mild cases like terbinafine cream for at least 2 weeks +/- mild steroids for itchiness. Oral terbinafine if no better - Offer scraping the skin for fungal microscopy if symptoms persist or severe
3 Y + photo showing widespread rash = Viral exanthem	Qs not to miss: - Red flags: unwell/ lethargy/ confusion/ photophobia/ non blanching rash/ dehydration/ headache/ neck stiffness/ breathing problems/ swelling of tongue - DDx: urticaria/ measles (koplik's spot/ non vaccinated)/ erythema infectiosum (slapped cheek)/ Roseola (high fever that resolves as a rash develop)/ scarlet fever (sandpaper rash + strawberry tongue + sore throat) - RFs: contact with others/ recent infection/ medication/ Hx of allergy Cues not to miss: - Widespread macular rash + generally well + glass test -ve + fever ICE: - Ideas: some serious illness like meningitis - Concern: meningitis? - Expectation: reassurance – antibiotics just in case Management vital points - Explain Dx as Viral rashes are quite common in young children as the body is fighting the virus. It is usually harmless and clear up after few days - Reassure about no need for antibiotics given the likely viral and possible delay of presentation for other DDx if given antibiotics - Conservative with paracetamol/ fluids/ rest

**3 F + mum
concerned that she
has been tampered
with + mark on back
= Safe-guarding**

Qs not to miss:

- Red flags: change behaviour/ bruises/ limping/ blood from anywhere/ vaginal discharge/ urination problems
- Social life and school
- Explore mark in the back (unusual)

Cues not to miss:

- Mum separated from dad who is alcoholic with group of friends

ICE:

- Ideas: child been hurt probably by ex-partner's friend
- Concern: mark in back – sexual abuse? Concerned about social services
- Expectation: make sure she is safe

Management vital points

- Address concerns + offer examination + proper documentation and photos, if possible, to support concerns. If abuse suspected, then involve other child protection/safeguarding agencies or the police
- Explain need to involve social services if needed to keep her safety than removing her from her family.
- Arrange safety plan in case of emergencies with safe place

ENT

24 M + swelling in the gum + can't get to a dentist = Dental abscess

Qs not to miss:

- Red flags: difficulty swallowing with drooling / difficulty opening mouth/ swelling of neck or face/ signs of airway compromise

Cues not to miss:

- Unaware of 111 emergency dental services + Financial problems

ICE:

- Ideas: swelling and pain due to dental issue
- Concern: getting worse and cost of dental care
- Expectation: antibiotics – toothpaste on the NHS – pain killers on the NHS

Management vital points

- Explain Dx as infection of the base of a tooth due to bacteria enters through a cavity or crack
- Prescribe analgesia given his financial problems
- Refer to urgent dental services
- Refer to social prescriber/ social services for financial support + explain available claiming healthcare travel costs + discounted transport services
- Prescribe antibiotics if systemically unwell or lymphadenopathy or cellulitis or diffuse swelling or immunosuppressed or have diabetes
- Self-care with warm saltwater rinses several times a day + oral hygiene (dentist only can prescribe toothpaste) + avoid extremely hot or cold food or drinks

61 M + ringing in ears = Tinnitus

Qs not to miss:

- Red flags: pulsatile tinnitus/ sudden hearing loss over periods of 3 days in the last 30 days / numbness of face/ visual disturbance/ vestibular symptoms/ head trauma/ suspected stroke/ high risk of suicide
- DDx: infection/ Meniere's/ noise induced hearing loss/ impacted wax/ age related
- RFs: medications/ infection/ exposure to noise at work/ age
- Impact on sleep/ work and relationship/ mood

Cues not to miss:

- Work in cement blocks manufacture with no ear protectors

ICE:

- Ideas: related to candesartan which started 12 months before?
- Concern: struggling with poor concentration due to poor sleep
- Expectation: treatment for the ringing/ sleeping pills/ stop Candesartan?

Management vital points

- Explain Dx as a common condition of hearing sounds like ringing/ buzzing or humming that happen due to different reasons like exposure to loud noise or age.
- Invite for examination to exclude any ear problems and assess hearing/ neurology
- Refer if any red flags or persistent symptoms to audio-vestibular services + refer all patient with hearing loss for audiology assessment + possible hearing aids
- Stop any possible triggers mainly NSAIDs/ loop diuretics/ SSRI/ TCA
- Candesartan unlikely to cause it
- Sound therapy – trial of low-level background noise especially at night

81 F+ off legs and dizzy + paramedic asking for Stemetil = Vestibular neuritis	Qs not to miss: • Red flag: neurological weakness/ numbness/ lack of coordination/ problems with speech or swallowing/ no vision or hearing problems/ delirium/ hypoglycaemia • DDx BPPV or Meniere's, orthostatic hypotension Cues not to miss: • Dizziness when moves head for 5 mins but never fully clears + nystagmus ICE: • Ideas: vertigo • Concern: patient concerns about symptoms and managing at home. Paramedic wants some stemetil • Expectation: Stemetil Management vital points • Explain Dx as inflammation of the nerve in the inner ear that help controlling balance usually due to viral infection. Usually improve within few weeks • Ensure safety at home: walking aid/ fall alarm/ avoid driving/ avoid dangerous activities/ avoid alcohol/ ensure hydration/ involve occupation therapy if needed • Short term prochlorperazine for 3 days to help the symptoms • Follow up after a week and refer if no improvement or after 6 weeks if not cleared
61 M + ongoing sore throat = Suspected - Laryngeal cancer	Qs not to miss: • Red flags: dysphagia/ hoarseness/ shortness of breath/ weight loss/ loss of appetite/ unilateral nasal symptoms or bleeding/persistent hoarseness of voice or ulceration in mouth > 3 weeks/ Unexplained neck lumps > 3 weeks • RFs: smoking Cues not to miss: • Normal examination + weight loss ICE: • Ideas: wasting the doctor's time – want some treatment • Concern: something serious? • Expectation: reassurance and treatment Management vital points • Explain Dx (breaking bad news) as worst-case scenario to be cancer given persistent symptoms > 4 weeks, weight loss, and smoking history • 2 WW referral to ENT • Smoking cessation advice + offer mental support
7 F with recurrent sore throat + mum requested Tonsillectomy	Qs not to miss: • Frequency of episodes + impact on activities and school • Red flags: signs of sepsis/ peritonsillar abscess or immunosuppression Cues not to miss: • Mum has removed her tonsils when she was young • Family belief in antibiotics as treatment for sore throat ICE: • Ideas: repeated episodes of tonsillitis need her tonsils out • Concern: recurrent tonsillitis • Expectation: referral for tonsillectomy Management vital points • Explain that tonsillectomies are no longer routinely performed because most sore throat in children are caused by viral infection which typically resolve on their own without unnecessary surgery • Explain current indication of surgery when at least seven episodes in the preceding year or five per year for 2 consecutive years or 3 per year for 3 consecutive years + sore throat due to acute tonsillitis + episodes are severe and disabling • Explain no need for antibiotics for viral infection due to unnecessary side effects and antibiotics resistance • Offer 2nd opinion if still concerned

58 F + Neck lump – suspected cancer

Qs not to miss:

- Red flags: weight loss/ night sweating/ generalized lymphadenopathy/ unexplained neck lump/ fever/ compression symptoms/ size with alcohol/ fatigue
- DDX: reactive LNs/ thyroid nodules/ TB
- RFs: smoking

Cues not to miss:

- Lump in left anterior triangle + firm non tender + getting bigger
- Patient booked holiday to see daughter next week

ICE:

- Ideas: probably large lymph gland
- Concern: concerned about it being getting bigger
- Expectation: reassurance

Management vital points

- Explained Dx as breaking bad news with the worst-case scenario is cancer given persistent of the symptoms > 6 weeks/ smoking history/ progressive course/ unexplained weight loss
- Invite for examination
- Advise delay the travel and provide letter for the insurance or the airway for refund + explain that travelling without letting her insurance know make it invalid
- Refer 2 WW to H&N for further scan and tests or biopsy (FNA)
- CXR to exclude local pathology
- Smoking cessation support
- Follow up if normal findings to investigate weight loss

28 F + hearing loss in one ear not responded antibiotics = Eustachian tube dysfunction

Qs not to miss:

- Red flags: sudden hearing loss/ headache/ facial weakness or altered sensation/ trauma/ immunosuppressed with otalgia not responding to antibiotics
- DDX: Barotrauma (flight Hx)/ otitis media with effusion (bulging TM)
- RFs: Recent infection
- Impact on work (call handler for 111)

Cues not to miss:

- Examination – indrawing of the TM with air bubbles behind it + Hx of viral URTI
- Pressure in ear like being in a plane or underwater

ICE:

- Ideas: infection needs stronger antibiotics?
- Concern: impact on work
- Expectation: antibiotics or quick fix

Management vital points

- Explain Dx as problem in the tube that connects your middle ear to the back of your nose when it is not opening and close properly. It is often go resolves on its own but it may take few weeks to months
- Conservative measures: swallowing/yawning/ chewing gum/ performing the Valsalva manoeuvre/ avoid triggers such as flying or diving
- Trial: nasal steroids for few weeks + oral decongestants + antihistamines
- Refer to ENT if symptoms > 4 weeks
- Explain equality act that employer need to do modifications to accommodate her till resolution of symptoms or offer sick note with amended duties

**48 M + presented
with tiredness + Hx
of obesity = OSA**

Qs not to miss:

- Rd flags: weight loss/ night sweating/ change in bowel habits/ lymphadenopathy
- DDx: OSA/ anaemia/ CFS/ cancer/ depression/DM
- RFs: obesity/ smoking/ alcohol/ large tonsils. Large neck circumference
- Impact on driving/ daily life
- Calculate Epworth sleepiness score questionnaire

Cues not to miss:

- I can fall asleep almost anywhere + loud snoring at sleep
- HGV driver

ICE:

- Ideas: Glandular fever?
- Concern: glandular fever can cause tiredness for years/ sleeping at the wheel
- Expectation: test for glandular fever

Management vital points

- Explain Dx as a condition in which your breathing briefly stops many times during sleep due to collapse of airway due to relaxation of your throat muscles causing unconscious disturbance of sleep and feeling tired and unrefreshed in the morning
- Offer referral to sleep clinic urgently as working as driver for further test and R/
- Advise to inform DVLA to ensure patient and public safety + insurance is invalid being under investigation + usually temporary measures
- Discuss lifestyle changes and mainly weight loss + stop smoking + less alcohol
- We can inform DVLA if patient is refusing to stop driving against our advice
- Explain no symptoms of glandular fever so unlikely

Eye

**19 F + eye watering
up + Hx of hay fever
= Dry eye syndrome**

Qs not to miss:

- Red flags: acute red painful eye/ vision loss/ double vision/ painful eye movement/ trauma/ rashes/ swollen eye/ weight loss/ dry mouth/ joint pain/ floater or flashes/ halos around eye / loss of peripheral vision/ loss of central vision/ impact on driving/ falls (safety)/ loss of colour vision
- DDx: infection (discharge or sticky eye lids)/ allergic conjunctivitis
- RFs: contact lenses/ smoking/ exposure to chemicals/ exposure to screen/ air conditioning/ Bell's palsy or PD/ drugs such as glaucoma meds/ autoimmune PMH

Cues not to miss:

- Important exam in the afternoon

ICE:

- Ideas: dry eye? But why they are running all the time
- Concern: have an exam today afternoon
- Expectation: advice about exam? Quick fix or treatment

Management vital points

- Explain Dx as a condition when your eyes don't make enough oily tears which results in more evaporation of the tears causing dry eye and overproduction of watery tears causing watery eye. As the tear film is formed of 3 layers.
- Explain that exposure to screen is on of the RFs that caused worsening of the symptoms given preparation for the exam due to less blinking. Try to follow 20-20-20 rule (every 20 min, take 20 sec, to look at sth 20 feet away, blink 20 mins.
- Offer OTC eye lubricants – can work quickly to help with the exam
- Or write supporting letter with a charge to explain his condition to delay the exam
- Refer to optician to check eye

Lifestyle measures: warm compresses/ lid hygiene/ lid massage/ less contact lenses time/ less screen time/ avoid alcohol or smoking

**42 F + Hx of Reiter's
syndrome + HLA-
B27 positive + new
acute red painful left
eye + photo of red
eye = Anterior
uveitis**

Qs not to miss:

- As above

Cues not to miss:

- Working as driver instructor + perfect vision in the right eye
- Redness more around the iris + no discharge + positive HLA-B27 + pain worse close-up activities like reading = uveitis

ICE:

- Ideas: eye strain from overworking or infection?
- Concern: impact on vision? Has to cancel lesson – financially? Stop driving?
- Expectation: quick fix with medications or drops

Management vital points

- Explain Dx as inflammation of the front part of your eye around the coloured area we call the iris. It is often due to your immune system attacking your body. HLA-B27 positive means that you have higher probability of autoimmune diseases
- Urgent referral to be seen within 24 hours by eye specialist with good prognosis. Topical steroids eye drops can be prescribed in the meantime to reduce inflammation + cycloplegic eye drops such as cyclopentolate to relieve pain
- No need to inform DVLA given only one eye is affected

67 M + recently Dx with Glaucoma + wants to know importance of PGs drops = Glaucoma

Qs not to miss:

- As above
- Social Hx: impact of his arthritis

Cues not to miss:

- Hand arthritis make it difficult to use the eye drops
- Loss of peripheral vision and driving
- Struggles to see cars while crossing the road

ICE:

- Ideas: eye drops might be not necessary as can compensate by moving head
- Concern: frustrated with the difficulty of using the drops due to arthritis
- Expectation: alternative to drops – want to continue driving

Management vital points

- Explain Dx as a condition when there is higher pressure in your eyes affecting the nerves in your eye and can lead to gradual vision loss more around the edges of what you see. The drops are vital to prevent vision loss by slowing progression of the condition. Due to your arthritis, dispensers can be used to help as Opticare Arthro
- As it is affecting vision and bilateral, it is legal requirement to inform DVLA for assessment and doesn't mean losing your licence to ensure your safety. If one only eye affected, no need to inform DVLA if other eye is meeting the standards
- Ensure compliance with the drops – advise gently pressing on the inside corner of the eye to increase contact time and decrease absorption

Offer support with any social issues and signpost to social prescriber

78 M + photo showing drusen in the retina + sudden decline in the vision = Wet AMD

Qs not to miss:

- Red flags as above
- RFs: smoking/ HNT/ age/ FH/ overweight/ lack of exercise
- Social Hx and impact on life and driving
- RFs

Cues not to miss:

- Age + blurry vision + dark spots in the centre of the vision + affecting reading (wavy line or bent) + drusen (yellow spots in the centre of the retina) +/- visual hallucinations
- Visiting his wife in a home care

ICE:

- Ideas: rapid vision loss due to age? Due to HTN?
- Concern: worried about losing independence? Not be able to visit wife? Help with daily tasks.
- Expectation: help to stabilize the vision. Referral?

Management vital points

- Explain Dx as eye condition that affects the central part of your vision due to degeneration of the cells in the centre of the retina with unknown reason by linked to some RFs as above. It doesn't cause total blindness but affect reading/ recognising faces/ daily activities
- Urgent referral to eye specialist within 1 week for possible injections
- Smoking cessation is critical to slow the progression
- Advise to avoid driving and arrange alternative transportation + inform DVLA
- Signpost to social prescriber for support and possible carer

Haematology

84 M + lab results suggestive of B-cell chronic lymphocytic leukaemia = CLL

Qs not to miss:

- Red flags: weight loss/ night sweating/ lymphadenopathy/ fever/ loss of appetite/ bleeding or bruising/ abdominal distension/ unexplained fatigue/ recurrent infections/ headaches
- Social Hx

Cues not to miss:

- Breaking bad news

ICE:

- Ideas: symptoms related to arthritis or age
- Concern: something serious due to weight loss and recurrent infection
- Expectation: Dx and treatment

Management vital points

- Explain Dx as a type of blood cancer that affect specific type of white blood cell called lymphocytes which help to fight infections
- Refer to haematology as 2 WW referral for further test and possible starting treatment which include chemo and immunotherapy
- Explain options of watchful waiting with regular monitoring (FBC/ U&Es/ LFTs) unless symptoms are severe
- Offer vaccinations like annual flu and 5 years pneumococcal

78+ feeling fatigue + bloods showing iron and folate deficiency + normal FIT = Low ferritin – poor diet

Qs not to miss:

- Red flags: weight loss/ night sweating/ / fever/ loss of appetite/ PR bleeding/ black stool/ change in bowel habits/ suicidal thoughts
- Impact on life and sleep

Cues not to miss:

- Social history and struggling to cope and cook/ loneliness

ICE:

- Ideas: No idea
- Concern: managing alone? Burden on children? Decline rapidly
- Expectation: Dx and treatment/ support for coping with wife's death and loneliness/ reassurance

Management vital points

- Explain Dx as blood showing nutritional deficiency especially low levels of iron storage and folate. It is likely related to your diet given depending on ready meals
- Encourage intake of iron and folate rich foods like lean meat and green veg
- Treat with iron and folate supplements + follow up after few months
- Signpost to social prescriber for social help and food (Meals on wheels) if eligible
- Referral for bereavement counselling and support group to cope with his loss
- Possible referral to a dietitian and mental health practitioner for talking therapy

60 F + rejected blood donation as blood count was too low = Myeloma	Qs not to miss: - Red flags: weight loss/ night sweating/ lymphadenopathy/ fever/ loss of appetite/ bleeding or bruising/ abdominal distension/ unexplained fatigue/ recurrent infections/ change in bowel habits/ black stool/ dysphagia/ back pain/ polyuria/ thirst/ Cauda equina symptoms/ pathological fractures - DDx: DM/ thyroids/ hyperparathyroidism Cues not to miss: - Caring for mum more challenging - Bone pain + fatigue_ weight loss + increased frequency ICE: - Ideas: stress or overexertion + back pain due to OA - Concern: declining ability to care for mother - Expectation: medications to manage fatigue and back pain Management vital points - Explain Dx as a type of cancer that affect certain cell inside your bones which usually help to protect from infections. This can also cause back pain and fatigue - Arrange bloods to confirm what's happening (myeloma screen) within 48 hours - Then if suspected to referral to haematology as 2 WW referral
8 M + intermittent leg pain + FBC showing anaemia and high retic count = Sick cell crisis	Qs not to miss: - Red flags: severe skeletal or abdominal pain (Acute painful crisis) chest pain/ SOB/ fever/ joint swelling/ fever/ headache or confusion/ pallor/ recurrent infection/ jaundice/ priapism/ stroke symptoms and seizures - RFs/triggers: cold weather/ dehydration/ exertion/ infection - Development/ vaccination/ screening and FH Cues not to miss: - Tummy feels funny at times + low grade fever - Was jaundiced for a short time as a baby + FH of blood problems - Children was born overseas (so not screened in the UK) ICE: - Ideas: no idea - Concern: no specific concern - only blood problems in the family - Expectation: Dx and treatment to get him well quickly Management vital points - Explain Dx as sick cell disease which a blood problem is affected the red blood cells which carry oxygen around the body and can cause such pains. - Refer urgently to acute paediatrics for further test such as Hb electrophoresis + provide good pain relief with paracetamol and ibuprofen - CXR if signs of acute chest syndrome (tachypnoea and chest pain) to exclude infections (common due to splenic dysfunction) - Possible need for blood transfusion if rapid fall in Hb (splenic sequestration/ aplasia or haemolysis)
67 F + AF on warfarin + low INR = Suboptimal INR	Qs not to miss: - Red flags: bleeding/ bruises/ falls/ chest pain or palpitations/ stroke - RFs for suboptimal INR: OTC supplement as St John's Wort/ compliance Cues not to miss: - St John's Wort for low mood – explore mood ICE: - Ideas: INR is fluctuating but no idea why - Concern: worried about switching to DOAC due to kidney function - Expectation: advice to stabilize INR Management vital points - Explain that INR is a measure of how well warfarin is thinning your blood to prevent stroke. A possible reason for low INR is interaction with St John's Wort which make warfarin doesn't stay in your system long enough to work properly - Recommend stopping the cause St John Wort and advise against OTC products - Close monitoring with warfarin clinic - Discuss options to swap to DOAC under careful monitoring - Discuss other options to support mood as counselling or referral to social prescriber given history of loneliness

**45 M + Patient
concerned about
FBC showing low
platelets of 125 with
normal examination
= Thrombocytopenia**

Qs not to miss:

- Red flags: weight loss/ night sweating/ lymphadenopathy/ fever/ loss of appetite/ bleeding or bruising/ abdominal distension/ unexplained fatigue/ recurrent infections/ headaches
- RFs: sulfa drugs/ heparin/ alcohol/ vitamin def/ pregnancy/ infections/ FH

Cues not to miss:

- Father had leukaemia – worried about cancer

ICE:

- Ideas: blood cancer or other blood disorder
- Concern: cancer? Dying early? Impact on family
- Expectation: urgent referral to haematologist

Management vital points

- Explain that platelets cells help blood to clot in case of injury or cut and it is slightly below the normal range but not low enough to suggest serious problem in absence of red flags. Mostly it is temporary and need repeating in 4-6 weeks
- Offer examination for more reassurance and exclude any red flags
- Consider bloods if concerned (repeat FBC/ blood film/ PT/ PTT/ LFTs/ ESR/ TSH) or folic or vitamin B12
- Address health anxiety and provide good reassurance and safety netting

Renal and urology

53 F + concerned about bloods showing AKI stage 2 after having diarrhoea = AKI

Qs not to miss:

- Red flags: blood in urine/ fever/ flank pain/ rigors/ anuria/ vomiting/ abdominal pain/ feeling unwell/ trauma/ bleeding/ SOB/ leg swelling
- DDx: dehydration/ infection/ nephrotoxicity
- Sick day rules – assess fluid status
- RFs: medications/ recent diarrhoea/ less fluids/ infection

Cues not to miss:

- Inform patient that this is not cancer first

ICE:

- Ideas: that you have stage 2 cancer
- Concern: bad news/ stage 2 cancer?
- Expectation: what is going on? Is it able to attend next trip within a week?

Management vital points

- Explain Dx as your kidney is not working as well as they should, and the stages is to categories how severe it is from stage 1 to stage 3. This is likely due to recent gastroenteritis
- Offer examination to check abdomen/ chest/ vitals/ signs of oedema
- Explain sick day rules and to stop nephrotoxic meds such as ACEIs/ metformin/ gliflozins
- Refer urgently if suspected obstruction or infection or dehydration or high K >6
- If no indications for referral, then 1ry care referral – advise maintaining fluid + sick day rule + regular monitoring, if possible, in few days
- Explain need to delay next trip and provide support letter for refund

39 F + blood in urine + treated with cellulitis 2 weeks ago = Post infectious glomerulo-nephritis

Qs not to miss:

- Red flags: BP > 180/120/ weight loss/ night sweating/ oliguria/ SOB/ confusion or seizures/ progressive deterioration in renal function

Cues not to miss:

- Urine like tea or cola + puffy eyes in the morning + ankle swelling + urine dipstick showing blood and protein + BP 160/95 (no Hx of HTN) + recent infection in the last 1-3 weeks

ICE:

- Ideas: UTI need antibiotics.
- Concern: symptoms impacting work and looking after her dad
- Expectation: Dx and treatment

Management vital points

- Explain Dx as inflammation of the kidney related to the recent infection causing your kidneys not to work properly as your body attacking itself. Unlikely to be UTI
- Offer examination to check vitals and signs of fluid overload + arrange urgent bloods + give advice about less salt and fluids
- Emergency admission if red flags OR urgent referral to SDEC if stable for urgent bloods (U&Es/ C3 & C4/ ASO/ FBC/ CRP) and possible starting medications to control blood pressure and ongoing monitoring
- Arrange urgent social care support for her dad

18 M + dipstick positive for blood for 6 weeks after treating UTI = Microscopic haematuria

Qs not to miss:

- Red flags: headaches/ weight loss/ night sweating/ SOB/ LUS/ trauma
- DDx: renal stones/ nephritis/ UTI/ cancer/ exercise or trauma/ medications
- RFs: Vigorous exercise/ medications as NSAIDs or anticoagulants

Cues not to miss:

- Play Rugby

ICE:

- Ideas no idea
- Concern: when mention kidney then concerned about ruptured kidney during rugby match? Able to play rugby
- Expectation: reassurance

Management vital points

- Explain Dx as tiny amounts of blood in your urine which can happen due to several reasons like after recent infection/ vigorous exercise/ underlying kidney issues
- Invite for examination to exclude DDx and assess vitals + arrange bloods and USS
- Refer if sings of glomerular disease (proteinuria or abnormal U&Es) or unexplained haematuria or age < 20 or history suggestive kidney disease

29 F + Hx of MS + urinary incontinence = urinary Incontinence

Qs not to miss:

- Red flags: weight loss/ fever/ night sweating/ blood in urine/ cauda equina symptoms/ urine retention
- RFs/ triggers: coughing/ sneezing/ laughing/ straining/ lifting heavy objects
- DDx: urge or stress or mixed or overflow incontinence/ UTI

Cues not to miss:

- Impact on daily activities

ICE:

- Ideas: due to flare up of MS or progression of disease
- Concern: impact on job security and social life
- Expectation: medications or referral to the continence clinic (no just pads)

Management vital points

- Explain Dx as urinary incontinence is common in people with MS as it affects your nerves that control the bladder leading to incontinence or retention
- Bladder diary to assess the pattern + MSU + advice about fluids
- Refer for post void residual scan – for self-catheterization if elevated
- Refer to pelvic exercise and bladder retraining exercises
- Offer medications such as Mirabegron or Oxybutynin
- Refer for 2ry care for urodynamic studies if symptoms worsen

**4 F + wetting herself
at night = Nocturnal
enuresis**

**8 F + ongoing
nocturnal enuresis
+ mum concerned
about school trip in
8 days = nocturnal
enuresis –
desmopressin**

Qs not to miss:

- Red flags: weight loss/ excessive stress/ blood in urine/ safeguarding concerns
- DDx: UTI/ constipation/ DM/ 1ry or 2ry (continent for > 6 months)
- RFs: stress/ excess fluids/ fizzy drinks/ caffeine/ constipation

Cues not to miss:

- Started nuresey 2 months ago – started having the symptoms 2 months before

ICE:

- Ideas: No idea
- Concern: is it normal? Something serious?
- Expectation: referral to specialist?

Management vital points

- Explain Dx as bedwetting is common and considered normal under the age of 5
- No need for referral as still 1ry as long as continent for just 2 months. Explain about RFs and need to have a dairy and record possible triggers and avoid RFs
- Urine sample to exclude infection and glucose in urine
- Reassure and encourage positive reward systems + discourage waking the child during the nights but if wakes at night, should be taken to toilet + avoid punishment
- If age > 5 years, then Offer desmopressin if rapid or short-term control (for example school trips) and offer enuresis alarm for long term treatment

Desmopressin case:

- Don't give if heart problems or VW disease – cautious with asthma due to risk of fluid retention and hyponatremia but safe if controlled
- Explain that desmopressin works by reducing amount of urine produced in the body at night from her kidneys
- It is usually taken at bedtime and child should have only sips of fluid from 1 hr before taking it until 8 hrs after and it is usually effective and once it is stopped, bedwetting comes bad.
- Use at least 1 week before the trip. Start with low dose then increase if needed after 1 week, can be used for long term if age >5 years with 3 monthly monitoring to stop for 1 week to check for dryness

**64 F + Recurrent UTI
= post- menopausal
recurrent UTI**

**23 F Recurrent UTI
– sex related**

Qs not to miss:

- Red flags: weight loss/ night sweating/ abdominal bloating/ blood in urine/ loss of appetite/ abdominal pain/ fullness after eating/ flank pain or rigors/ pregnancy
- DDx: UTI/ STI/ Urogenital atrophy/ cancers
- RFs: hygiene practice/ sexual activity/ less fluid intake/ used of personal care products / post menopause/ immunosuppression/ DM/ catheter

Cues not to miss:

- UTI frequency increased after having menopause 10 years ago
- Hx of breast cancer on tamoxifen

ICE:

- Ideas: why happen? How to stop?
- Concern: impact on social life and looking after her grandson in few weeks
- Expectation: options to discuss prevention of it

Management vital points

- Recurrent UTIs are common after menopause due to changes in the hormonal level with decreased oestrogen levels
- Refer to 2ry care if female >60 years + recurrent or persistent unexplained UTI OR recurrent upper UTI OR children < 16 years or transwomen or pregnant
- 2 WW if suspected cancer
- If acute – send MSU before starting antibiotics + don't prescribe antibiotics for asymptomatic bacteria except if pregnant
- Lifestyle: maintain adequate hydration >1.5 L a day + avoid douching or wearing occlusive underwear + wipe from front to back after defecation + avoid delaying urination + consider adding cranberry products
- Offer options such as topical vaginal oestrogen in postmenopausal (not in this case due to breast cancer Hx) or prophylactic antibiotics either as single dose post coital prophylaxis or long-term low dose daily prophylaxis or daily antiseptic methenamine Hippurate if not pregnant
- Follow up within 6 months

**Recurrent UTI – Male
+ long term catheter
+ paraplegia =
Catheter related
recurrent UTI**

Qs not to miss:

- Red flags as above
- RFs: inadequate catheter care

Cues not to miss:

- Not emptying catheter regularly bag due to mobility issues

ICE:

- Ideas: UTI needs antibiotics
- Concern: frequency of infections/ impact on life
- Expectation: treatment and prophylactic antibiotics

Management vital points

- Explain that recurrent UTI are common in patient with long term urinary catheter due to bacteria directly introduced to the bladder especially if the catheter is not emptied regularly or if in place for extended period
- Catheter care advice: ensure catheter correct position and draining + advice changing the catheter regularly every 7 days + advice about regular change of the bag
- Treat acute infection with antibiotics for 7 days + catheter change + follow up
- Involve district nurses to support catheter care + reinforce catheter hygiene catheter (keep the bag below the bladder level and empty regularly)
- Address need for referral to social prescriber

Respiratory

22 M + ongoing cough + normal peak flow = Asthma – new

Qs not to miss:

- Red flags: sudden chest pain/ palpitations/ SOB/ haemoptosis/ weight loss/ trauma/ night sweating/ calf muscle tenderness/ orthopnoea/ leg swelling/ dizziness/ confused
- RFs/ triggers: cold/ FH/ smoking/ environmental triggers/ exercise/ infections/ non-compliance to medications

Cues not to miss:

- Breathing is generally fine except when you play Rugby +/- slight wheezing + FH + symptom patterns such as nighttime or seasonal worsening

ICE:

- Ideas: damp and mould in house causing it?
- Concern: impact on Rugby
- Expectation: reassurance

Management vital points

- Explain asthma as common lifelong condition that affects airways causing narrowing of air ways usually with FH and with some triggers like exercise. The good news that we have various options to control it
- To confirm Dx: FBC and FeNO (> 50ppb in adults and >35 in children) or with spirometry (increase > 12% from baseline) or PEF variability (>20% over 2 weeks)
- 1st line > 12 years: as needed Air therapy with low dose inhaler steroids/ formoterol when needed for short term relief of symptoms given his coming matches
- Then shift to low dose maintenance and reliever therapy (MART) if more symptomatic. Consider adding LTRA or LAMA to moderate dose MART if not controlled
- If children > 5 years: 1st line is BD low dose ICS + SABA as needed, then shift to low dose MART if not controlled. If children < 5 years, then trial of low dose ICS with SABA and review in 3 months for pause and reassess
- Triggers diary and try to avoid all possible triggers

Paediatric asthma case

- Clue: not using inhalers as advised (only when needed) – no Dx of asthma
- For Dx – FeNO or Spirometry or PEF
- Educate parents importance of daily use of preventer inhaler with a spacer
- Asthma plan at home and at school including emergency inhalers for school
- Consider change to low dose MART if no better after trying good compliance
- Check inhaler technique and follow up in 8-12 weeks + annual reviews

Asthma concern over allergy case

- Cues – rarely uses the spacer + symptoms not getting worse after petting animals or being around grasses + PF 380/450 + night times symptoms + frequent use of blue inhalers and variability of peak flow readings + no skin reactions
- Expecting urgent referral for allergy studies
- Explain unlikely related to allergens due to cues above
- Offer trial of alternative inhalers that is easier to use due to cues above like Breath actuated inhalers (act by automatic medication release upon inhalation) or DPIs

Note: acute cough < weeks and chronic cough > 8 weeks

7 F + on inhalers + no Dx of asthma + ongoing cough = paediatric asthma

16 F + Hx of poor inhaler techniques with spacer prescribed + mum requesting allergy test = Asthma concern over allergy

58 F + Hx of recurrent chest infection + spirometry showing obstructive pattern = COPD

Qs not to miss:

- Red flags: sudden chest pain/ palpitations/ SOB/ Haemoptysis / weight loss/ trauma/ night sweating/ calf muscle tenderness/ orthopnoea/ leg swelling/ unwell
- RFs: smoking/ occupational exposure
- DDx asthma (as above) or interstitial lung disease/ HF
- Screen for depression and impact on life

Cues not to miss:

- Heavy smoker/ worked in Bakery + FEV1/FVC < 70% + FEV1 48% (Stage 3)

ICE:

- Ideas: one of the persons prone to chest infections
- Concern: not concerned
- Expectation: antibiotics when have chest infection. When COPD mentioned, impact on life? Medications for how long?

Management vital points

- Explain Dx as a long-term lung condition where the air flow is obstructed due to damage of the airways due to smoking. It is not curable but is controlled with some medications to slow progression and improve the symptoms.
- Stop smoking is the main treatment – offer referral for smoking clinic for other options such as patches or medications
- Offer pneumococcal and annual influenza vaccine
- Off pulmonary rehabilitation for all patient with recent exacerbation or uncontrolled patients
- 1st line starts with SABA or SAMA when needed then if no improvement offers regular LABA + LAMA inhaler then if no better offer triple therapy LABA+ LAMA+ ICS
- Discuss inhalers options and use spacers if needed
- Consider rescue medications if exacerbation + follow up with nurse

63 M + recurrent COPD exacerbation + frequent DNA + exacerbation in homeless

Qs not to miss:

- Red flags as above
- RFs: smoking/ non-compliance to medications
- Social Hx: homeless/ not have mobile

Cues not to miss:

- Notes about frequent DNAs and looks unkempt
- Struggle with appointments and collection of meds

ICE:

- Ideas: COPD + need antibiotics and steroids
- Concern: struggle to access regular healthcare
- Expectation: antibiotics + steroids + inhalers + put note that he has no phone or address

Management vital points

- Explain Dx as flare up of your COPD when your usual symptoms suddenly get worse usually due to infections. It happens more frequently due to noncompliance to the inhalers which I can understand why. It is important to treat your acute symptoms with antibiotics (if increased sputum/ purulence of signs of pneumonia) and steroids + increase frequency of inhalers
- Treat just infection might be dangerous and can impact your quality of life
- Offer options to make it easy for him like change his registered address to the practice or friend address + 3 months prescriptions at a time or repeat dispensing + ensure the patient has fast access to GP with flexible appointment + note that this patient has no registered address and to avoid letters or text messages
- Refer to social prescriber to help with the social situation
- Advise about stopping smoking
- Try to find a third-party mobile number which can be a flatmate or any post office or supermarket where he usually go to

**73 F + paramedic
requesting
antibiotics for chest
infection = PE**

Qs not to miss:

- Red flags: sudden chest pain/ palpitations/ SOB/ Haemoptysis / weight loss/ trauma/ night sweating/ calf muscle tenderness/ orthopnoea/ leg swelling/ unwell
- RFs: recent immobilization or hospitalization or long flight/ FH/ cancer

Cues not to miss:

- Sudden sharp chest pain in the right upper part of the back + worse with breathing + SOB then started having cough with no other systemic features

ICE:

- Ideas: chest infection
- Concern: leaving dog unattended is difficult
- Expectation: to receive some antibiotics

Management vital points

- Explain Dx as a blood clot that develops in a blood vessel in the lung which block the blood supply part of the lung causing these symptoms. It is unlikely to be infection due to the upper cues and PE can sometimes present with similar symptoms to a chest infection that's why might be misleading to my colleague. As GPs we have different scope and exposure, and it is not uncommon to have different opinions.
- Refer urgently to the hospital for suspected PE as it is an emergency and any delay can be life threatening + give options to find someone to look after her dog.

**69 M + Lung cancer
+ photo of rash not
responding to
antifungal**

Qs not to miss:

- Red flags as above
- RFs smoking/ FH

Cues not to miss:

- Cough > 4 weeks + loss of appetite + loss of weight + rash not responding to antifungal (erythema gyratum repens) + rusty coloured sputum

ICE:

- Ideas: something is wrong
- Concern: worst thing is cancer and promised daughter to be with her in her wedding
- Expectation: reassurance from GP

Management vital points

- Explain Dx with breaking bad news mentioned cancer as a worst-case scenario given the above cues
- Urgent CXR with 2 weeks (or direct 2 WW referral if haemoptysis) – if abnormal then refer as 2 WW for CT scan
- Explain that rash may be related, and treatment of the cause will help
- Offer counselling and psychological support

79 M + deteriorating metastatic Lung cancer + speak to wife = palliative lung cancer

Qs not to miss:

- Red flags: agitation/ severe pain/ vomiting/ respiratory secretions
- Advanced care plan: DNACPR/ SOI/ Preferred place of care and death
- Screen for carer stress and depression

Cues not to miss:

- Need form for financial support (called SR1 form)

ICE:

- Ideas: Actively dying? Suffering?
- Concern: financial problems/ he is suffering? What to expect if dying? What is next? Not getting his morphine as not able to swallow
- Expectation: fill a form that the palliative nurse mentioned to help financially

Management vital points

- Explain our aim to ensure that he is comfortable and providing relief of symptoms by various medications tailored for his need for pain/ agitation/ any secretions.
- Explain available financial support options for those who are terminally ill and their families like Disability living allowance, personal independence payment, attendance allowance and care's allowance. Also, possibility to have support toward funeral expenses. The palliative team will help with that as well
- Prescribe anticipatory SC medications such as SC morphine
- Involve district nurses/ update OOH
- Possible stopping of other regular medications as no benefits
- If passed away to ring OOH or high own GP to confirm death and ring district nurses to remove syringe driver
- Fill SR1 (patient with progressive disease and expected dying within 12 months for financial support - <https://www.gov.uk/government/publications/dwp-factual-medical-reports-guidance-for-healthcare-professionals/e5b3b502-9067-4b20-93c0-1006d9cb0edb>)

Notes: if suspecting pneumonia, Use CRB-65 score to assess severity of CAP

Each of the following has 1 point; 2 points or more indicates severe CAP

- Confusion (new disorientation in person, place, or time)
- RR >30/min
- sBP <90; dBP <60
- Age 65 years or more

Genomic medicine

28 F + concerned about CF risk in his children as his brother got CF = Cystic fibrosis

Qs not to miss:

- RFs: FH (mum or dad)
- Ask about symptoms – recurrent chest infection

Cues not to miss:

- Autosomal recessive (both patient needs to be carrier)

ICE:

- Ideas: risk is low even if he is a carrier? new medication can reverse CF?
- Concern: don't want a child to suffer with CF
- Expectation: reassured about low risk and more explanation

Management vital points

- Explain cystic fibrosis as a serious inherited genetic disorder that mainly affects the lungs lead to inability to clear the mucus causing repeated chest infection, and also can affect other parts such as the gut affecting digestion causing poor growth.
- Explain incidence in general population is 1/2500 and the carrier state is 1/25. If both parents are carrier risk of having it is 1/4 and to be a carrier is 2/4 and 1/4 will be normal. In her case he is either a carrier 66% or normal 33% and her partner is either a carrier 4% or normal which give low risk of having a baby with CF of 0.65%
- Explain that the new Dx Kaftrio improves the prognosis and prolong life expectancy to 82.5 if initiated between 12 and 17 years old
- Explain that newborn screening includes CF at birth, and it is possible to test it during pregnancy using amniocentesis or CVS
- Offer referral for genetic testing for her (given his brother has CF) then offer testing for her partner if proved to be a carrier

36 F + mum passed away at 61 with brain bleed + post-partum showed PKD = Polycystic kidney disease

Qs not to miss:

- Red flags: abdominal pain or swelling/ blood in urine/ headache/ dizziness

Cues not to miss:

- Mum sudden death with no PMH
- Wanted to be tested for it and her kids
- It is autosomal dominant

ICE:

- Ideas: This condition can be passed on
- Concern: to know about it? Chances of, having it? Any cure? Will have brain bleed?
- Expectation: to be tested and also her children

Management vital points

- Explain PKD as an inherited genetic condition that causes small fluid filled cysts to form in the kidney that might cause high blood pressure and kidney failure when getting old. It can be associated with dilated vessels in the brain in <10%. If a person has a faulty gene, then every child they 50:50 chance of inhering the faulty gene. It is autosomal dominant which means one faulty gene is enough to have it
- Offer renal USS to help to confirm or rule out PKD – but negative result doesn't exclude it + blood pressure monitoring is essential
- Offer referral for genetic testing for her and discuss that testing her children might do more anxiety than benefit and to leave it until they can decide themselves as it is usually asymptomatic < 30 years
- Screen for brain aneurysms if FH – be referral to neurology + safety netting
-

42 F + 1st pregnancy + wants to know more about Down's syndrome	Qs not to miss: - Red flags: abdominal pain/ PV bleeding/ vomiting/ nausea - RFs: age/ FH - Sexual Hx/ previous children/ fertility issues/ folic acid - Social Hx: partner/ work/ support/ smoking and alcohol Cues not to miss: - Midwife mentioned that I qualify for amniocentesis given high risk ICE: - Ideas: Do I need to go through all test due to high risk? - Concern: risks of the tests? If Down's, can I terminate? - Expectation: what is next? Management vital points - Explain Down's syndrome as a genetic condition due to extra copy of chromosome 21 in the cells of their body. They have some level of learning disability with different level of independent. - Explain risk of Down's as 1:100 at 40 years (1:50 > 45) - For people < 40 years, usually offered screening test (the combined test) with a combination of blood test and SS to check nuchal thickness. If screening risk came as 1/150 or more then offer diagnostic test with CVS or amniocentesis with a miscarriage risk of 0.5 and 2% respectively - Regarding concerns about termination, can refer to other services who can provide help to answer her questions and offer information that most of them have go on live and a lot of support
21 F + dad died at 51 + concerned about post-mortem showed hemochromatosis	Qs not to miss: - Red flags: Diabetes/ abdominal distension/ vomiting of blood/ abdominal pain/ SOB or chest pain - FH: other family members Cues not to miss: - Sudden death of her dad at 51 years - It is autosomal recessive as in CF ICE: - Ideas: want to know about the disease? Prognosis? Treatment? why not picked earlier in dad? - Concern: with tiredness and skin tan, possibility of having it too? - Expectation: explain and reassurance Management vital points - Explain haemochromatosis as an inherited genetic condition causing the body to absorb too much iron from the diet with excess iron can damage the organs. When treatment start early, prognosis is very good, however untreated cases can lead to complications as cirrhosis, DM, cardiac problems (sudden death) - Offer screening with transferrin saturation (suspect if > 45%) and ferritin levels (suspect if > 300 in men and > 200 in women). Also, hemochromatosis UK offer gene testing in the post for 49£ - Explain the mode of inheritance as autosomal recessive – need both parents to have the faulty to have it but she will be carrier
35 M + patient terrified about letter with new Dx of Huntington's syndrome	Qs not to miss: - Symptoms: clumsiness/ forgetful/ jerking movement/ muscle problems as rigidity/ balance problems/ speech or swallowing difficulty/ depression/ psychosis Cues not to miss: - Has 2 sons – autosomal dominant ICE: - Ideas: no clear idea about this disease - Concern: the future, impact on health, work, children? - Expectation: explain this diagnosis and next steps Management vital points - Explain Dx as inherited genetic condition affecting the brain and causes changes in movement, thinking, behaviour with symptoms appear between 30-50 years with progressive course and no cure. There is 50% chance to pass it to your children - Explain available help and support with MDT with neurologist and community with medications and social support + regular monitoring and advanced care planning - Discuss testing in children might lead to psychological implications on children

Smoking, alcohol and substance abuse

52 M + worried about drinking much alcohol = Alcohol misuse

Qs not to miss:

- Red flags: tremors/ confusion/ seizures/hallucinations/agitations/ safety/ balance issues/ nystagmus/ vision problem
- Ask about alcohol misuse/dependence: impaired control over drinking/ for at least 3 months continuously or intermittently over 12 months persistence despite harm/ intense urge to consume alcohol/ drink 1st thing in the morning. increase more alcohol to achieve desired effects (tolerance)/ symptoms with try to reduce or stop it for few hours to days (withdrawal)/ impact on social or interpersonal relationship (harmful alcohol use)
- Ask: Do u feel guilty about drinking? Plans for cutting down?
- Impact on Driving

Cues not to miss:

- Audit score 13 / around 52 unites per week/ drink mainly in the evening
- Works as a bus driver – has to call in sick 5 times in the last 6 months due to alcohol intoxication

ICE:

- Ideas: drink too much due to habit or bored. Don't think I'm alcoholic with normal blood test recently
- Concern: excess alcohol is damaging for health long term and the liver
- Expectation: guide to cut it down – no need to attend alcoholic services

Management vital points

- Explain Dx as alcohol misuse (not dependant or abuse yet) when you drink in a way that's harmful, or when you're dependent on alcohol. given high risk Audit score
- Discuss options to cut down drinking such as putting weekly limit on number of drinks/ choosing lighter drinks or spacing them out with water/ try hobbies you enjoy/ make yourself busy/ delay the first drink/ avoid trigger such as sitting at home in the evening doing nothing by doing exercise or seek support from others
- Refer to ACHIEVE
- Follo up to review progress and offer support
- Explain that he must not drive if persistently misuse alcohol and legally required to inform DVLA – has to control drinking or complete abstinence for at least one year
- Can arrange USS scan to check liver and assess fatty liver

29 M + Hx Alcohol excess + requesting sick note = alcohol misuse and Driving

Qs not to miss:

- Hx as above + ask about stress and mood

Cues not to miss:

- Works as a driver – caught driving under the influence of alcohol 3 months ago and been given 3-month ban – doesn't want to tell his employer not to be fired

ICE:

- Ideas: want sick note to avoid going to work and explain the situation with his boss
- Concern: if work know about the conviction – will fire him
- Expectation: no to list the conviction on the sick note and to write stress

Management vital points

- Explain the sick notes should be based on genuine impact on ability to work. However, we can offer support with regular review s and help to cope
- Encourage him to speak to his employer as they should offer support
- Possible to issue short term sick note for 1-2 weeks to start stress management but cannot something that is not true and can not do it for 3 months
- Discuss reducing alcohol to the recommended amount 14 units/ week

**71 F + A&E letter
with recurrent falls
after Alcohol excess
= alcohol excess
safeguarding**

Qs not to miss:

- Hx as above + Ask about Hx of falls + ask about her granddaughter (impact on control their emotions with angry outburst? Child safety? Who else there?)

Cues not to miss:

- Audit 13/40
- Caring for her granddaughter 3 times a week

ICE:

- Ideas: A&E doctors are overcautious
- Concern: that GP will inform her daughter about her drinking habits
- Expectation: GP will leave me alone as I drink only 2 nights per week with friends

Management vital points

- Explain alcohol misuse as before and its impact on the patient safety giving recurrent falls that might lead to serious fractures or brain bleed
- Explain that sharing information with her daughter is necessary to ensure the child safety. If thinks child at high risk might need to break confidentiality
- Advice about alcohol cessation as last 2 cases
- Arrange follow up to discuss fracture risk and preventive treatment with vita D and calcium – advise about driving
- Offer referral to occupational therapy to do home adaptation to prevent falls

**31 F + want to stop
smoking = Smoking
cessation**

Qs not to miss:

- Ask about symptoms: cough/ chest pain/ SOB/ weight loss/ night sweating
- Discuss barrier to smoking / previous attempts and relapses
- Triggers: places/ friends/ stress/ withdrawal

Cues not to miss:

- New relationship with new partner support stopping smoking

ICE:

- Ideas: you will do anything to stop smoking
- Concern: impact with relationship with her boyfriend
- Expectation: E-cigarettes or tablets?

Management vital points

- Explain benefits of stopping smoking and consequences of keep smoking (reduced fertility and pregnancy issues/ breathing problems/lung cancer)
- Discuss stages: set a quit date, encourage preparations (remove smoking-related triggers and inform friends of your plan), discuss withdrawal symptoms and coping strategies)
- Refer to local NHS stop smoking services for behavioural support and medical options as nicotine replacement patches for 8-12 weeks + Bupropion tablets (not in this patient due to seizures Hx, so varenicline is recommended here) to reduce withdrawal symptoms (start while still smoking and set quit date for 1-2 weeks)
- Explain that E-cigarettes are 95% less harmful than smoking

**22 M + feeling down
and irritable =
Cannabis abuse**

Qs not to miss:

- Red flags: severe anxiety/ suicidal thoughts
- RFs: Coming exams/ isolation – coping with Marijuana
- Symptoms of cannabis: low mood, irritability, difficult concentrating, euphoria, altered perception + physical effects (dry mouth, bloodshot eyes, increase HR.
- Withdrawal symptoms: irritability, anger, insomnia, anxiety and physical symptoms (headaches, abdominal cramps 4-7 days after cessation)

Cues not to miss:

- Using Marijuana (we have to ask about drugs)

ICE:

- Ideas: marijuana helping to relax
- Concern: coming exams and might fail
- Expectation: help from GP

Management vital points

- Explain that cannabis (illegal) even it causes relations but can actually contribute to feeling low mood and irritability with difficult concentrating + can impact your motivation and ability to cope with stress.
- Explain risk of cannabis abuse: cognitive impairment, impaired memory, depression and anxiety, risk of schizophrenia, psychosis, cannabis use disorder (craving, tolerance and withdrawal symptoms)
- Advise to stop cannabis + referral to ACHIEVE if needed + stress management advice and signposting to relaxation therapy and meditations

**25 F + Heroin abuse
in pregnancy**

Qs not to miss:

- Red flags of pregnancy: abdominal pain/ vaginal bleeding/ dizziness
- Red flags of heroin: safety including driving/ self-harm
- Triggers: stress/ withdrawal symptoms/ financial issues/ lack of support
- Ask about other drugs/ smoking or injections
- Social History and safeguarding issues

Cues not to miss:

- Worried about heroin in pregnancy + smoking as well + has sex for money to pay for heroin

ICE:

- Ideas: uncertain about heroin effects in baby
- Concern: impact on baby/ baby taken away by social services/ withdrawal symptoms
- Expectation: nonjudgmental space to get help. Support for her and baby

Management vital points

- Explain that drugs as heroin can be harmful to both you and your baby with higher risk of miscarriage, premature birth and effect on the baby development.
- Urgent referral to specialist services (ACHIEVE) for opioid substitution therapy with methadone or buprenorphine + urgent antenatal care referral for monitoring
- Explain needed exchange programs to prevent infections if still using
- Smoking cessation advice
- Screen for infections (HIV/B/C/syphilis) + refer to sexual health clinic
- Recommend folic acid and vitamin D + dietary advice
- Consider involving social services if baby is at risk

**15 F + recurrent faint
at school + irregular
HR = Drug use
(ecstasy/ Molly) at
school**

Qs not to miss:

- Before, during and after the episode
- Red flags: chest pain/ SOB/ palpitations/ seizures/ FH of sudden death
- Symptoms of MDMA: hypertension/ fainting/ hyperthermia/ restless legs/ lack of appetite, jaw clenching, joint stiffness

Cues not to miss:

- Only happen after morning break in school and settle after few hours + atrial ectopic
- Will disclose only when mum asked to leave the room – that he is taking MDMA from friends

ICE: only when mum leave the room

- Ideas: you know it is wrong and dangerous to take but pressurised to take it
- Concern: permanent damage to the heart/ don't want mum or school to know
- Expectation: to fix the heart and promised not to take again

Management vital points

- Explain the risk of having such drugs – even it makes you feel happy and energetic for a while – but it does over stimulation of the brain and heart which is harmful. It can raise body temperature, dehydration which can be life threatening like heatstroke.
- Effect of ecstasy can last upto 6 hours only so safety netting within this period
- Main point is negotiation confidentiality by encourage to inform mum but if not so discuss writing anonymized letter to the school stating what is happening

Neurodevelopmental

10 M + mum concerned about his behaviour as suspended from school again= ADHD

Qs not to miss:

- Red flags: headache/ seizures/ developmental issues/ safeguarding issues

Cues not to miss:

- Running around breaking things/ so much energy/ never sit still/ less sleeping/ easily bored/ struggle to focus/ below average in school
- To Dx: age <12 + symptoms in more than one setting and affect all activities + for > 6months + significantly impact the child's functioning

ICE:

- Ideas: ADHD?
- Concern: worried he will be taken away as people might think you're a bad mother
- Expectation: Due to long waiting list, we need to double check if a company called psychiatry assessments which hold Right to Choose contract is accepted to do such ADHD Dx. Also, to prescribed sedative or melatonin to help him sleep

Management vital points

- Explain Dx as a common condition that affect behaviour of the children causing difficulties in concentration, restlessness and impulsive actions
- Offer evidence-based parenting programmed and behavioural therapy the child
- Need referral for 2ry care for medications. Patient can be referred to right to choose provider as long as they have a contract to provide ADHD services
- Re-sleeping, advice about sleep hygiene and avoid stimulants or screen then if no better to refer to specialist to prescribe melatonin

2 Y +mum worried about his behaviour as not interested in things and always on the quite side = Autistic spectrum disorders

Qs not to miss:

- Red flags: developmental problems/ seizures/ safeguarding
- Ask about symptoms as delayed speech/ strong attachment to certain toys/ perfectionism/ less friends

Cues not to miss:

- Always on the quite side + delayed taking until 15 months + not interested in things + prefer one toy all times + not play with others

ICE:

- Ideas: thinks that he is just a bit slow in developing
- Concern: don't want horrible tests and don't want to give a label of autism if doesn't have it
- Expectation: Reassurance

Management vital points

- Explain Dx as a group of condition that are usually Dx in childhood when children have difficulty with communicating to other people, interacting in social situations and struggle with things involve imagination
- Offer examination then refer if suspected to community paedrs for MDT assessment (paeds/ SALT/ mental health)
- Offer support to mum by signposting to support groups and parenting support

**26 M+ Hx of autism
+ carer concerned
about tics/ epilepsy
= Autism stimming**

Qs not to miss:

- Red flags: head trauma/ aggression/ safety/ infection symptoms
- Triggers: change in environmental factors/ constipation/ pain/ infection/ stress
- DDX epilepsy (biting tongue/ incontinence/ loss of consciousness/ post ictal fatigue/ FH) / medications side effects

Cues not to miss:

- Tics + sudden loud scream

ICE:

- Ideas: myoclonus epilepsy?
- Concern: Wants antiepileptic medications + possible need for oral lorazepam in case of seizures
- Expectation: as concerns + how to handle these outbreaks? Need to increase aripiprazole dose?

Management vital points

- Explain Dx as stimming is a self-soothing mechanism which is common in autism when used as a way to manage sensory overload, reduce anxiety or express emotions. It is natural and necessary aspect of autism and not harmful. Unlikely to be epilepsy as no symptoms of it
- Understand the purpose of stimming and identify the triggers for example sensory overloads or change in routine (change staff for example) – so avoid triggers and create a supportive and safe environment
- Educate staff and residents to be more supportive and avoid any aggression

**45 F + congenital
hemiplegia +
concerned about
discrimination at
work = Disability
discrimination**

Qs not to miss:

- Red flags: suicidal thoughts

Cues not to miss:

- Requesting letter stating discrimination at work as per legal advice

ICE:

- Ideas: what a letter to support her but not sure about information in it
- Concern: without this letter, not able to keep this job and progress
- Expectation: GP write a letter specify bullying at work due to disability and specify what you can do and what you can not

Management vital points

- Give options for supporting her such as joining a union to get their help/ taking some time off as work stress (as bullying may cause problems)
- Explain that GPs can't decide what you can do and what you can't and offer referral to occupational health department or Citizen's Advice Bureau)
- Explain that letter can't include subjective matters such as being bullies as can't approve it and should stick only to facts (describe symptoms + effect of symptoms + treatment they are receiving + symptoms if they stopped treatment + Dx of conditions + duration of symptoms + prognosis + copy of records)

**46 F + Down's
syndrome + carer
concerned about
behavioural change**

Qs not to miss:

- Red flags: Signs of infection (chest/ UTI)/abnormal bleeding / trauma /seizures / self-harm/ signs of abuse (bruises/ fear)
- RFs: constipation/ pain/ medications change/ environmental changes/ drugs/ vision or hearing problems

Cues not to miss:

- Increased aggression + less appetite + weight loss + withdrawn + new incontinence + disturbed sleep + confusion + amenorrhea + more forgetful

ICE:

- Ideas: something wrong?
- Concern: hurting herself or others unintentionally – new staff might unsettle her?
- Expectation: diagnosis and referral if needed

Management vital points

- Explain DDX in this case given her Hx as early onset Alzheimer's or dementia, depression, thyroidal problems, hormonal changes (menopause), vitamin deficiencies, or environmental changes, medications side effects, UTI or abuse.
- First to offer home visit then possible need for further test to exclude possible DDX and possible referral for neuroimaging then finally to the memory clinic

Metabolic problems and endocrinology

**35 F+ Hx of hypo-thyroidism
+constant tiredness
+ blood showing
borderline low Na
and high K with
normal TSH =
Addison's crisis**

Qs not to miss:

- Red flags of tiredness + of Addison's: weight loss/ night sweating/ change in bowel habits/ bleeding/ lymphadenopathy/ V/D/ unwell/ dizziness/ acute abdominal pain
- DDx: cancer/ thyroid/ anaemia - RFs: stress/ autoimmune diseases

Cues not to miss:

- Extreme fatigue + muscle weakness + low Na + high K + weight loss + appetite loss + skin discoloration + FH of autoimmune disease + salt craving
- Patient has vomiting and diarrhoea

ICE:

- Ideas: thyroid glands need adjustment
- Concern: impact on work and being a mother
- Expectation: some tests to find out what's wrong and treatment

Management vital points

- Explain Dx as Addison's disease which means your adrenal glands are not producing enough hormones to help with energy, blood pressure, and salt balance. This can lead to Addisonian crisis that need urgent care
- If stable, arrange morning cortisol level and U&Es then refer to 2ry care for more tests and scans if suspected
- If unstable, urgent admission for IVF and hydrocortisone (can be initiated in 1ry care if expected delay in ambulance)
- If patient started on medications, educate about sick day rules and need to increase hydrocortisone dose if stress/ illness/ injury

**31 F + weird
symptoms
(abdominal pain/
constipation/ renal
colic)
Hypercalcemia**

Qs not to miss:

- Red flags: pregnancy related problems / PR bleeding/ weight loss/ fever/ abnormal PV bleeding/ suicidal thoughts/ bone pain/ palpitations
- RFs: calcium intake/ OTC vitamins/ antacids

Cues not to miss:

- New onset renal colic + abdominal symptoms of pain and constipations + mood changes + fatigue + bone pain + polyuria, polydipsia

ICE:

- Ideas: related to change to new pills?
- Concern: something sister
- Expectation: quick fix

Management vital points

- Explain Dx as possibility of having high calcium, so we need to do more bloods to confirm it and exclude other DDx
- Renal USS to identify kidney stones and urine drip
- If patient has calcium above 3.4 or symptomatic, to speak to urgent medical unit
- Refer as 2ww if suspected cancer
- Lifestyle – more fluids – less calcium – avoid OTC calcium

45 F + poorly controlled HTN + hypokalaemia = Primary hyperaldosteronism	Qs not to miss: - Red flags: palpitations/ chest pain/ severe headache/ dizziness/ vision changes - DDx Addison's/ thyroid problems Cues not to miss: - Low K + high normal Na+ Intermittent headaches/ muscle cramps/ tiredness/ urinary frequency +polydipsia all the time + early onset HTN + fatigue worse in the afternoon and feels lightheaded when standing up quickly ICE: - Ideas: something wrong? High BP related - Concern: serious problems? Heart disease? - Expectation: Dx and treatment Management vital points - Explain Conn's disease as condition where the adrenal glands which sit above the kidney produce too much hormone called aldosterone which regulate your BP and salt and water in the body. This led to low K that cause muscle weakness and fatigue. It has good prognosis usually - Arrange aldosterone – renin ratio to confirm Dx then Refer to 2ry care for more test and imaging - Offer spironolactone or eplerenone to manage symptoms + lifestyle changes with low salt intake + Monitor K in the meantime
62 F + feeling HRT is not working + multiple medications for neuropathy = Serotonin syndrome	Qs not to miss: - Red flags: high fever/ seizures/ irregular heartbeats/ hallucinations/ spasms - DDx menopause (irregular bleeding/ vaginal dryness/ mood swings) Cues not to miss: - Patient has the symptoms since increased duloxetine + Patient on SSRI/ SNRI/ Tramadol/ pregabalin +Sweats (but no hot flushes) + tachycardia + high BP + feeling agitated + muscle spasms ICE: - Ideas: HRT medications need increasing - Concern: impact on work - Expectation: increase dose of HRT Management vital points - Explain Dx as excess serotonin, a chemical in the brain, can cause all these symptoms and if severe can cause fever and seizures. This is due serotonergic medications (citalopram/ duloxetine/ pregabalin/ tramadol) - In this case, can be managed in 1ry care given no red flags – stop medications + follow up + good safety netting
32 M + bilateral gynecomastia = testicular cancer 69 M + HF + taking spironolactone + gynecomastia	Qs not to miss: - Red flags: pituitary (headache vision problems)/ testicular (masses/ pain)/ adrenal - RFs: medications (spironolactone/ finasteride/ ranitidine/ digoxin)/ marijuana/ alcohol/ anabolic steroids/ liver cirrhosis/ obesity - Impact on the patient Cues not to miss: - Patient has testicular lump ICE: - Ideas: due to weight training? - Concern: coming beach season - Expectation: tamoxifen? Management vital points - FTF appt to check genitals + 2WW referral - Bloods to exclude DDx (LFTs/TFTs/ U&Es/ testosterone/ LH/ FSH/ prolactin) + tumour markers (AFP/hCG) - Explain that gynecomastia usually resolves after treating the cause - Tamoxifen is one of the options for persistent symptoms under the specialist or other options such as cosmetic surgery - Encourage weight loss Case with gynecomastia taking spironolactone - Replace with eplerenone after specialist input –then breast size typically reduces over time over few weeks to months

75 M + Erectile dysfunction

Qs not to miss:

- Red flags: testicular lumps/ weight loss/ back pain/ LUS/ blood in urine/priapism
- Assess cardiac risk to make sure safety of sexual activity

Cues not to miss:

- Patient has DM/ angina on GTN spray

ICE:

- Ideas: current Viagra/ Cialis not working
- Concern: cost of potential treatment/ quick fix
- Expectation: quick fix

Management vital points

- Offer ftf examination for testicular and prostatic/ genital examination
- Discuss options: vacuum pump to draw blood into the penis or Alprostadil (stick/ gel/ injection) to dilate the vessels
- Advise about lifestyle changes with better control of DM/ cholesterol/ BP
- Explain that he can get free prescription via NHS as he got DM. Advise that sildenafil 50 mg tablets (Viagra Connect®) can be purchased OTC
- Ensure correct maximum doses and duration (4-8 ti) of PDE-5 inhibitors before switching to other options.
- Offer psychotherapy

48 M + Wanting Viagra on repeat = Low testosterone

Qs not to miss:

- Red flags: testicular lumps / LUS/ back pain/ gynecomastia/ headache/ blurred vision)
- DDx psychological or organic (morning erection/ desire)
- Ask about sexual orientation/ ejaculation/ libido
- RFs: smoking, alcohol, drugs, cycling, obesity

Cues not to miss:

- ED responding to Viagra + low libido + tired/less energy + beard growth slowing down + less hairy

ICE:

- Ideas: getting older
- Concern: wife getting upset
- Expectation: To add this medication to your repeat

Management vital points

- Explain ED as a condition where there is difficulty or maintaining an erection sufficient for sexual activity. It can be caused by different factors including hormonal, lifestyle, organic and psychological factors
- Offer bloods HbA1C, TSH, testosterone, PSA, LFT, U&Es, fasting testosterone (repeat if low with FSH/ LH/ SHBG/ prolactin)
- Offer examination for prostate examination and external genitalia
- Can prescribe non branded sildenafil
- Lifestyle changes + psychosexual therapy

55 M + HTN + tired and weak + requesting Testosterone replacement therapy	Qs not to miss: - Red flags: testicular lumps/ weight loss/ night sweats - Ask about symptoms of testosterone def: reduced testicles size/ less body hair/ gynecomastia/ hot flushes/ less libido/ absent morning erections Cues not to miss: - Had private testosterone level 9.4 saying low level + not trusting NHS lab ICE: - Ideas: low testosterone/ not believe NHS tests - Concern: BP meds masking problem - Expectation: TRT – referral if needed Management vital points - Offer examination + bloods (including LH/FSH/ prolactin/ testosterone/PS) to confirm (if raised, need to be referred for possible TRT) - Explain accuracy of lab results with high quality control standards to ensure accuracy. Private test can use different methods or reference ranges - Explain that even with this level is not an indication for TRT (TT>12 doesn't require / < 8 does need/ from 8-12 might need). Explain that his current symptoms might be related to stress/ ageing or lifestyle factors (lack of exercise/ weight/ unhealthy diet) - Treat ED with Viagra + advise Lifestyle changes
Grave's	As per notes
Hypothyroid	- Clue for Dx: patient has T1DM presented with worsening fatigue - As per notes
Post-partum thyroiditis	- Clue for Dx – postnatal review, patient report being anxious with palpitations + weight loss + bloods showing hyperthyroidism - Explain Dx: as immunity attacking thyroid glands usually in the 1st year after childbirth causing hyperthyroidism for 1-3 months then hypothyroidism for 3-6 months before returning to normal in most patients 20% will go to permanent hypothyroidism - Bloods Anti-TPO (high) to differentiate it from Grave's - In the thyrotoxic stage: offer BBs if severe symptoms, usually antithyroid will not be helpful as hyperthyroidism due to inflammation not increased production – refer all cases to 2ry care. In hypothyroidism state, treat if significant symptoms.
Osteoporosis – new	- As per note - As pt worried about risk of osteonecrosis of the jaw or atypical femoral fractures with the treatment. Explain rare incidence of these as 1/1000 – 1/10000 - Explain the -3 SD means that your bone density is very low in the lowest 0.2% of the population. Encourage weight bearing and muscle strengthening activities
Osteoporosis – review	- As per notes - Explain that DEXA showing some improvement but still in the osteoporotic range which means that you need to continue treatment for another 5 years to prevent fractures.
Pre-diabetes 1	- Prediabetes means that your blood sugars are higher than usual, but not high enough for you to be diagnosed with type 2 diabetes (HbA1C 42-47 mmol/mol) . However, if left untreated, there is a chance to go into diabetes. - Calculate QRISK - It is more about lifestyle changes in terms of 5-10% weight loss/ dietary changes/ more exercise/ less alcohol and smoking with 6 monthly review - Consider metformin if all changes failed to bring it down/ Can offer Orlistat if eligible - No Driving restriction
T1DM - DKA	- Clue for Dx: 2ry bedwetting then abdominal pain/ vomiting/ unwell/ drinking plenty of fluids/ no diarrhoea/ deep breathing/ weight loss - Explain DKA as serious condition that can happen in people with diabetes where a lack of insulin causes harmful substances called ketones to build up in the blood. It can be 1st presentation of DM in 40% of cases - It can be life threatening and needs urgent treatment in hospital for fluids via the drip and insulin with monitoring - Usually need ambulance, but in this case allow mum to take her to them given few minutes from the hospital

T2DM – new

T2DM -Diet

T2DM – social

T2DM – review

- As per notes
- Advise about carbohydrate counting and low glycaemia index food with leaflet
- Signpost to Diabetes UK resources
- Tailor your options based on patient preferences (e.g. MR metformin BD is better than TDS in patient working long shifts and might forget taking meds)
- Give options for healthy food: cooking for the week and store in the freezer/ healthy meals options/ free meals on wheels for eligible patients
- Artificial sweeteners is better than sugar. No clear evidence about it relation to cancer.

**Diabetes – insulin
and Ramadan**

- As per guide
- Main points: frequent blood sugar checking (2-4 times with dose change then 2-4 time weekly)/ educate about symptoms of hypos (<4 mmol) and hyperglycaemia (>16 mmol) / not to omit suhoor dose/ more fluids/ less sweets or CHO/ sick day rules/ less dates and less caffeine/ adjustment to their diabetes tablets or insulin
- High risk if poor control of T1DM/ DKA within 3 months prior Ramdan/ Advanced macrovascular complications/ CKD4 or 5/ pregnancy/old age/ unwell/ T2DM new on insulin without safe fasting before
- Metformin if OD – at iftar/ if BD higher dose at iftar and another dose at Suhoor/ If TDS, switch to BD. Avoid glibenclamide and shift to gliclazide and take once at iftar or twice with iftar and Suhoor (but decrease this suhoor dose if well control). No modifications for other oral agents
- Insulin: if OD: take at iftar – reduce by 15-30% / if BD: take morning dose at iftar and reduce evening dose by 50% and take at suhoor. If short acting: take iftar dose as normal, omit lunchtime dose, and reduce suhoor dose by 25-50%

CVD

11 M + dad worried about sudden cardiac arrest = cardiac arrest

Qs not to miss:

- Red flags: chest pain/ SOB/ leg swelling/ headaches/ palpitations/ collapse/ FH of sudden death < 50 years/ Known hereditary arrhythmias or cardiomyopathy
- RFs: FH of cardiac problems or sudden deaths or collapse

Cues not to miss:

- Bad experience of seeing son's colleague with sudden cardiac arrest
- If mentioned about ECG is not enough to exclude – then will ask for other invx

ICE:

- Ideas: Son's risk of dropping dead
- Concern: if son dropping dead with no defibrillator on site
- Expectation: GP arranges ECG – normal ECG then will reassure him about the risk

Management vital points

- Explain that sudden cardiac death is rare but serious where the heart suddenly stops working properly usually due to an underlying heart condition.
- Explain most causes are structural so no need for ECG and offer examination to exclude any murmur and for reassurance
- Educate about red flags and when to seek urgent care
- Not routinely offering ECG but for reassurance might offer ECG but can't rule other structural causes – can go privately for ECHO if still concerned

83 F + feeling lightheaded and irritation from down below + Postural drop = postural hypotension and polypharmacy

Qs not to miss:

- Explore fall as before/ during and after approach
- Red flags: chest pain/ SOB/palpitations/ falls/ seizures/ symptoms of stroke
- DDx postural hypotension/ vertigo/ mechanical falls/ seizures/ diabetes related
- RFs: less fluids/ polypharmacy/ diuretics/ caffeine
- Social history is very critical

Cues not to miss:

- Patient recently started furosemide for (leg swelling) and dapagliflozin
- Patient presented with 2 problems lightheaded and itchiness from down below with urinary frequency not responding to antifungal
- Less drinking as going a lot to the toilet

ICE:

- Ideas: have a weak bladder like your friend + read about oxybutynin can help with weeing a lot as her + no idea about light-headedness
- Concern: recurrent light-headedness and possible injury and fracture
- Expectation: sort out thrush and medications for the bladder to stop weeing as much + sort out lightheaded

Management vital points

- Explain the leg swelling in hot weather might be related to the weather and no need for furosemide now
- All related to medications

78 M + letter from A&E with recurrent falls = Anticholinergic burden	Qs not to miss: • Red flags: falls with fractures/ gait disturbance/ backout/ chest pain/ SOB/ palpitations • DDx orthostatic hypotension/ vertigo/ PD/ vision problems Cues not to miss: • Patient is on Solifenacin/ amitriptyline/ nefopam/ prochlorperazine/ furosemide • Patient has dry eye/ more sluggish bowels/ dizzy ICE: • Ideas: low iron? • Concern: affecting hobbies/ future injury • Expectation: diagnosis Management vital points • Explain Dx as a combined side effect of some medications with anticholinergic activities. ACB calculator to see this risk (3 or more is high risk) • Discuss need to stop these medications based on patient preferences • Involve occupational team if needed to assess safety at home
Exertional SOB = angina	• As per notes • Refer to rapid chest clinic for CT Angio + can start GTN spray and aspirin with BBs and statins if high QRISK • Patient presented with chest tightness on exertion • Driving – for group 1 – DVLA need not be notified – driving must cease when have symptoms at rest or with emotion or at the wheel • For group 2 – Must not drive and must notify DVLA – relicense if not angina for at least 6 weeks or met the requirement for DVLA • If angina occurs during sex, then GTN spray prior to sex can be used preventatively • Nitrates should not be taken with phosphodiesterase inhibitors (e.g. sildenafil, tadalafil): ○ Nitrate should not be taken for at least 24 hours after the last dose of sildenafil or vardenafil, and for at least 48 hours after the last dose of tadalafil ○ Phosphodiesterase inhibitor should not be taken for at least 24 hours after the last dose of a nitrate ○ If angina occurs during sex, do not use GTN, stop sex and phone 999 if pain does not resolve within 10 minutes
Chest pain (Acute MI) Acute MI 2	• MI is a serious life-threatening medical emergency that occurs when blood flow to the heart is blocked. Explain danger to life if not treated urgently • Chest/arm/jaw/back pain > 15 minutes + dull, central or crushing + with N/V/SOB/sweating + of new onset or abrupt deterioration of stable angina + occur on little or no exertion • We must call the ambulance for the patient – patient shouldn't drive • Patient can be presented with heartburn by worse than before • Explain we can arrange urgent care for dependant or pets via social care (upto 2 weeks) • If chest pain within the last 12 hours – refer as emergency. If chest pain > 12 hours ago, then refer to SDEC. If chest pain stopped >72 hours ago and no complications, consider Rapid Access Chest Pain. • While awaiting the ambulance advice to take GTN spray and another 3 aspirin tablets
Unstable angina	• Unstable angina– chest pain that happens when your heart muscle isn't receiving enough oxygen, and the attacks are more unpredictable (they may not have a trigger) and can continue despite resting • Ask about the last attack and if there any current pain • Explain risk to turn into full MI if left untreated • Immediate referral to SDEC and safety netting in the interim

Post-MI	• Explain importance of taking meds to prevent further heart attacks and prevent complications. • Confirm that the patient is enrolled with cardiac rehabilitation programme within 10 days (supervised exercise/ health education and stress management) • Work: can return to work once physically stable + involve occupational health for rules that need more heavy lifting or high stress • Driving: no need to tell DVLA for group 1 and usually stop for 1 week after PCI or uncomplicated or after 4 weeks if no PCI . For group 2 – must not drive for weeks and to inform DVLA • Sex: can resume once able to climb flight of stairs comfortably usually after 4 weeks. If ED, usually safe is stable and follow 24 hours rule as in angina case • Explain medications: dual antiplatelets/ ACEI/ BBs/ statins • Send leaflet about lifestyle modifications
CVD risk 1ry prevention	• Explain that statins can reduce risk by 1/3. If your risk of having MI or stroke is 10 in 100, with statin will be 7 in 100 in the next 10 years • Explain QRISK includes non-modifiable RFs such as age/gender/ethnicity/genetic and modifiable such as smoking/ diet/ weight/ exercise/ blood pressure and cholesterol • If QRISK $\geq 10\%$, start atorvastatin 20mg with aim to reduce non-HDL cholesterol by 40% within 3 months • Discuss lifestyle changes • Address concerns about side effects as being rare about 5 excess cases/10,000 over 5 years. Statin actually decrease the risk of dementia by 15%
CVD risk 2ry prevention	• As per notes • Inclisiran = Leqvio is used only for 2ry prevention if CVD+ acting by inhibition of production of PCSK9 in the liver + high LDL level 2.6 mmol or more despite max tolerated lipid lowering agents. Dose is 284mg SC then 2nd dose after 3 months then once every 6 months
HTN- new	• As per notes • Replace ACEIs with ARBs if persistent dry cough • For homeless patient: choose amlodipine given less need for bloods/ arrange double appointment if needed to do bloods and consultation/ refer to homeless health services/ engage social prescribers/ they can use GP address as their address/ tailor communication according to them/
HTN – old	
HTN – homeless	

Angry patient – for stopping Aspirin	- Complaint procedure: need to be acknowledged within 48 hours then written response within 10 days - Explain changes in guidelines and now not recommending aspirin in HTN
DNACPR without informing patient	- Explain DNACPR- DNACPR means if your heart or breathing stops your healthcare team will not try to restart it - Need explanation that DNACPR decision can sometimes made by healthcare on behalf of a patient if they are too unwell to participate in this decision when CPR is unlikely to prevent their death for more peaceful and dignified passing - Explain that DNACPR doesn't affect other treatment or personal care
ECG showing AF AF – anticoagulation	- As per notes - Driving: if group 1 – no need to notify DVLA but should stop driving if unable to safe control or stop the vehicle. For group 2, must notify DVLA - C (CHF)H (HTN)A₂ (Age)D(DM)S₂ (Stroke))-VASc (Vascular) Score
HFpEF HF HF deterioration	- As per notes - CCBs can be switched to ACEIs - For HF deterioration, NSAIDs is the cause for it. When stop them, some improvement in HF symptoms within few days. You may consider increase dose of diuretics temporarily
PAD	- As per notes - Driving: group 1, no need to notify DVLA. Group 2 must notify DVLA even may drive - Advise not to for the hiking holiday due to risk of ischemia due to less oxygen availability and advise to inform travel insurance
Aortic stenosis presented with systolic murmur	- Explain aortic stenosis as stiffness and narrowing of the aortic valve that control blood flow from heart to rest of the body putting more stress on the heart - Refer to urgent ECHO + ECG and bloods including BNP - If confirmed to refer to cardiology for possible valve replacement - CCBs is contraindicated in significant AS and given alternative as indapamide as the safest option to manage both HTN and the SOB due to overload - Inform DVLA and not drive
Palpitations	- DDx: cardiac/ pulmonary embolism/ thyroid/ anxiety/ anaemia/ menopause - RFs: caffeine/ medications/ drugs/ smoking - Red flags: chest pain/ syncope/ sudden cardiac death/ DVT signs/ unwell - Offer ftf for vitals and heart examination + bloods to exclude DDx + ECG - Avoid triggers - Notify DVLA if syncope or cardiac cause